

Senior-Care Operators — Buyer Research

(summary of the live page)

Generated 2026-09-10 from the same files that build <https://vincestars-cloud.github.io/campaign-previews/home-care-agency-owners/research/>. Every quote carries its source link. Numbers come from measured counts, never estimates; hypotheses are labeled.

TLDR

- **Who the buyer is:** the owner or administrator of a senior-care business across the whole ladder — home care, home health, hospice, assisted living / residential care homes, memory care, senior living, adult day, placement agencies, in-home ABA. Corpus: 14,451 documents read, **1,868 in the buyer's own voice (1,525 spoken by operating owners)**; sources are owner Facebook groups first, then owner subreddits, YouTube, provider reviews and trade press (chapter "Who the buyer is").
- **What they are trying to buy:** the phone ringing without renting families from a directory. A Place for Mom is the most-discussed paid source (42 buyer documents): \$50–68 a lead, the same family sold to several agencies, billed whether or not it converts; the second job is hiring and keeping caregivers (chapters "Lead Sources", "Frustrations", "Previous Solutions").
- **What they have already tried:** hospital and discharge-planner relationships (trusted, slow), the directories (shared), Facebook ads that brought caregivers instead of clients, coaches and licensing consultants. Nobody in buyer voice describes an inbound system they own (chapter "Previous Solutions").
- **Who sells to them today:** 95 advertisers with 233 live ads (203 to running businesses, 30 to would-be owners). The lane leaders sell *exclusive booked tours or assessments under your own brand, called back in minutes, guaranteed in 90 days* — Occupancy Partners, HomecareGrow, OBB, PatientsPipeline, Wisdom First, Census Home. Cleaned 2026-09-10: consumer and recruiting ads are out of the B2B set, adjacent-practice vendors are set aside (chapter "Competitor Ads").
- **The lane for inLeap:** families finding *you* instead of the directory — owned inbound plus intake, exclusive, priced on the result, with the caregiver-applicant by-product the same campaigns produce (chapters "Why They Buy", "Awareness → Offer Map").
- **Decision:** treat the "Why They Buy" chapter as the ad brief and build the offer + landing page from it next; the cost of waiting is that the six vendors above are already teaching this buyer what to expect.

Contents

1. [Who the buyer is \(Tab 10 — Buyer Summary, measured\)](#)
2. [Tab 1 — Fears](#)
3. [Tab 2 — Hopes & Dreams](#)
4. [Tab 3 — Relationship Fears](#)
5. [Tab 4 — Relationship Soundbites \(COPY TAB — how they say it to others\)](#)
6. [Tab 5 — Frustrations](#)

7. Tab 6 — Previous Solutions
8. Tab 7 — Solution Soundbites (*COPY TAB — hooks*)
9. Tab 8 — Desired Outcomes
10. Tab 9 — Typical Day
11. Lead Sources — what they have paid for and what it returned (first 6 voices per source; all 12 are on the page)
12. Why They Buy — the five answers an ad needs (Ad Brief)
13. Economics, Valuation & Exit — what the established buyer scores by
14. Retention at Scale — the growth ceiling and the valuation lever
15. Competitor Ads across the operator ladder (cleaned 2026-09-10)
16. Landing pages (B2B only)
17. Reviews by Star — buyer-side voice
18. Trade press & owner surveys (numbers with URLs)
19. Competitor marketing agencies — inLeap's direct set
20. Vendor / competitor map
21. Go-deeper thread cards
22. Headline candidates (Tab 11)
23. Frequency ranking (buyer-voice complaint patterns)
24. Vocabulary ledger
25. Awareness → Offer Map
26. Operator ladder, speaker gate and sources
27. Self-audit and backfill

Who the buyer is (Tab 10 — Buyer Summary, measured)

Who is speaking (measured, whole corpus 14,451): other 9,450, owner 1,525, employee 1,067, family 952, press 818, aspirant 343, vendor 296. Buyer voice = 1,868 (13% of the corpus): 1,525 operating and 343 pre-launch.

Stage (buyer documents): owner (operating) 1525, aspirant (pre-launch) 343, owner: brand new / first year 46, owner: established (years / revenue / locations) 19. The pre-launch share is a property of the sources (how-to-start videos, the startup subreddits), not a claim about the market; the tabs rank operating owners first so the copy is written to them.

Vertical named in the text (buyer documents, multi-count): home care (non-medical) 296, assisted living / RAL / care home 178, home health (skilled) 118, senior living / IL / CCRC / SNF 97, ABA / autism services 61, placement / referral agency 40, hospice 32, memory care 30, adult day 17, independent referral (realtor / transition / move manager) 8.

Payer named: private pay 104, Medicaid / waiver 89, commercial insurance (ABA) 42, Medicare 26, VA 6, LTC insurance 2. **Structure:** franchise 12, independent / from scratch 43.

Owner size statements (operating owners): names a client / resident / patient count 41, names revenue / run-rate 20, solo / owner-operator 13, names a caregiver / staff count 9, multiple locations / buildings 8, years in business stated 1.

Background stated: nurse / RN / LPN 9, corporate / other-industry 5, BCBA / therapist 4, CNA / caregiver first 2, realtor / real estate 1. **Gender markers:** female 9, male 10.

Geography (self-stated): Florida 32, California 30, Texas 29, Ohio 12, Houston 12, Georgia 11, Virginia 11, Canada 11, Michigan 9, Missouri 8, Indiana 7, New York 7, New Jersey 7, Wisconsin 7, Illinois 6.

Lead sources they name (buyer documents, from the Lead Sources ledger): Hospital / SNF / discharge planner referral relationships 128, Coaches, courses, consultants 91, Networking / BNI / chamber / senior centers / community events 62, SEO / website / Google Business Profile 58, A Place for Mom 42, Placement agents / referral agencies 39, Insurance panels / pediatricians / schools 25, Google Ads / LSA / PPC 17, VA / Medicaid waiver / LTC insurance contracts 16, Franchise systems 13.

Top buyer-voice pain themes (frequency, complaint-specific patterns): How to start / consultant / is it worth it 249, Reimbursement / Medicaid / private pay rates 149, Referral sources & placement agents: dependence, fees, how to get them 135, Marketing: ads / digital / SEO / website 113, Referral relationships: hospitals, discharge planners, SNFs, physicians 109, Licensing / accreditation / compliance / regs (agency or facility license, not a personal one) 103, Can't get clients / low census / need leads 90, ABA: credentialing, insurance panels, waitlists, referrals 88, Google Ads / Facebook ads / SEO / website that didn't pay 76, Franchise vs independent 70.

Read (synthesis; every number above is measured, this paragraph is the reading): the buyer is an operator-owner, most often a nurse, CNA or BCBA who went out on her own, running one location and doing the marketing herself. The pre-launch share of the corpus is large because the sources are large (how-to-start videos, the startup subreddits, the "Homecare for newbies" and "START A HOME CARE AGENCY" groups); the operating owners are the ones the tabs rank first. Their job, in their words, is *clients* (home care), *referrals* / *patients* (home health, hospice, ABA), *residents* / *beds* / *occupancy* / *tours* / *move-ins* (assisted living, RCFE, adult family home, senior living, nursing home), and *families* (placement and transition agents). They have a Google Business Profile and a

website, they have visited the hospitals, and when the phone does not ring they ask the group two questions in this order: "has anyone used A Place for Mom / Caring.com / CareInHomes?" and "does anyone know a good marketer who knows home care?" The answers they get are warnings about shared leads and a chorus of vendors. **Awareness:** problem-aware to solution-aware; they know they need inquiries, they equate marketing with directories, ads or a marketer, and they have no model of an owned inbound engine. **Buying trigger:** a call from A Place for Mom's sales rep, an empty unit, a first month with zero clients after licensing, or a referral source drying up ("lead quality dropped in the last 18 months and the cost per converted client stopped pencilling out"). **What they will not buy:** anything that looks like a directory, shared leads, a retainer without a number attached, or a cold vendor DM in the group. **Line that would stop the scroll (hypothesis, from the voices):** "Stop paying A Place for Mom for the family who was going to call you anyway." **Targeting (hypothesis):** members of the owner groups named on this page; page admins and engaged commenters; interests HCAOA, Home Care Pulse, AxisCare/WellSky/AlayaCare, RAL Academy, CentralReach; job titles owner / administrator / executive director / BCBA-owner; lookalikes of the feed advertisers' engagers (Occupancy Partners, Phoebe, Plena).

Tab 1 — Fears

Themes, measured (buyer documents; operators / pre-launch): Dependent on placement agents / A Place for Mom for every client 50 (48/2) · Getting scammed / bleeding money on leads 26 (25/1) · Waitlist, credentialing and insurance panels (ABA) 24 (23/1) · No clients / can't land the first one 23 (21/2) · Paid a referral site and the leads went to four other agencies 15 (15/0) · Going under / very close to giving up 6 (6/0) · The phone isn't ringing: no leads, no inquiries, empty pipeline 5 (5/0)

DEPENDENT ON PLACEMENT AGENTS / A PLACE FOR MOM FOR EVERY CLIENT — 50 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads. Initially, it was to be a few hrs a day. I drove over 30 minutes to complete the assesment, only to be turned away. I sat with our now client and her cousin, gave her some safety tips, and left our information with her. I then followed up with the cousin who initiated the contact via email, thanking her for the opportunity and letting her know that we are only a call away. Fast forward, one month later, on Oct 30, i received two back to back calls from the same cousin telling me that our now client was in the ED and could not go home unless there was a home care agency that would take her. Oherwise, she'd be sent to a TCU or nursing home. Cloent said she I agreed to the case, and confirmed a next day asesment. The cousin stated the client was only interested in a few hrs a day. Upon completing my assessment, I informed the client that she required around the clock. She wasn't happy but she signed up. All that to say, yes APFM is high risk, however as many say, the buy-in is in the followup."

Homecare for newbies · Dumah Darling (operator) · ↑13 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/1130570192569224>

"Has anyone ever had a A Place for Mom actually reach out to them? They contacted me today, signed me up, waived all my fees, and even set me up in the portal to start receiving private pay clients. I usually hear people say 'A Place for Mom ain't it,' so now I'm curious... how's it going for you?"

Homecare for newbies · Anonymous participant (operator) · ↑13 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

GETTING SCAMMED / BLEEDING MONEY ON LEADS — 26 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"Hello all! What are your top 2 referral sources? Is it google? referral partnerships? Facilities? We are trying to find what works and I know it's different for everyone. We have wasted time and money with A Place for Mom. Learned that lesson early"

Homecare for newbies (operator) · ↑11 reactions · <https://www.facebook.com/groups/706524484973799/posts/976062864686625>

"Not worth it from what I have heard. They sell same leads to other companies. Some are really old. You're better off going into hospitals and nursing homes, handing out business cards to social workers or nurses on each floor. Also go to community health clinics"

Private Pay Clients looking for Affordable Home Care Services. · Eva Horvath (operator) · ↑5 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

WAITLIST, CREDENTIALING AND INSURANCE PANELS (ABA) — 24 buyer documents

"I started a private practice AMA I'm a BCBA that went fully independent in 2021. I do all the aspects of business and clinical myself - including credentialing, billing, marketing, and provide all clinical services myself. I'm in CA. Ever wanted to go your own way? AMA!"

r/bcba (operator) · ↑66 upvotes · https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too! HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions · <https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I'm not in sales, but I'm a department head and know that in my company there are monthly census quotas, sale quotas and coming off the COVID situation lots of companies really cranked the pressure up on Assisted Living Communities back in May when the president declared the Covid state of emergency in the nation was "over". Almost immediately it was an expectation that everything get back to "business as usual" and they doubled/tripled the quotas for move ins. There are high quotas each months if the building is not fully occupied and even if it is fully occupied there is an expectation of having a sizeable wait list with people who have placed a deposit to hold apartments for the future. A lot of people aren't aware that assisted living communities are mostly owned by real estate investment trusts that have a contract with management companies whose names are on the signs of these properties. So the owners' groups who own the property put a lot of pressure on the management companies and at the community level it just compounds with expectations that are intensified. Our executive director and sales director have weekly conference calls with the corporate office where they are grilled about move ins on the books already confirmed, projected move ins with inquiries that have sounded promising and what it will take to get them to sign and put a deposit down, how many inquiries are in the database that haven't come into the community to tour and why and what can we do to get them in to tour and how many leads are in the database to follow up on as well as any move outs and a lot of details on the factors that led to the move out (death, needing skilled care, financial reasons, dissatisfaction). If you ever make an inquiry at a community that is part of a chain, even a smaller company, you will be logged into the system and the sales team will contact you every month until you formally demand to be taken off the list or until you say your relative has passed away. If you have come into the building for a tour, there will be even more contact until you say you have chosen another community (they will ask why), that your relative has passed or if you formally request no further contact. Edited to add: please also be aware that the deal that has been made is not permanent. The rate increases every January- this is industry wide. Depending on how much of a deal you get now, you may experience a higher percentage increase the next year. Everyone's rent goes up. It may be 3% it may be 12%. 6/8% seems to be the average in our area this year. Also- most communities give you a room rate and that is one thing, but care charges are a completely different thing. If your father begins requiring help with anything or if his needs of what is already being done increases the care charges will increase. If there is a rapid cognitive decline and needs change with dressing, bathing, continence, cueing to meals and activities, cueing to get up in the morning or not being oriented to place and time there will be a need for him to move to a memory care setting and that is generally significantly more expensive. There are also situations in my community where care needs are so personalized that a family hires a third party caregiver as well to be with their loved one and this could be for a variety of reasons. I know it can all be overwhelming and a lot of pressure to move in, feel free to ask all of the questions- staffing ratios, when shift changes are, percentage ratios of staff turnover, resident turnover, other charges that could arise for anything like transportation to doctor appointments to the community providing continence supplies. Just lots of information in a short time. Take your time absorbing the info and ask all the questions you need. Verbalize that you don't like the feeling of being pressured. You can put that in a google review as well (there is a huge push for people just moving in to do online reviews for a lot of these companies). Hoping this info is helpful and wishing you luck and know that so many of the people who work in the communities, like myself, feel so privileged to be trusted to care for people's loved ones each day, we are passionate about what we do, we love sharing time getting to know each resident and work so hard every day to help each person have a wonderful day!"

r/AssistedLiving (operator) · ↑5 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

NO CLIENTS / CAN'T LAND THE FIRST ONE — 23 buyer documents

"I am still not landing any leads for my Private Home Care Provider Agency. I have given out my business cards, brochures, sent emails, and gone to outreach community functions, but I haven't received even one call. What am I doing wrong?"

HOME CARE BUSINESS OWNERS (operator) · ↑21 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

"Your first two years will be your hardest I know agencies that opened 2 years now and still no clients. But clients will come trust and believe. Don't quit."

HOME CARE BUSINESS OWNERS · SkilledDeer5120 (operator) · ↑11 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/283021973066269...>

"Hi Jenny Jen Congrats on getting licensed — the fact that you've already mailed 4,000 flyers and are working referral leads tells me you're serious, which is more than most. The challenge is that those channels are cold. Your fastest path to a first client is almost always warm referrals — hospital discharge planners, social workers, and senior living advisors who personally hand you a name. Building those relationships takes showing up consistently in person, not just dropping off brochures. Happy to share what's worked for agencies I've helped scale. Feel free to connect."

HOME CARE BUSINESS OWNERS · Albert Raven (operator) · ↑10 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/284819363219864...>

PAID A REFERRAL SITE AND THE LEADS WENT TO FOUR OTHER AGENCIES — 15 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"It's hard to make it make sense. You are paying \$60 for a lead that's being shared with 5 or more other agencies at the same time, so your likelihood of closing on any of them is extremely low. A much better approach would be to learn how to generate and nurture your own leads through Google and Meta ads and other lead generation approaches, so the lead is all yours with no competition. My ads typically cost me around \$6 per lead and my close rate is pretty high. Hands down the best decision I've made for my agency. Hope this helps."

HOME CARE BUSINESS OWNERS · Edward Davis (operator) · ↑9 reactions · <https://www.facebook.com/groups/2156388261379184/posts/288229603545506...>

"Not worth it from what I have heard. They sell same leads to other companies. Some are really old. You're better off going into hospitals and nursing homes, handing out business cards to social workers or nurses on each floor. Also go to community health clinics"

Private Pay Clients looking for Affordable Home Care Services. · Eva Horvath (operator) · ↑5 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

GOING UNDER / VERY CLOSE TO GIVING UP — 6 buyer documents

"I NEED HELP, I started my home care company in 2019. Last year for 5 months I saw a little profit. Because of staffing I lost most of my clients and now I find myself having a hard time rebuilding. I have no money and don't know where to go from here. I am very close to giving up. The only thing keeping in the game this is truly my passion."

youtube · @ebonymonroe-p3v · Starting a non-medical home care agency (operator) · ↑6 likes · <https://www.youtube.com/watch?v=P3bLuxFwAl&lc=Ugwb6rtk3xFmDTOfKN...>

"Selena Mack you need a coach/mentor to help you get back on track. I'm not saying you should use me but you should use someone. I would hate to see you give up on everything you've worked for. You just need help. The help is out here. Interview several coaches, talk with them and see what they offer. See if you can fit it into your budget. And choose one you connect with. The biggest step was writing this post!"

HOME CARE BUSINESS OWNERS · Ann Businger to Selena Mack's reply (operator) · ↑4 reactions · <https://www.facebook.com/groups/2156388261379184/posts/279719584063175...>

"Don't give up ! I just started and don't have a single client if your employees want to be paid they need to obey by your rules"

HOME CARE BUSINESS OWNERS · Kristal James (operator) · ↑3 reactions · <https://www.facebook.com/groups/2156388261379184/posts/279719584063175...>

THE PHONE ISN'T RINGING: NO LEADS, NO INQUIRIES, EMPTY PIPELINE — 5 buyer documents

"I am still not landing any leads for my Private Home Care Provider Agency. I have given out my business cards, brochures, sent emails, and gone to outreach community functions, but I haven't received even one call. What am I doing wrong?"

HOME CARE BUSINESS OWNERS (operator) · ↑21 reactions · <https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

"Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

ABA Business Owners · Anonymous participant (operator) · ↑4 reactions ·
<https://www.facebook.com/groups/669886889110965/posts/942369488529369>

"Struggling to get referrals for ABA company?. Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

Non-BCBA Owners ABA Business Collaborative · Anonymous participant (operator) · ↑3 reactions ·
<https://www.facebook.com/groups/1081018657046251/posts/143052813209530...>

Tab 2 — Hopes & Dreams

Themes, measured (buyer documents; operators / pre-launch): Be my own boss / make my dream real 21 (5/16) · Give back / help seniors 20 (13/7) · Build, scale, sell 8 (5/3)

BE MY OWN BOSS / MAKE MY DREAM REAL — 21 buyer documents

"I used to sjust stay quiet and mind my own business but there are some of us on these platforms who are genuinely looking to exchange help, ideas and function as peers. I don't mind paying for coaching or mentorship which I am doing. However, I pay someone I have seen demonstrate that they know what they are talking about and can get me the results I am looking for."

HOME CARE BUSINESS OWNERS · Pamela Styles (operator) · ↑10 reactions · <https://www.facebook.com/groups/2156388261379184/posts/273224561379344...>

"Thank You 🙏 this was much needed information. I started my home health care business back in 2018 and we were doing well until a few bad partners decided to steal from me and then we was hit hard by Covid which took out most of my clients so now I'm starting over. I revamped my business by changing the name and building a better team it's been so hard and at times discouraging but it's my passion and my dream for as long as I can remember. I love serving others especially the elderly people in my community. Thanks again 🙏💕🌹"

youtube · @tamelatate9087 · How To Start A Home Care Agency | Episode 1 - Getting Starte (operator) · ↑6 likes · <https://www.youtube.com/watch?v=H2PQbatWAWs&lc=UgzsaifPZHdzxdatLEd...>

"I have been a BCBA for over 15 years and I have decided to start my own business/clinic. I already have my LLC, and I am credentialed with TriWest and BCBS. I am planning on starting out with just 2 RBT's and myself and grow as appropriate. My plan is to have a relatively small clinic and hope to do the billing and manage referrals/reauthorizations myself. I need mentoring on billing, general business set up, referral/authorization process (I know how to write ITPs & PRs) with Triwest, BCBS, and Aetna in Texas specifically. I have posted before, but I got recommendations for companies that DO the billing, etc. for you -- I need coaching on how to do it myself. Any recommendations would be appreciated!"

ABA Business Owners · Anonymous participant (operator) · ↑5 reactions · <https://www.facebook.com/groups/669886889110965/posts/951864540913197>

GIVE BACK / HELP SENIORS — 20 buyer documents

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life. Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you. Case Manager: Good morning. It's nice to meet you. Administrator: Thank you. Before I leave, may I ask you one quick question? Case Manager: Of course. Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations. Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital. Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises. Case Manager: I like that. Thank you for taking the time to stop by. Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.) Case Manager: Thank you. I'll definitely keep this for future reference. Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community. Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit. Case Manager: Thank you so much. That's very thoughtful of

you. Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"Almost exactly 5 years ago, I bought my first assisted living facility. A 10-home, 94-bed CBRF in Wisconsin. I wish I could tell you the last 5 years have been easy and it's been nothing but success stories. That would be a lie, at least partly. The truth? We got our asses kicked lol. Things started out good. We were cashflow positive, starting to better understand the business and get things down. The first thing we knew we had to work on was staffing. This is right around the time the government programs related to Covid kicked in, and many people, not all, realized they could make more money staying home than they would if they went to work. And, well, a lot of people chose to stay home. We tried to combat it by raising the pay for our caregivers. In many cases, we increased caregiver pay by 50%+. But it still didn't solve the problem, so we started working with 3rd party staffing companies. At one point, we were paying around \$60k every single month just in 3rd party staffing. Our profitable business soon turned cash flow negative. At the same time, we were also battling vacancies. We fluctuated from 7 to 14 empty beds on a monthly basis. It seemed like no matter how hard we worked, we couldn't keep up. We would fill three beds and then lose 4. Then we would fill another one and lose two. It was a constant battle of up and down, without ever really getting ahead. Everyone was stressed, including me, and nothing we tried seemed to solve the occupancy problem. Today, as I write this, all 94 of our beds are full and we have a wait list. We are fully staffed without spending a single dollar on 3rd party staffing. Our staff is happier, stronger, and working together better than ever. How did we go from where we were to where we are today? We changed what we were doing. It doesn't matter how hard you work. If you're working on the wrong things, working harder isn't going to fix it. You need a different approach and a different system. And that's exactly what we did. I'm sharing this because I constantly see people in these groups struggling with staffing and keeping their beds full. I'm thinking about doing a Zoom call for this community and walking through exactly what we're doing to keep our beds full and our facilities staffed.. I'll have my team on the Zoom as well so you can ask questions at the end. I'm not going to charge for it and I'm not selling anything on the call. I just want to help people who are struggling with these same issues or know they could improve in these areas. If you're interested, hop on a Zoom call with us on August 20th at 8pm EST and we'll figure it out together a day at a time. Comment "BEDS" and I'll DM you the link. Facebook sometimes sends messages from people you aren't connected with to your Message Requests folder. If you comment "BEDS," keep an eye on your Message Requests in case my message lands there."

Rcfe Owners Network Group (operator) · ↑28 reactions · <https://www.facebook.com/groups/352285274494478/posts/976832502039749>

"Thank You 🙏 this was much needed information. I started my home health care business back in 2018 and we were doing well until a few bad partners decided to steal from me and then we was hit hard by Covid which took out most of my clients so now I'm starting over. I revamped my business by changing the name and building a better team it's been so hard and at times discouraging but it's my passion and my dream for as long as I can remember. I love serving others especially the elderly people in my community. Thanks again 🙏💖🌹"

youtube · @tamelatate9087 · How To Start A Home Care Agency | Episode 1 - Getting Starte (operator) · ↑6 likes · <https://www.youtube.com/watch?v=H2PQbatWAWs&lc=UgzsaifPZHdzdatLEd...>

BUILD, SCALE, SELL — 8 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"I'm feeling frustrated and overwhelmed, and I could really use some advice from those who have been in my shoes. I'm in California and received my home care agency license on March 30, 2026. Since then, I haven't had any luck getting clients. My first thought was to reach out to the assisted living facility where I previously worked. I was hopeful they would add my agency to their referral list, but I've been ignored. I've also contacted other assisted living facilities, but it seems like the wellness directors, administrators, or the people who handle caregiver referrals either don't answer calls, are never available, or won't give me an opportunity to meet with them. I'm blessed to have a wonderful support system at my church. Some of our seniors live in assisted living facilities, and their families have even tried speaking with the facility directors about using my agency. Unfortunately, they were told the facilities already work with five agencies and aren't willing to add any more. I'm honestly heartbroken. I invested a lot of time, money, and faith into building this business, and right now it feels like I'm not making any progress. Thankfully, I still have my full-time job, which helps keep me busy, but I truly want to grow my agency and make it successful. For those of you who have started a home care agency in California, what helped you get your first clients? How did you build referral relationships when doors kept closing? Are there strategies that worked for you besides assisted living facilities? I would truly appreciate any advice, encouragement, or lessons you've learned. Thank you so much"

HOME CARE BUSINESS OWNERS (operator) · ↑11 reactions · <https://www.facebook.com/groups/2156388261379184/posts/289670824068051...>

"Jelena Jeca Hey, good morning! Did you read the item description or click the link to the product? This list is vendors and commercial companies that actually send you clients once you're credentialed with them. It's literally my own research from trial and error — my success blueprint from the companies I used to grow my agency. Think of it like me giving you a list of all the jobs hiring in your area so you don't have to waste time searching. I'm giving you a roadmap of vendors and commercial companies you can contract with to get clients"

Private Pay Clients looking for Affordable Home Care Services. · Toya TK to Jelena Jeca's reply (operator) · ↑1 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

Tab 3 — Relationship Fears

Themes, measured (buyer documents; operators / pre-launch): Marriage & family pulled into the business 10 (6/4) · Burnout / on-call / the grind wears you down 9 (6/3)

MARRIAGE & FAMILY PULLED INTO THE BUSINESS — 10 buyer documents

"My wife and I started our home healthcare business February of last year 2021 and thank you for this it is definitely helpful and inspiring. Thanks to Gooch we will be connect real soon.. 🙌 "

youtube · @tyronejones5813 · The 31 Year Old Multimillionaire Who Started A Home Healthca (operator) · ↑17 likes · <https://www.youtube.com/watch?v=q2hvm10d4uo&lc=UgzTJqa0a-ZKE7laPOZ...>

"Hello @TheProcessLife It sounds fair to put your husband as an owner as well if he is going to help with the business but that is just my opinion. Discuss it with him and see how he feels about it. Thanks for watching! Good luck with everything ❤️ "

youtube · @triciathomasrn9529 · You Can Make Over \$500k Owning a Homecare Agency! (operator) · ↑3 likes · <https://www.youtube.com/watch?v=o7zYMIJXVRw&lc=UgxFp24UXodTYTzlezB...>

"I own a non-medial homecare agency. We are in the back 6 months of our 3rd year of operation and reached 7 figures in our 2nd year for gross income. We have found high turnover rates occur at the beginning of operations. Once you're settled with a decent book of business, you're able to find good employees who then refer their friends. Our turnover rates are closer to 50%. We are not certified with Medicaid and we are part of a franchise. I run the business with my wife and one other administrator and we have about 40 employees working part time and full time. Feel free to ask questions."

r/Entrepreneur (operator) · ↑2 upvotes · https://www.reddit.com/r/Entrepreneur/comments/1rqjot3/pe_is_dumping_b...

BURNOUT / ON-CALL / THE GRIND WEARS YOU DOWN — 9 buyer documents

"Any insight on Lic nursing home administrator and/ or AIT program. Looking for Pros and Cons. How difficult is it being an admin? I'm looking for a better work/life balance. Plus I'm burned out working clinical. I've worked in SNFs for 11 years as a therapist. 7 years as the DOR. I've worked closely with MDS. I've been a part of a wound care team. I've also worked with nursing to implement fall programs and other collaborations. My point is I've seen a lot of how the back of the house works. Also, my previous career was owner/operator of a high volume family restaurant. I have experience with budgets and P&L. I know this kinda sounds like a resume. But I think it's beneficial offering opinions if one knows my experience. Even when this post becomes old, any advice and comments would be appreciated."

r/healthcareadmin (operator) · ↑20 upvotes · https://www.reddit.com/r/healthcareadmin/comments/sych6p/any_insight_o...

"This is a good question. My gut tells me that a nursing home administrator would pay more than an assisted living administrator because it's a higher level of care, but I also think it depends on the size of the facility. One thing you want to be aware of for both, is as the administrator, you are responsible for everything going on at the facility and you need to make sure people (staff and residents) are following all the rules. You'll need to be great at communicating, and also make sure you stay on top of all the rules and regulations. You're also required to cover any shifts that get dropped, and you'll be on call 24/7. It can be a very demanding job. As the administrator though, you are in charge and can have a huge impact on the staff and residents and it can be extremely rewarding. I hope that helps!"

youtube · @assistedlivinginvesting · DAY IN THE LIFE OF AN ASSISTED LIVING FACILITY ADMINISTRATOR (operator) · ↑2 likes · https://www.youtube.com/watch?v=GWgKaNjsG_s&lc=Ugy6dgpU6Qa323zIHS1...

"Royal Citizen Ind honestly it's not all about money. There are a lot of entrepreneurs who don't know about the conference, so I was giving them a chance to be in the room for other valuable info as well. When I first started and was looking and researching information I noticed groups and ppl only tell you bits and pieces and that's where I come in giving the full recipe instead of crumbs. Some ppl are burnout and they want the info as some would rather gsin the info in person. Wish you the best"

HOME CARE BUSINESS OWNERS · Kakila Taylor to Royal Citizen Ind's reply (operator) · ↑2 reactions · <https://www.facebook.com/groups/2156388261379184/posts/273224561379344...>

Tab 4 — Relationship Soundbites (*COPY TAB — how they say it to others*)

Themes, measured (buyer documents; operators / pre-launch): Starting out / hold my hand 201 (76/125) · How do I get referrals? 183 (167/16) · Placement agents, referral fees, A Place for Mom 83 (78/5) · How do I start, license, partner, buy an existing license? 67 (27/40) · How do I get referrals from hospitals, discharge planners, doctors? 53 (50/3) · How do I get clients? 37 (31/6) · How do I hire / keep caregivers? 20 (19/1) · How do I bill / get paid? 13 (9/4) · Is RAL / a facility worth it? Is it profitable? 10 (3/7) · I'm not landing any leads / clients — what am I doing wrong? 8 (6/2) · How do I get clients as an independent placement / transition specialist? 8 (8/0)

STARTING OUT / HOLD MY HAND — 201 buyer documents

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life. Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you. Case Manager: Good morning. It's nice to meet you. Administrator: Thank you. Before I leave, may I ask you one quick question? Case Manager: Of course. Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations. Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital. Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises. Case Manager: I like that. Thank you for taking the time to stop by. Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.) Case Manager: Thank you. I'll definitely keep this for future reference. Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community. Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit. Case Manager: Thank you so much. That's very thoughtful of

you. Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions · <https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"Hi everyone, I'm in the final stages of finalizing my application for my home care business. I'm starting out with private pay clients only. I wanted to ask what systems everyone uses to keep track of caregiver information (profiles, credentials, background checks, availability, etc.). Do you use a specific caregiver management or scheduling software, or do you keep records another way? I'd really appreciate any examples or recommendations of what works well when starting out. Thank you!"

Home Health Care Agency Owners & Entrepreneurs · Chrystal Bowen (operator) · ↑27 reactions · <https://www.facebook.com/groups/1154838852454654/posts/158909693569550...>

"Start your home care agency.. Hi! I know how overwhelming and tedious the home care licensing can be. When I was looking for help I felt like everyone was trying to poach me for money. Literally no one would help me without a price tag. I created a SKOOL platform to help everyone for free. It took me over a YEAR to start and submit because I literally trusted no one. Please join! We have a community of people who are from all over the states and we can help each other there, and for free. ["

r/RunAHomeCareAgency (operator) · ↑11 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1cejzsp/start_you...

"Start your home care agency. Hi! I know how overwhelming and tedious the home care licensing can be. When I was looking for help I felt like everyone was trying to poach me for money. Literally no one would help me without a price tag. I created a SKOOL platform to help everyone for free. It took me over a YEAR to start and submit because I literally trusted no one. Please join! We have a community of people who are from all over the states and we can help each other there, and for free. ["

r/RunAHomeCareAgency (operator) · ↑11 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1cejzsp/start_you...

HOW DO I GET REFERRALS? — 183 buyer documents

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life. Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you. Case Manager: Good morning. It's nice to meet you. Administrator: Thank you. Before I leave, may I ask you one quick question? Case Manager: Of course. Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations. Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital. Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises. Case Manager: I like that. Thank you for taking the time to stop by. Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.) Case Manager: Thank you. I'll definitely keep this for future reference. Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community. Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit. Case Manager: Thank you so much. That's very thoughtful of

you. Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions ·

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"I'm feeling frustrated and overwhelmed, and I could really use some advice from those who have been in my shoes. I'm in California and received my home care agency license on March 30, 2026. Since then, I haven't had any luck getting clients. My first thought was to reach out to the assisted living facility where I previously worked. I was hopeful they would add my agency to their referral list, but I've been ignored. I've also contacted other assisted living facilities, but it seems like the wellness directors, administrators, or the people who handle caregiver referrals either don't answer calls, are never available, or won't give me an opportunity to meet with them. I'm blessed to have a wonderful support system at my church. Some of our seniors live in assisted living facilities, and their families have even tried speaking with the facility directors about using my agency. Unfortunately, they were told the facilities already work with five agencies and aren't willing to add any more. I'm honestly heartbroken. I invested a lot of time, money, and faith into building this business, and right now it feels like I'm not making any progress. Thankfully, I still have my full-time job, which helps keep me busy, but I truly want to grow my agency and make it successful. For those of you who have started a home care agency in California, what helped you get your first clients? How did you build referral relationships when doors kept closing? Are there strategies that worked for you besides assisted living facilities? I would truly appreciate any advice, encouragement, or lessons you've learned. Thank you so much"

HOME CARE BUSINESS OWNERS (operator) · ↑11 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/289670824068051...>

PLACEMENT AGENTS, REFERRAL FEES, A PLACE FOR MOM — 83 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxy/i_put_toge...

"LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads. Initially, it was to be a few hrs a day. I drove over 30 minutes to complete the assesment, only to be turned away. I sat with our now client and her cousin, gave her some safety tips, and left our information with her. I then followed up with the cousin who initiated the contact via email, thanking her for the opportunity and letting her know that we are only a call away. Fast forward, one month later, on Oct 30, i received two back to back calls from the same cousin telling me that our now client was in the ED and could not go home unless there was a home care agency that would take her. Oherwise, she'd be sent to a TCU or nursing home. Cloent said she I agreed to the case, and confirmed a next day asesment. The cousin stated the client was only interested in a few hrs a day. Upon completing my assessment, I informed the client that she required around the clock. She wasn't happy but she signed up. All that to say, yes APFM is high risk, however as many say, the buy-in is in the followup."

Homecare for newbies · Dumah Darling (operator) · ↑13 reactions · <https://www.facebook.com/groups/706524484973799/posts/1130570192569224>

"Has anyone ever had a A Place for Mom actually reach out to them? They contacted me today, signed me up, waived all my fees, and even set me up in the portal to start receiving private pay clients. I usually hear people say 'A Place for Mom ain't it,' so now I'm curious... how's it going for you?"

Homecare for newbies · Anonymous participant (operator) · ↑13 reactions · <https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

HOW DO I START, LICENSE, PARTNER, BUY AN EXISTING LICENSE? — 67 buyer documents

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions · <https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"What are the steps to start a residential care facility in Roseville, CA?. Hi RCFE owners/mgrs, May I please ask for your help and mentorship with getting my first RCFE started in Roseville, CA? I recently purchased a home and will be taking the administrator course soon and then apply for a facility license. I have been unable to find out the following: 1) where to get patients from? 2) how to get paid by the patients? Do patients pay me directly or does insurance pay me? 3) where to hire caretakers from? 4) what type of business or worker comp insurance will I needed? 5) while I am applying for facility license, should I rent out the house with a one year lease, to help with mortgage payments? 6) getting facility licensure may take a while, so can I submit facility license application before I obtain an admistrator license? I am sure there are many more questions I need to ask. Please help me with your expertise and experience. Thank you for your time and support!"

Residential Care Homes Owners/Operators · Van Voong (operator) · ↑6 reactions ·
<https://www.facebook.com/groups/732719531430807/posts/1399974124705341>

"Partner with local hospitals, rehabilitation centers, social service workers etc. Visit senior communities and set dates to do activities such as bingo, movie events etc. Take marketing info to churches, neighborhoods, recreation centers etc., Create field days where you strictly use the day to get out and be visible. Have on your company shirts etc and smile!! Let the community you are in know you are there!!! Put in the work!!! It pays off!! I am a certified professional coach, I coach in non medical home care and other businesses as well! Here to help anyone in need of building from the ground up, marketing and other sources of business that can be added to your healthcare agency!"

Home Care & Home Health Network · Coachc Stella a year ago (operator) · ↑5 reactions ·
<https://www.facebook.com/groups/1429922527043737/posts/981818785488378...>

"'Boots On The Ground' is what I call it and how to bring in referrals. You can definitely pay different companies to barely give you more than they take but I do marketing for an agency and getting out posting flyers, doing presentations at different centers and events for seniors bringing compassion in with me as well as providing them with treats etc and not just presenting a service to them. They have to feel safe and trust you. We try to engage with them before hand because remember you are still a stranger although you are trying to help and let's also make sure they KNOW you just HELPING they don't want to feel like a hinder or burden and some transitions are hard but you assure them in those presentations that it's only assistance. Build bonds, stand out different from other agencies especially when it comes to marketing. Get in touch or try to start a partnership with doctors, case managers, social workers, rehabilitation centers, health departments, etc. You need to make sure the providers and people that regularly care for the clients are aware of your services and know that you can provide the best services possible as well."

HOME CARE BUSINESS OWNERS · Teywana Gill a year ago (operator) · ↑3 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/254985463536587...>

HOW DO I GET REFERRALS FROM HOSPITALS, DISCHARGE PLANNERS, DOCTORS? — 53

buyer documents

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life. Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you. Case Manager: Good morning. It's nice to meet you. Administrator: Thank you. Before I leave, may I ask you one quick question? Case Manager: Of course. Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations. Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital. Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises. Case Manager: I like that. Thank you for taking the time to stop by. Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.) Case Manager: Thank you. I'll definitely keep this for future reference. Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community. Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit. Case Manager: Thank you so much. That's very thoughtful of

you. Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions · <https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"Contact VA social workers/case managers. You have to do footwork. Go to veteran organizations and market yourself. It took me 6 months to get my 1st client and after that I had 30 clients 3 months after that"

HOME CARE BUSINESS OWNERS · Paris Justice (operator) · ↑11 reactions · <https://www.facebook.com/groups/2156388261379184/posts/285582417143558...>

"Hi Jenny Jen Congrats on getting licensed — the fact that you've already mailed 4,000 flyers and are working referral leads tells me you're serious, which is more than most. The challenge is that those channels are cold. Your fastest path to a first client is almost always warm referrals — hospital discharge planners, social workers, and senior living advisors who personally hand you a name. Building those relationships takes showing up consistently in person, not just dropping off brochures. Happy to share what's worked for agencies I've helped scale. Feel free to connect."

HOME CARE BUSINESS OWNERS · Albert Raven (operator) · ↑10 reactions · <https://www.facebook.com/groups/2156388261379184/posts/284819363219864...>

HOW DO I GET CLIENTS? — 37 buyer documents

"Anyone received a call from a place for mom? What are your thoughts they said they found my business on the licensing web and want to see if I need clients"

HOME CARE BUSINESS OWNERS (operator) · ↑13 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/273192195049247...>

"Looking for marketing ideas! . I have started my own home care agency in Alberta, Canada. I am looking for great ideas on how to get my first clients. I am currently doing a lot of in person visits to various places such as independent living facilities, seniors centers, pharmacies, doctors offices,etc. Besides the in person stuff, we have our website, some social media (looking at having it professionally managed). Any helpful insight is appreciated!"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...

"Looking for marketing ideas! I have started my own home care agency in Alberta, Canada. I am looking for great ideas on how to get my first clients. I am currently doing a lot of in person visits to various places such as independent living facilities, seniors centers, pharmacies, doctors offices,etc. Besides the in person stuff, we have our website, some social media (looking at having it professionally managed). Any helpful insight is appreciated!"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...

"Need Help Generating Leads for Senior Living Communities I'm working on something that really matters.....helping **senior living communities connect with the people who need them most**. But here's the thing: I'm struggling to **generate quality leads** that actually turn into real conversations. If you've spent years **cracking the code on healthcare marketing, senior living, or high-intent lead generation**, I'd love to talk. I'm not looking for generic advice I'm looking for someone who's been in the trenches, knows what works, and wants to make a real impact."

r/SeniorLivingMarketing (operator) · ↑5 upvotes · https://www.reddit.com/r/SeniorLivingMarketing/comments/1q4tvqw/need_h...

HOW DO I HIRE / KEEP CAREGIVERS? — 20 buyer documents

"I am concerned about the high staff turnover rate in childcare. Members of the management in most to all centre's seem perplexed about the reasons behind this trend. The practice of maximising room occupancy to meet required ratios, as well as enrolling more children without the necessary staff resources, is resulting in situations where children with potential undiagnosed needs are not receiving the support and attention they require. This not only compromises the quality of care provided but also puts undue pressure on our dedicated educators. It is concerning to witness talented educators leave in search of better working environments where their contributions are valued and supported. The lack of a strong duty of care towards both our employees and the children in centre's is evident and requires immediate attention. I strongly urge all to consider the well-being and professional development of our employees, as they are the cornerstone of our childcare profession. By fostering a nurturing and supportive work environment, we can not only retain valuable staff members but also provide a higher standard of care for the children under our supervision"

ECE Community · Anonymous participant (operator) · ↑50 reactions ·

<https://www.facebook.com/groups/248675758568554/posts/6929343677168362>

"Finally licensed!. Finally licensed here in Sacramento, Roseville area and hired my first two caregivers. If anyone needs help with anything at all please let me know. I feel like an expert, at least with the licensing and start up stage. I will keep everyone posted as I go along! So many people have messaged me about this, I am creating a free licensing forum on my Skool platform. Please join me there, I am hardly on here. [Update: My Skool community blew up! And I made \$1.1Million in revenue my first year. It is now March and we are already at \$250K for the year. I have since went to paid for Skool to continue providing valuable resourses with the live QAs from industry professionals and future guests. There are 5 modules everything from email domination for placement agents that got me 3 placement agents (I sent it to 36 agent total), how to hire caregivers and all the marketing things I did."

r/RunAHomeCareAgency (operator) · ↑10 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

"Finally licensed! Finally licensed here in Sacramento, Roseville area and hired my first two caregivers. If anyone needs help with anything at all please let me know. I feel like an expert, at least with the licensing and start up stage. I will keep everyone posted as I go along! So many people have messaged me about this, I am creating a free licensing forum on my Skool platform. Please join me there, I am hardly on here. [Update: My Skool community blew up! And I made \$1.1Million in revenue my first year. It is now March and we are already at \$250K for the year. I have since went to paid for Skool to continue providing valuable resourses with the live QAs from industry professionals and future guests. There are 5 modules everything from email domination for placement agents that got me 3 placement agents (I sent it to 36 agent total), how to hire caregivers and all the marketing things I did."

r/RunAHomeCareAgency (operator) · ↑10 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

"Anyone know how to digital market for senior care?. I've become way too dependent on the placement agents that have been bringing me clients. Im looking to see if anyone has tried digital marketing or run ads? Update: I started running ads and even though I have not gotten a single client i got a ton of caregivers and everyone in our community started to recognize us. Because of this by product of running these ads we have started to become known. Win? Sure, I'll take it. I am posting my experience with digital marketing on my Skool platform. The first 7 days are free and then it a monthly subscription to be apart of my community. ["

r/RunAHomeCareAgency (operator) · ↑5 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1ffa065/anyone_kn...

HOW DO I BILL / GET PAID? — 13 buyer documents

"I realize this is an older video. At the time of this writing it is May 2023. I think some things have changed in the insurance world. I started a private practice in late 2022. For the most part, all of my clients prefer to use insurance over self pay. I have only had one denial so far that I am still trying to appeal. And, so far, I only have one case of an insurance company losing my claims (they say they have no record of claims being sent to them when I have evidence that they were sent in). So, there have been a couple of problems that have needed my attention, but for the most part, I am getting paid on a regular basis. I do not live in an affluent area of the USA and most people do not want to pay cash if their insurance will cover their sessions. The city I live in is the capital of my state and many working individuals in my area are employed by the state and have insurance that is reliable. Another source of income for me is EAP counseling. Although EAP does not pay very well, the clients who use it usually decide to remain on my caseload after their EAP benefits run out and then I can bill their insurance companies. There is one particular insurance company I work with that does not pay me well at all, and I might not renew my contract with them next year. I just want to provide some encouragement for people who are thinking about taking insurance. I love Private Practice Skills. It has been a huge help to me. Thank you."

youtube · @lesliesalmon6116 · Insurance vs Private Pay Private Practice | My Experience wi (operator) · 15 likes · <https://www.youtube.com/watch?v=zvtl029wKTU&lc=Ugznprtsa1TsTp-QrY1...>

"Requirements for Home Health rates in Iowa? . Does anyone know where to find your states going rates that they charge for a Home health CNA versus a home health aide versus a nurse? I'm having a lot of difficulty finding the nursing rates. My agency just started doing home health and we want to make sure we're within or below the threshold for rates. Upon hours of research i'm at a dud with it. We are currently only accepting private pay and private insurances. We are in the application process of getting approved for medicare/medicaid. hoping someone has some insight, thanks!"

r/RunAHomeCareAgency (operator) · 12 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1b244t7/requireme...>

"Requirements for Home Health rates in Iowa? Does anyone know where to find your states going rates that they charge for a Home health CNA versus a home health aide versus a nurse? I'm having a lot of difficulty finding the nursing rates. My agency just started doing home health and we want to make sure we're within or below the threshold for rates. Upon hours of research i'm at a dud with it. We are currently only accepting private pay and private insurances. We are in the application process of getting approved for medicare/medicaid. hoping someone has some insight, thanks!"

r/RunAHomeCareAgency (operator) · 12 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1b244t7/requireme...>

"How do you do add ons/ extras for your packages? Do you charge more per hour or what? Me and my team are thinking about doing packages with add ons but we are lost on how to add extras"

Home Health Care Agency Owners & Entrepreneurs · Anonymous participant 627 to Real Is's comment (operator) · 12 reactions · <https://www.facebook.com/groups/1154838852454654/posts/150554251071761...>

IS RAL / A FACILITY WORTH IT? IS IT PROFITABLE? — 10 buyer documents

"I can assist you! There are two parts to our service. The first is your online presence. Making sure your agency shows up online without adding to your overhead cost. This is also where you can list any other payment methods you accept and information that sets you apart from the other agencies in your area. An example would be if you have caregivers that speak another language and the services you offer. The second part are warm and hot leads. These are families that reach out to us directly, they are non medical (iadl) and private pay. Meaning it's not a cold call. These are families looking for care to start now or within the next 30 days."

Home Health Care Agency Owners & Entrepreneurs · Maria Black (operator) · ↑11 reactions ·

<https://www.facebook.com/groups/1154838852454654/posts/162114221249098...>

"Due to the economy the prices are rising.....overhead cost...I would google price increase letters to client's."

Homecare for newbies · Nora Chineth a year ago (operator) · ↑3 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/904770625149183>

"If you're needing clients private and insurance paid??? Contact me.. our significant referral source is extremely great indeed Referrals are offered in all states which are accepted or denied by you as the referrals belong to you and not multiple agencies around you... Tired of paying for office space and overhead costs with no clients or revenue Let me help you change that!!"

Residential Care Homes Owners/Operators (operator) · <https://www.facebook.com/groups/732719531430807/posts/1296426678393420>

"Is Residential Assisted Living worth looking into? First time posting, please let me know if there are any mistakes in the post. In the process of looking into getting into a real estate deal, I came across an opportunity while doing some research. The business is definitely complex but the overview is: *Buy a house (preferable 1 story)* Remodel and furnish the home to meet local and state zoning rules *Get Assisted Living license* Profit? I see the national average and prices in my area exceed 5k+/month per resident. Plus, if the business doesn't work out, I would still have a house I can rent out or possibly lease to someone else that wants to venture into the residential assisted living business. Is this too good to be true? I feel like I'm missing a major drawback here. If you have any experience in this field, I would love to hear about your experience! How has it worked out for you?"

r/realestateinvesting (pre-launch) · ↑31 upvotes · https://www.reddit.com/r/realestateinvesting/comments/uujosm/is_reside...

I'M NOT LANDING ANY LEADS / CLIENTS — WHAT AM I DOING WRONG? — 8 buyer documents

"I am still not landing any leads for my Private Home Care Provider Agency. I have given out my business cards, brochures, sent emails, and gone to outreach community functions, but I haven't received even one call. What am I doing wrong?"

HOME CARE BUSINESS OWNERS (operator) · ↑21 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

"Tips on how to find clients? Will be opening my non-medical home care business in a month and will start with Facebook advertising and targeted google ads in my area. Our prices are about 15-20% cheaper than our competitors and we do plan on being aggressive with caregiver pay as well. Any tips or ideas on how to get clients? Live in a small town with 100K people and largest metro city is 1 hour away. Thanks!"

r/HomeCareOwners (operator) · ↑1 upvotes · https://www.reddit.com/r/HomeCareOwners/comments/1i6q4vl/tips_on_how_t...

"Hello everyone, I'm a new home care agency owner and would really appreciate your advice and support. I recently started my agency and I'm looking for guidance on how to grow and find clients. For those who have experience in this field: • Can a non-medical home care agency partner or tie up with Medicare, In-Home Supportive Services (IHSS), home health, or hospice agencies for referrals? • What are the best ways to get clients when starting out? • Do you recommend working with hospitals, rehabilitation centers, assisted living communities, or case managers? • Are there insurance companies or long-term care plans that contract with small agencies? I would really appreciate any advice, tips, or referrals from those who have successfully grown their agencies. Thank you so much for supporting new providers in this field."

HOME CARE BUSINESS OWNERS (operator) · <https://www.facebook.com/groups/2156388261379184/posts/276366040398529...>

"Any tips on getting my first client? I did in person marketing and networking, haven't had any success yet"

HOME CARE BUSINESS OWNERS (operator) · <https://www.facebook.com/groups/2156388261379184/posts/284946983873768...>

HOW DO I GET CLIENTS AS AN INDEPENDENT PLACEMENT / TRANSITION SPECIALIST? — 8 buyer documents

"In my view: My CAPS certification (Certified Aging in Place Specialist) has given me credibility & a significant boost to opportunity when combined with other skills, experience & networking. (I also have a Real Estate license.) Opportunities have appeared via - community events & actions & getting to know people. - educating older adults on fall prevention & related topics. - Associations with professional organizations, social media content creation - speaking engagements about aging topics Having other senior sector pros on the team - an OT, move manager, tech integrator, contractor, etc. can create mutual Referral fees . But sometimes their fee for service (OT consultation for home assessment, for example) eats into profit More about CAPS"

Aging in Place Network · Eve Hill a year ago (operator) · ↑2 reactions · <https://www.facebook.com/groups/aginginplacenetw...>

"Hi been an SRES specialist for years, volunteering thru my church to go every Sunday to visit and pray with the residents, founder of Katie's Ark, a non-profit that brings stuffed animals to senior homes. I love working with seniors and bringing solutions. I am patient, with a good sense of humor and always striving for a win win for all parties. Oakland and Macomb County, Michigan. Thank you."

Real Estate Agent Referral Network & Marketing Tips · Anamaria Tet (operator) · ↑2 reactions · <https://www.facebook.com/groups/therealestateagentreferralnetwork/post...>

"Thank you for the shout out Andy:) Yes, Faith my business bridgingseniors.com works with The Senior Transitions Group and we help senior home owners transition into long term living solutions like yours. No cost to you whatsoever and nothing up front for the senior. Clearly we need to get coffee!"

Senior Placement Network · Tess Keim to Andy Bauman's comment (operator) · ↑1 reactions · <https://www.facebook.com/groups/2447813518868667/posts/439960940368905...>

"I'm an SRES broker in Oregon, I provide services to all of Bend, Redmond, Prineville, Madras, La Pine, and Sunriver. I also specialize in probate sales and work with local hospice programs and attorneys for probates, estate planning, downsizing, etc. happy to help any clients!"

Real Estate Agent Referral Network & Marketing Tips · Catherine Emert (operator) · ↑1 reactions · <https://www.facebook.com/groups/therealestateagentreferralnetwork/post...>

Tab 5 — Frustrations

Themes, measured (buyer documents; operators / pre-launch): Leads & referral sources that wasted money or ghosted 29 (27/2) · Red tape / compliance / working capital 29 (25/4) · ABA: insurance won't credential me, authorizations, the waitlist I can't serve 27 (26/1) · Placement agents take a month's rent / commission and own the family 24 (21/3) · Referral sites sell the same lead to five agencies and still charge 13 (13/0) · Caregiver turnover / no-show / quit (the leaky bucket) 12 (12/0) · Medicaid rates, private pay, slow pay, wages, nickel-and-dime 10 (7/3) · No leads, no referrals, hospitals and rehabs say no 9 (8/1) · Quotas, tour metrics and 'sign today' pressure from corporate 4 (4/0)

LEADS & REFERRAL SOURCES THAT WASTED MONEY OR GHOSTED — 29 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"Hello all! What are your top 2 referral sources? Is it google? referral partnerships? Facilities? We are trying to find what works and I know it's different for everyone. We have wasted time and money with A Place for Mom. Learned that lesson early"

Homecare for newbies (operator) · ↑11 reactions · <https://www.facebook.com/groups/706524484973799/posts/976062864686625>

"Wasted my time and money. I wish someone had told me not to use them. They send you dead end leads"

Private Pay Clients looking for Affordable Home Care Services. · Annie Thompson (operator) · ↑8 reactions ·

<https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

RED TAPE / COMPLIANCE / WORKING CAPITAL — 29 buyer documents

"Respectfully... don't quit LEVEL UP. You're not behind, you're just operating without the structure your next level requires. Fire who needs to go, tighten compliance, and stop tolerating clients and staff that don't respect your business. Everybody can't come with you to the next level and that includes problematic clients. You didn't come this far to fold. This is your reset season. I'm free to chat if you ever need some encouragement! I'm praying for you."

HOME CARE BUSINESS OWNERS · Niecy ThaCoach (operator) · 123 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/279719584063175...>

"ABA Business Owners - This is a Liability Nightmare Someone posted on r/ABA about a clinic operating without running water for two days. Management knew about it before opening Monday but kept doors open anyway. Before you think "that's not my problem" - let me break down why this should terrify every ABA business owner: The Legal Exposure: Health code violations: Most states require running water for any facility serving children. Penalties range from fines to immediate closure orders Insurance fraud: Billing Medicaid/insurance for services in non-compliant facilities = fraud investigation territory Liability: If a child gets sick, you're looking at negligence claims. Your insurance may not cover willful violations OSHA violations: Staff working without basic sanitation = workplace safety violation The Business Impact: According to CentralReach's 2025 data, turnover in ABA already runs 77-103% annually depending on agency size. You know what makes it worse? Staff watching leadership prioritize billables over basic safety. Your best BCBA's leave. Your RBTs leave. Word spreads. Recruiting gets harder. Your Glassdoor rating tanks. The Revenue Impact You're Not Calculating: One health department shutdown: 5-30 days closed while you remediate Legal fees defending against negligence claims: \$50K-\$500K+ Insurance premium increases after violations: 30-200% Contract terminations from funders after compliance issues: catastrophic Reputation damage in your market: permanent Versus closing for ONE DAY to fix plumbing. Here's Your Protocol: If you have a facilities emergency (plumbing, HVAC, electrical, etc.): 1. Close immediately - notify families, offer makeup sessions 2. Document everything - contractor quotes, repair timeline, photos 3. Communicate proactively with staff and families 4. Don't reopen until fully compliant - get inspections if needed 5. Review your emergency protocols - do you even have them? The BCBA's you employ have ethical obligations to report unsafe conditions. If they report you to state licensing, health departments, or insurance companies, that's not disloyalty - that's their professional obligation under the BACB Ethics Code. The smart business move is making sure they never have to. What emergency shutdown protocols do you have documented? How do you communicate facilities issues to staff and families?"

Non-BCBA Owners ABA Business Collaborative · Karen Chung (operator) · 13 reactions ·
<https://www.facebook.com/groups/1081018657046251/posts/131382049043273...>

"Job Title: Office Manager – Nurse Staffing Agency Location: FFive Star Professional Home Care Staffing Agency Jacksonville Florida Job Type: Full-Time Reports to: Director of Operations / CEO Position Overview: We are seeking a highly organized and proactive Office Manager to oversee daily operations at our Nurse Staffing Agency. The ideal candidate will play a critical role in supporting our administrative team, healthcare professionals, and clients by ensuring the smooth and efficient functioning of the office. This role requires strong organizational, communication, and problem-solving skills, along with the ability to thrive in a fast-paced healthcare environment. Key Responsibilities: Oversee and coordinate day-to-day administrative operations of the office Maintain organized records for staffing assignments, employee documentation, and compliance records Manage office supplies, equipment, and vendor relationships Maintain confidentiality and data security in line with HIPAA and agency policies Staffing Support: Assist in scheduling and coordinating shifts for nurses and healthcare staff Communicate with health care vendors regarding staffing needs and updates Ensure timely follow-up with staff to confirm availability and shift changes Maintain and update the staffing software or scheduling platforms Human Resources Coordination: Support onboarding of new clinical and non-clinical staff, including credentialing and background checks Track expiration dates for licenses, certifications, and training requirements Ensure up-to-date employee files and compliance with regulatory standards Customer Service: Serve as the first point of contact for clients, nurses, and administrative inquiries Foster positive relationships with partner facilities and nursing staff Resolve issues and escalate problems as needed in a timely and professional manner Financial and Reporting: Support payroll preparation by collecting and verifying timecards and shift records Assist with invoicing, billing inquiries, and basic bookkeeping tasks Generate and analyze reports related to staffing, scheduling, and office performance Qualifications: Associate degree in Business Administration, Healthcare Management, or a related field (preferred but not necessary) 2+ years of office management or administrative experience; healthcare or staffing agency experience is highly desirable Strong knowledge of Microsoft Office Suite, scheduling software, and databases Excellent organizational, communication, and interpersonal skills Ability to multitask and prioritize effectively in a deadline-driven environment Familiarity with healthcare staffing regulations and credentialing practices is a plus Work Environment: Office-based Fast-paced environment with frequent communication with nurses and healthcare clients Salary & Benefits: Competitive salary based on experience Opportunities for professional development Please email resume to Fivestarthomecarejax@gmail.com or call 904-673-8416 to schedule an interview"

HOME CARE BUSINESS OWNERS · Melonie Miller (operator) · ↑2 reactions · <https://www.facebook.com/groups/2156388261379184/posts/252654748436325...>

ABA: INSURANCE WON'T CREDENTIAL ME, AUTHORIZATIONS, THE WAITLIST I CAN'T SERVE — 27 buyer documents

"I started a private practice AMA I'm a BCBA that went fully independent in 2021. I do all the aspects of business and clinical myself - including credentialing, billing, marketing, and provide all clinical services myself. I'm in CA. Ever wanted to go your own way? AMA!"

r/bcba (operator) · ↑66 upvotes · https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions ·

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I'm not in sales, but I'm a department head and know that in my company there are monthly census quotas, sale quotas and coming off the COVID situation lots of companies really cranked the pressure up on Assisted Living Communities back in May when the president declared the Covid state of emergency in the nation was "over". Almost immediately it was an expectation that everything get back to "business as usual" and they doubled/tripled the quotas for move ins. There are high quotas each months if the building is not fully occupied and even if it is fully occupied there is an expectation of having a sizeable wait list with people who have placed a deposit to hold apartments for the future. A lot of people aren't aware that assisted living communities are mostly owned by real estate investment trusts that have a contract with management companies whose names are on the signs of these properties. So the owners' groups who own the property put a lot of pressure on the management companies and at the community level it just compounds with expectations that are intensified. Our executive director and sales director have weekly conference calls with the corporate office where they are grilled about move ins on the books already confirmed, projected move ins with inquiries that have sounded promising and what it will take to get them to sign and put a deposit down, how many inquiries are in the database that haven't come into the community to tour and why and what can we do to get them in to tour and how many leads are in the database to follow up on as well as any move outs and a lot of details on the factors that led to the move out (death, needing skilled care, financial reasons, dissatisfaction). If you ever make an inquiry at a community that is part of a chain, even a smaller company, you will be logged into the system and the sales team will contact you every month until you formally demand to be taken off the list or until you say your relative has passed away. If you have come into the building for a tour, there will be even more contact until you say you have chosen another community (they will ask why), that your relative has passed or if you formally request no further contact. Edited to add: please also be aware that the deal that has been made is not permanent. The rate increases every January- this is industry wide. Depending on how much of a deal you get now, you may experience a higher percentage increase the next year. Everyone's rent goes up. It may be 3% it may be 12%. 6/8% seems to be the average in our area this year. Also- most communities give you a room rate and that is one thing, but care charges are a completely different thing. If your father begins requiring help with anything or if his needs of what is already being done increases the care charges will increase. If there is a rapid cognitive decline and needs change with dressing, bathing, continence, cueing to meals and activities, cueing to get up in the morning or not being oriented to place and time there will be a need for him to move to a memory care setting and that is generally significantly more expensive. There are also situations in my community where care needs are so personalized that a family hires a third party caregiver as well to be with their loved one and this could be for a variety of reasons. I know it can all be overwhelming and a lot of pressure to move in, feel free to ask all of the questions- staffing ratios, when shift changes are, percentage ratios of staff turnover, resident turnover, other charges that could arise for anything like transportation to doctor appointments to the community providing continence supplies. Just lots of information in a short time. Take your time absorbing the info and ask all the questions you need. Verbalize that you don't like the feeling of being pressured. You can put that in a google review as well (there is a huge push for people just moving in to do online reviews for a lot of these companies). Hoping this info is helpful and wishing you luck and know that so many of the people who work in the communities, like myself, feel so privileged to be trusted to care for people's loved ones each day, we are passionate about what we do, we love sharing time getting to know each resident and work so hard every day to help each person have a wonderful day!"

r/AssistedLiving (operator) · ↑5 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

PLACEMENT AGENTS TAKE A MONTH'S RENT / COMMISSION AND OWN THE FAMILY — 24 buyer documents

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"Anyone know how to digital market for senior care?. I've become way too dependent on the placement agents that have been bringing me clients. Im looking to see if anyone has tried digital marketing or run ads? Update: I started running ads and even though I have not gotten a single client i got a ton of caregivers and everyone in our community started to recognize us. Because of this by product of running these ads we have started to become known. Win? Sure, I'll take it. I am posting my experience with digital marketing on my Skool platform. The first 7 days are free and then it a monthly subscription to be apart of my community. ["

r/RunAHomeCareAgency (operator) · ↑5 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1ffa065/anyone_kn...

"Personally, I have no issue with referral fees. I respect it even more if the referral fee is quite high, that the person who is charging allows the person who's paying it to complete the payment within 30 days. Otherwise, I have no qualms!"

Senior Placement Network · Celina Whitney a year ago (operator) · ↑5 reactions · <https://www.facebook.com/groups/2447813518868667/posts/402589742772692...>

REFERRAL SITES SELL THE SAME LEAD TO FIVE AGENCIES AND STILL CHARGE — 13 buyer documents

"Not worth it from what I have heard. They sell same leads to other companies. Some are really old. You're better off going into hospitals and nursing homes, handing out business cards to social workers or nurses on each floor. Also go to community health clinics"

Private Pay Clients looking for Affordable Home Care Services. · Eva Horvath (operator) · ↑5 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

"I'm sorry you experienced this. Unfortunately, a lot of lead companies sell the same lead to multiple home care agencies, so you end up paying to compete with several companies for the same family. I wouldn't let this discourage you, especially since you've already proven you can find a client on your own. At only 4 months in, I would focus less on buying leads and more on building your own referral pipeline. Start developing relationships with senior communities, rehab/SNF social workers, elder law attorneys, geriatric care managers, hospice companies, physicians, churches, senior centers, and other professionals who regularly encounter families needing care. Also make sure your Google Business Profile and social media clearly show what makes your agency different and exactly who you serve. Private-pay marketing takes consistency and follow-up, it's usually not one visit, one flyer, or one phone call. You want referral sources to start recognizing your name and trusting you enough to recommend you. If you'd like, I help newer home care owners build a step-by-step marketing and referral strategy so they know exactly who to target, what to say, and how to follow up without wasting money on shared leads. Feel free to message me."

Home Care Business Owners Tools · Tina Hemmings (operator) · ↑2 reactions · <https://www.facebook.com/groups/homecareownersunite/posts/107527837501...>

"A place for mom charges quite a lot. Contact social services in every hospital that you can. They keep a list of licensed care homes that APS has cases on and refers to the displaced residents. I was in a shutdown of an unlicensed care home and was sent to the hospital, is how I know. I noticed there was several homes on facebook, is there a way to network them and if someone's has a full house refer to someone that needs clients, in other words, help each other."

Private Pay Clients looking for Affordable Home Care Services. · Patti Wyckoff (operator) · ↑2 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

CAREGIVER TURNOVER / NO-SHOW / QUIT (THE LEAKY BUCKET) — 12 buyer documents

"I am concerned about the high staff turnover rate in childcare. Members of the management in most to all centre's seem perplexed about the reasons behind this trend. The practice of maximising room occupancy to meet required ratios, as well as enrolling more children without the necessary staff resources, is resulting in situations where children with potential undiagnosed needs are not receiving the support and attention they require. This not only compromises the quality of care provided but also puts undue pressure on our dedicated educators. It is concerning to witness talented educators leave in search of better working environments where their contributions are valued and supported. The lack of a strong duty of care towards both our employees and the children in centre's is evident and requires immediate attention. I strongly urge all to consider the well-being and professional development of our employees, as they are the cornerstone of our childcare profession. By fostering a nurturing and supportive work environment, we can not only retain valuable staff members but also provide a higher standard of care for the children under our supervision"

ECE Community · Anonymous participant (operator) · ↑50 reactions · <https://www.facebook.com/groups/248675758568554/posts/6929343677168362>

"Don't forget YOU are not the manager, you are the owner/operator. So no.... actually you don't show up when caregivers call out, you're not licensed to do their work, the manager/administrator gets called and fills the shift with another caregiver or does the job themselves. You won't even know about it, because they are handling the day to day."

youtube · @RALAcademy · Residential Assisted Living Is The Number One Real Estate In (operator) · ↑19 likes · <https://www.youtube.com/watch?v=Q9bl7hQL0w8&lc=UgzvNileUuUMvGqtlyh...>

"I'm not in sales, but I'm a department head and know that in my company there are monthly census quotas, sale quotas and coming off the COVID situation lots of companies really cranked the pressure up on Assisted Living Communities back in May when the president declared the Covid state of emergency in the nation was "over". Almost immediately it was an expectation that everything get back to "business as usual" and they doubled/tripled the quotas for move ins. There are high quotas each months if the building is not fully occupied and even if it is fully occupied there is an expectation of having a sizeable wait list with people who have placed a deposit to hold apartments for the future. A lot of people aren't aware that assisted living communities are mostly owned by real estate investment trusts that have a contract with management companies whose names are on the signs of these properties. So the owners' groups who own the property put a lot of pressure on the management companies and at the community level it just compounds with expectations that are intensified. Our executive director and sales director have weekly conference calls with the corporate office where they are grilled about move ins on the books already confirmed, projected move ins with inquiries that have sounded promising and what it will take to get them to sign and put a deposit down, how many inquiries are in the database that haven't come into the community to tour and why and what can we do to get them in to tour and how many leads are in the database to follow up on as well as any move outs and a lot of details on the factors that led to the move out (death, needing skilled care, financial reasons, dissatisfaction). If you ever make an inquiry at a community that is part of a chain, even a smaller company, you will be logged into the system and the sales team will contact you every month until you formally demand to be taken off the list or until you say your relative has passed away. If you have come into the building for a tour, there will be even more contact until you say you have chosen another community (they will ask why), that your relative has passed or if you formally request no further contact. Edited to add: please also be aware that the deal that has been made is not permanent. The rate increases every January- this is industry wide. Depending on how much of a deal you get now, you may experience a higher percentage increase the next year. Everyone's rent goes up. It may be 3% it may be 12%. 6/8% seems to be the average in our area this year. Also- most communities give you a room rate and that is one thing, but care charges are a completely different thing. If your father begins requiring help with anything or if his needs of what is already being done increases the care charges will increase. If there is a rapid cognitive decline and needs change with dressing, bathing, continence, cueing to meals and activities, cueing to get up in the morning or not being oriented to place and time there will be a need for him to move to a memory care setting and that is generally significantly more expensive. There are also situations in my community where care needs are so personalized that a family hires a third party caregiver as well to be with their loved one and this could be for a variety of reasons. I know it can all be overwhelming and a lot of pressure to move in, feel free to ask all of the questions- staffing ratios, when shift changes are, percentage ratios of staff turnover, resident turnover, other charges that could arise for anything like transportation to doctor appointments to the community providing continence supplies. Just lots of information in a short time. Take your time absorbing the info and ask all the questions you need. Verbalize that you don't like the feeling of being pressured. You can put that in a google review as well (there is a huge push for people just moving in to do online reviews for a lot of these companies). Hoping this info is helpful and wishing you luck and know that so many of the people who work in the communities, like myself, feel so privileged to be trusted to care for people's loved ones each day, we are passionate about what we do, we love sharing time getting to know each resident and work so hard every day to help each person have a wonderful day!"

r/AssistedLiving (operator) · ↑5 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

MEDICAID RATES, PRIVATE PAY, SLOW PAY, WAGES, NICKEL-AND-DIME — 10 buyer documents

"I trained in major medical hospitals inpatient and outpatient hospital clinics; so I had absolutely no experience with insurance companies and their Drama Rama. When I opened a private practice, I kept my medical staff privileges and also applied for medical staff privileges at other hospitals. My referrals for my private practice all came from my medical colleagues. I have never been paneled with insurance companies for my private practice. Private Pay Only. all master level therapists and doctoral and postdoctoral psychologists in my territory only accept Private Pay. ~~thank you for the video Dr. Marie"

youtube · @DrShawnaFreshwater · Insurance vs Private Pay Private Practice | My Experience wi (operator) · ↑5 likes · <https://www.youtube.com/watch?v=zvtl029wKTU&lc=Ugw0mTsfjhjMx6LEwIF...>

"I don't know why they said it was best to start private pay maybe because you get paid instantly, but you can also sign up for Medicaid and Medicaid pays weekly"

Home Health Care Agency Owners & Entrepreneurs · Real Is (operator) · ↑4 reactions · <https://www.facebook.com/groups/1154838852454654/posts/153810140746172...>

"Rcfe Owners Network Group · Anonymous participant · 31 May · Hi all! I'm looking for some advice from experienced RCFE owners/operators. I'm in the process of building out my facility in Los Angeles and one of my biggest concerns is maintaining occupancy and consistently filling beds. For those of you who have been successful, what have been your most effective referral sources? I'm also very interested in learning more about the CalAIM program and hearing real-world experiences from operators who participate. For those currently accepting CalAIM referrals: Has it been a reliable source of residents? How long did the onboarding process take? What has your reimbursement experience been like? Any surprises, challenges, or lessons learned? Would you do it again if you were starting over? I'm trying to be proactive and build a sustainable occupancy strategy from the beginning, so I'd appreciate any advice, lessons learned, or things you wish you knew when you first started. Thank you in advance!"

Rcfe Owners Network Group | Hi all! I'm looking for some advice from experienced RCFE owners/operators · 4.6K members · Join Group (operator) · ↑2 reactions · <https://www.facebook.com/groups/352285274494478/posts/914098108313189>

NO LEADS, NO REFERRALS, HOSPITALS AND REHABS SAY NO — 9 buyer documents

"Struggling to get referrals for ABA company?. Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

Non-BCBA Owners ABA Business Collabrative · Anonymous participant (operator) · ↑3 reactions · <https://www.facebook.com/groups/1081018657046251/posts/143052813209530...>

"You can grab my ebook and get all the vendors that actually send REAL clients and private-pay leads—for a fraction of what those companies charge. I've already done the trial and error so you don't waste your money like I did. Stop paying high referral fees for leads that don't convert! I literally had to pay thousands for coaching and courses to get me clients and get through all the gate keeping !"

Private Pay Clients looking for Affordable Home Care Services. · Toya TK (operator) · ↑2 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

"I have been running my domiciliary care agency for 10 months now and I wanted to reach out to this community for some honest advice. I have been struggling to get clients and have tried quite a few things, Facebook ads, Lottie, Homecare.co.uk and even Yell. I have also reached out to the local commissioning team but unfortunately haven't had any response. I know there are some really experienced people in here and I would love to hear how you grew your client base, especially in the early days. What worked for you? What would you do differently? Any advice at all is massively appreciated!"

Care Managers Network UK - CQC Registered Professionals (operator) · <https://www.facebook.com/groups/1971829479945179/posts/250346596678152...>

QUOTAS, TOUR METRICS AND 'SIGN TODAY' PRESSURE FROM CORPORATE — 4 buyer documents

"I'm not in sales, but I'm a department head and know that in my company there are monthly census quotas, sale quotas and coming off the COVID situation lots of companies really cranked the pressure up on Assisted Living Communities back in May when the president declared the Covid state of emergency in the nation was "over". Almost immediately it was an expectation that everything get back to "business as usual" and they doubled/tripled the quotas for move ins. There are high quotas each months if the building is not fully occupied and even if it is fully occupied there is an expectation of having a sizeable wait list with people who have placed a deposit to hold apartments for the future. A lot of people aren't aware that assisted living communities are mostly owned by real estate investment trusts that have a contract with management companies whose names are on the signs of these properties. So the owners' groups who own the property put a lot of pressure on the management companies and at the community level it just compounds with expectations that are intensified. Our executive director and sales director have weekly conference calls with the corporate office where they are grilled about move ins on the books already confirmed, projected move ins with inquiries that have sounded promising and what it will take to get them to sign and put a deposit down, how many inquiries are in the database that haven't come into the community to tour and why and what can we do to get them in to tour and how many leads are in the database to follow up on as well as any move outs and a lot of details on the factors that led to the move out (death, needing skilled care, financial reasons, dissatisfaction). If you ever make an inquiry at a community that is part of a chain, even a smaller company, you will be logged into the system and the sales team will contact you every month until you formally demand to be taken off the list or until you say your relative has passed away. If you have come into the building for a tour, there will be even more contact until you say you have chosen another community (they will ask why), that your relative has passed or if you formally request no further contact. Edited to add: please also be aware that the deal that has been made is not permanent. The rate increases every January- this is industry wide. Depending on how much of a deal you get now, you may experience a higher percentage increase the next year. Everyone's rent goes up. It may be 3% it may be 12%. 6/8% seems to be the average in our area this year. Also- most communities give you a room rate and that is one thing, but care charges are a completely different thing. If your father begins requiring help with anything or if his needs of what is already being done increases the care charges will increase. If there is a rapid cognitive decline and needs change with dressing, bathing, continence, cueing to meals and activities, cueing to get up in the morning or not being oriented to place and time there will be a need for him to move to a memory care setting and that is generally significantly more expensive. There are also situations in my community where care needs are so personalized that a family hires a third party caregiver as well to be with their loved one and this could be for a variety of reasons. I know it can all be overwhelming and a lot of pressure to move in, feel free to ask all of the questions- staffing ratios, when shift changes are, percentage ratios of staff turnover, resident turnover, other charges that could arise for anything like transportation to doctor appointments to the community providing continence supplies. Just lots of information in a short time. Take your time absorbing the info and ask all the questions you need. Verbalize that you don't like the feeling of being pressured. You can put that in a google review as well (there is a huge push for people just moving in to do online reviews for a lot of these companies). Hoping this info is helpful and wishing you luck and know that so many of the people who work in the communities, like myself, feel so privileged to be trusted to care for people's loved ones each day, we are passionate about what we do, we love sharing time getting to know each resident and work so hard every day to help each person have a wonderful day!"

r/AssistedLiving (operator) · ↑5 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

"Former Marketing Director for Assisted Living for 6 years. I'll start off by saying, and I'm sorry if this is harsh but this is what's most likely going on. Yes, there are monthly quotas. Mine was at least three move ins a month and that was just the bare minimum to keep your job secured. Corporate liked to have weekly meetings (twice a week if my census was low) to put unnecessary pressure on the sales team to "make families sign the dotted line" the first time they come tour. Giving you time to come look more than once was not acceptable to them. I can't tell you how many times my job was threatened because a family came by three times and STILL hadn't signed on. They didn't understand that the family NEEDED TIME as they were making the biggest decision of their lives. Corporate looked at ME like I wasn't doing my job well enough because each time they walked out, there was still not a check in my hand. Unfortunately, this is a money hungry industry and it can feel so disgusting at times. I left Marketing because I was tired of feeling like I was on "the dark side" of Assisted Living. Praying residents move in and that they at last at least 30 days so I get my bonus and my census wouldn't drop because if it did, I had the big bosses to answer to as to why I don't already have someone else to replace them. It's a "heads in bed" mentality. (I told you this would be harsh and I'm sorry) You would go from hero to a zero as soon as the previous month ended. I could have 6 move in for one month and on the first of the next, here came all of the pressure. I will say, I was not a pushy Marketer and maybe that's why I had to leave that position. I hated the tongue lashings. I hated my job being threatened. I hate that people were treated like they had dollar signs above their heads when they walked into my community. I was subject to video conferences to discuss each lead in my system and basically instructed to make people feel like complete s*** for not moving their loved one in. Basically make you, the daughter, feel like you are the absolute worst child ever for keeping your dad at home one day longer. I couldn't do it. I just couldn't. I asked my Regional one time (when I was being threatened about a family not signing that very day) if she wanted me to hold them hostage in my office until they agreed to write me a check. Like, seriously?! My advice is take your time. Tour as many times as you need to to ensure you are making the right decision for YOUR father when YOU are ready. If you're not ready, don't sign up just because they make you feel like there is one room left. Most of the time, that's a load of crap. I've even had to put fake names on doors before (per corporate instruction) to make it seem like we had more occupied rooms than we actually did. Disgusting and deceitful if you ask me. Anyways, off of my soap box."

r/AssistedLiving (operator) · ↑3 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

"I own an assisted living and we do not have a sales department.....there is no need for one. Plus these are difficult decisions for a family therefore we do not allow anyone to push families into any kind of decision.."

r/AssistedLiving (operator) · ↑3 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

Tab 6 — Previous Solutions

Themes, measured (buyer documents; operators / pre-launch): Referral sources: hospitals, discharge planners, SNF, rehabs, funeral homes 152 (136/16) · Google Ads / Facebook ads / SEO / website / brochures 128 (116/12) · Placement agents, A Place for Mom, referral agencies 89 (86/3) · Franchise / consultant / scheduling software 69 (57/12) · Insurance panels, pediatrician referrals, school contracts (ABA) 55 (44/11) · Lead directories (Care.com, A Place for Mom) 49 (45/4) · A Place for Mom / Caring.com / Care.com / CareInHomes: what it cost, what it returned 49 (46/3) · Software, EMR, pendants, AI (PCC, EVV, AIRA) 23 (23/0) · RAL courses, consultants, administrator certification, Facebook groups 22 (17/5) · Networking / BNI / chamber 13 (13/0) · Pay-per-lead vendors and lead-gen companies 11 (11/0)

REFERRAL SOURCES: HOSPITALS, DISCHARGE PLANNERS, SNF, REHABS, FUNERAL

HOMES — 152 buyer documents

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life. Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you. Case Manager: Good morning. It's nice to meet you. Administrator: Thank you. Before I leave, may I ask you one quick question? Case Manager: Of course. Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations. Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital. Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises. Case Manager: I like that. Thank you for taking the time to stop by. Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.) Case Manager: Thank you. I'll definitely keep this for future reference. Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community. Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit. Case Manager: Thank you so much. That's very thoughtful of

you. Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions ·

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources: Social service agencies Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

GOOGLE ADS / FACEBOOK ADS / SEO / WEBSITE / BROCHURES — 128 buyer documents

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life. Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you. Case Manager: Good morning. It's nice to meet you. Administrator: Thank you. Before I leave, may I ask you one quick question? Case Manager: Of course. Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations. Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital. Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises. Case Manager: I like that. Thank you for taking the time to stop by. Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.) Case Manager: Thank you. I'll definitely keep this for future reference. Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community. Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit. Case Manager: Thank you so much. That's very thoughtful of

you. Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"Hey everyone! Can anyone recommend a really good marketer who knows the home care industry? I'm looking for someone who can help with Google SEO, Google Ads, getting my agency to rank higher on Google, improving my Google Business Profile, and bringing in more qualified leads. If you've personally used someone and had a good experience, I'd love their contact information. Thanks in advance!"

HOME CARE BUSINESS OWNERS (operator) · ↑22 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/291140860921047...>

"I am still not landing any leads for my Private Home Care Provider Agency. I have given out my business cards, brochures, sent emails, and gone to outreach community functions, but I haven't received even one call. What am I doing wrong?"

HOME CARE BUSINESS OWNERS (operator) · ↑21 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

PLACEMENT AGENTS, A PLACE FOR MOM, REFERRAL AGENCIES — 89 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads. Initially, it was to be a few hrs a day. I drove over 30 minutes to complete the assesment, only to be turned away. I sat with our now client and her cousin, gave her some safety tips, and left our information with her. I then followed up with the cousin who initiated the contact via email, thanking her for the opportunity and letting her know that we are only a call away. Fast forward, one month later, on Oct 30, i received two back to back calls from the same cousin telling me that our now client was in the ED and could not go home unless there was a home care agency that would take her. Oherwise, she'd be sent to a TCU or nursing home. Cloent said she I agreed to the case, and confirmed a next day asesment. The cousin stated the client was only interested in a few hrs a day. Upon completing my assessment, I informed the client that she required around the clock. She wasn't happy but she signed up. All that to say, yes APFM is high risk, however as many say, the buy-in is in the followup."

Homecare for newbies · Dumah Darling (operator) · ↑13 reactions · <https://www.facebook.com/groups/706524484973799/posts/1130570192569224>

FRANCHISE / CONSULTANT / SCHEDULING SOFTWARE — 69 buyer documents

"Hi everyone, I'm in the final stages of finalizing my application for my home care business. I'm starting out with private pay clients only. I wanted to ask what systems everyone uses to keep track of caregiver information (profiles, credentials, background checks, availability, etc.). Do you use a specific caregiver management or scheduling software, or do you keep records another way? I'd really appreciate any examples or recommendations of what works well when starting out. Thank you!"

Home Health Care Agency Owners & Entrepreneurs · Chrystal Bowen (operator) · ↑27 reactions · <https://www.facebook.com/groups/1154838852454654/posts/158909693569550...>

"My husband and I own 2 adult family homes, similar to this. We make 80k a month from these two businesses. We bought two residential homes turned them into senior/mental health facilities and we have living caregivers aids there as well. We house 12 adults who are vulnerable adults/DDA. It's a rewarding career and very lucrative. Best thing I could have done, I hated working 9-5 in corporate America."

youtube · @Nawal0408 · The 2-Bedroom Rental That Makes \$30,000/Month! (Assisted Liv (operator) · ↑14 likes · https://www.youtube.com/watch?v=e_hF_qArFXI&lc=UgwlpUMEBaQL8VKEI41...

"Scheduling software for 24/7 home care. I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p). Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑8 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

INSURANCE PANELS, PEDIATRICIAN REFERRALS, SCHOOL CONTRACTS (ABA) — 55 buyer documents

"I started a private practice AMA I'm a BCBA that went fully independent in 2021. I do all the aspects of business and clinical myself - including credentialing, billing, marketing, and provide all clinical services myself. I'm in CA. Ever wanted to go your own way? AMA!"

r/bcba (operator) · ↑66 upvotes · https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions · <https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"Hi everyone! I'm in the early stages of opening my first ABA clinic and would love to learn from those who've already been through it. I know costs vary widely, but I'm curious: Roughly how much did you spend to get your clinic up and running? What ended up being the biggest expense during the first 1–2 months? Any insight or lessons learned would be greatly appreciated. Thank you in advance!"

BCBA Resources · ProficientPapaya8472 (operator) · ↑12 reactions ·
<https://www.facebook.com/groups/bcbaresources/posts/1237058455216954>

LEAD DIRECTORIES (CARE.COM, A PLACE FOR MOM) — 49 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"Has anyone ever had a A Place for Mom actually reach out to them? They contacted me today, signed me up, waived all my fees, and even set me up in the portal to start receiving private pay clients. I usually hear people say 'A Place for Mom ain't it,' so now I'm curious... how's it going for you?"

Homecare for newbies · Anonymous participant (operator) · ↑13 reactions ·
<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

"Anyone received a call from a place for mom? What are your thoughts they said they found my business on the licensing web and want to see if I need clients"

HOME CARE BUSINESS OWNERS (operator) · ↑13 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/273192195049247...>

A PLACE FOR MOM / CARING.COM / CARE.COM / CAREINHOMES: WHAT IT COST, WHAT IT RETURNED — 49 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads. Initially, it was to be a few hrs a day. I drove over 30 minutes to complete the assesment, only to be turned away. I sat with our now client and her cousin, gave her some safety tips, and left our information with her. I then followed up with the cousin who initiated the contact via email, thanking her for the opportunity and letting her know that we are only a call away. Fast forward, one month later, on Oct 30, i received two back to back calls from the same cousin telling me that our now client was in the ED and could not go home unless there was a home care agency that would take her. Oherwise, she'd be sent to a TCU or nursing home. Cloent said she I agreed to the case, and confirmed a next day asessment. The cousin stated the client was only interested in a few hrs a day. Upon completing my assessment, I informed the client that she required around the clock. She wasn't happy but she signed up. All that to say, yes APFM is high risk, however as many say, the buy-in is in the followup."

Homecare for newbies · Dumah Darling (operator) · ↑13 reactions ·
<https://www.facebook.com/groups/706524484973799/posts/1130570192569224>

"Has anyone ever had a A Place for Mom actually reach out to them? They contacted me today, signed me up, waived all my fees, and even set me up in the portal to start receiving private pay clients. I usually hear people say 'A Place for Mom ain't it,' so now I'm curious... how's it going for you?"

Homecare for newbies · Anonymous participant (operator) · ↑13 reactions ·
<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

SOFTWARE, EMR, PENDANTS, AI (PCC, EVV, AIRA) — 23 buyer documents

"Hi everyone, I'm in the final stages of finalizing my application for my home care business. I'm starting out with private pay clients only. I wanted to ask what systems everyone uses to keep track of caregiver information (profiles, credentials, background checks, availability, etc.). Do you use a specific caregiver management or scheduling software, or do you keep records another way? I'd really appreciate any examples or recommendations of what works well when starting out. Thank you!"

Home Health Care Agency Owners & Entrepreneurs · Chrystal Bowen (operator) · ↑27 reactions ·
<https://www.facebook.com/groups/1154838852454654/posts/158909693569550...>

"Scheduling software for 24/7 home care. I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p).* Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) *Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑8 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

"Scheduling software for 24/7 home care I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p).* Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) *Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

RAL COURSES, CONSULTANTS, ADMINISTRATOR CERTIFICATION, FACEBOOK GROUPS — 22 buyer documents

"I got my clients by calling nursing homes and posting in local Facebook groups/ Nextdoor."

HOME CARE BUSINESS OWNERS · Ja Nay (operator) · ↑19 reactions · <https://www.facebook.com/groups/2156388261379184/posts/284819363219864...>

"What are the steps to start a residential care facility in Roseville, CA?. Hi RCFE owners/mgrs, May I please ask for your help and mentorship with getting my first RCFE started in Roseville, CA? I recently purchased a home and will be taking the administrator course soon and then apply for a facility license. I have been unable to find out the following: 1) where to get patients from? 2) how to get paid by the patients? Do patients pay me directly or does insurance pay me? 3) where to hire caretakers from? 4) what type of business or worker comp insurance will I needed? 5) while I am applying for facility license, should I rent out the house with a one year lease, to help with mortgage payments? 6) getting facility licensure may take a while, so can I submit facility license application before I obtain an admistrator license? I am sure there are many more questions I need to ask. Please help me with your expertise and experience. Thank you for your time and support!"

Residential Care Homes Owners/Operators · Van Voong (operator) · ↑6 reactions · <https://www.facebook.com/groups/732719531430807/posts/1399974124705341>

"We have a total of 6 clients so putting it in percentages will not be helpful. I'll give you some hard numbers. Outlay: \$250 on logo creation \$2500 to build website (including photo shoot and video) \$500 on SEO and Google maps setup (currently with 3 reviews) \$600 for design and printing of 500 brochures \$400 to join the local Chamber of Commerce for networking \$0 for Facebook, Nextdoor, passing out brochures, visiting care facilities Conversion: 3 clients from Facebook group post 1 client from Nextdoor post 1 from networking 1 from word of mouth It's important to note that each of the above conversions from free activities was undoubtedly strongly enhanced by a good website and Google presence. Finding high-quality caregivers has not been easy as well. Our website has been particularly helpful with that. For reference, we have been in business for 6 months and are grossing \$20,000 per month now. We have 4 caregivers, not including my partner, myself, and my son, who all work with clients, too."

r/RunAHomeCareAgency (operator) · ↑3 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dj6i94/marketing...>

NETWORKING / BNI / CHAMBER — 13 buyer documents

"A few things I've learned that may help: 1. Be in an area with a large senior population. If there are more home care agencies than seniors, it will be much harder to grow. 2. Consider building relationships with rehabilitation centers. Many patients need home care before they're discharged, so rehab facilities can be a great source of referrals instead of relying only on agencies. 3. Connect with your local Chamber of Commerce and Council on Aging. They often have referral networks and can introduce you to families looking for services. 4. Keep in mind that assisted living communities are also competing for many of the same clients, so it's important to clearly communicate what makes home care the better option for those who want to remain at home."

HOME CARE BUSINESS OWNERS · Jenae Wilson Frazer (operator) · ↑5 reactions · <https://www.facebook.com/groups/2156388261379184/posts/289670824068051...>

"We have a total of 6 clients so putting it in percentages will not be helpful. I'll give you some hard numbers. Outlay: \$250 on logo creation \$2500 to build website (including photo shoot and video) \$500 on SEO and Google maps setup (currently with 3 reviews) \$600 for design and printing of 500 brochures \$400 to join the local Chamber of Commerce for networking \$0 for Facebook, Nextdoor, passing out brochures, visiting care facilities Conversion: 3 clients from Facebook group post 1 client from Nextdoor post 1 from networking 1 from word of mouth It's important to note that each of the above conversions from free activities was undoubtedly strongly enhanced by a good website and Google presence. Finding high-quality caregivers has not been easy as well. Our website has been particularly helpful with that. For reference, we have been in business for 6 months and are grossing \$20,000 per month now. We have 4 caregivers, not including my partner, myself, and my son, who all work with clients, too."

r/RunAHomeCareAgency (operator) · ↑3 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dj6i94/marketing...>

"Try Senior Expos, Veteran Stand Down events, local networking groups. I'm sure your local Chamber of Commerce does. Volunteer at the food pantry and ask if you can provide education and materials. Contact local senior and disabled housing. Offer to play bingo while you talk about your company. FYI - they love toiletries as prizes. (Hard to get those at the food bank.) Your local Area Agency on Aging likely sponsors a Nutrition Site. They are always looking for someone to come in and talk. I was terrified of speaking in front of others. Small groups are a good way to start. You got this!"

HOME CARE BUSINESS OWNERS · PastelCapybara7811 (operator) · ↑2 reactions · <https://www.facebook.com/groups/2156388261379184/posts/286671865701280...>

PAY-PER-LEAD VENDORS AND LEAD-GEN COMPANIES — 11 buyer documents

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"It's hard to make it make sense. You are paying \$60 for a lead that's being shared with 5 or more other agencies at the same time, so your likelihood of closing on any of them is extremely low. A much better approach would be to learn how to generate and nurture your own leads through Google and Meta ads and other lead generation approaches, so the lead is all yours with no competition. My ads typically cost me around \$6 per lead and my close rate is pretty high. Hands down the best decision I've made for my agency. Hope this helps."

HOME CARE BUSINESS OWNERS · Edward Davis (operator) · ↑9 reactions · <https://www.facebook.com/groups/2156388261379184/posts/288229603545506...>

"Another question for my fellow agency owners: Have any of you used a referral source or lead generation company for private pay clients that you found to be effective and reasonably priced? I'm looking for recommendations that don't cost an arm and a leg. I'd love to hear what has worked for you and what you'd avoid."

HOME CARE BUSINESS OWNERS (operator) · ↑7 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/286769206024879...>

Tab 7 — Solution Soundbites (**COPY TAB — hooks**)

Themes, measured (buyer documents; operators / pre-launch): I wish / what I need 82 (71/11) · Attract, don't beg: add value to referral partners 19 (16/3) · What finally worked 13 (13/0) · I'd pay for results, not a retainer 9 (8/1) · If someone would just show me a system that brings clients predictably 6 (6/0) · The key is / predictable (the market's promise) 4 (4/0)

I WISH / WHAT I NEED — 82 buyer documents

"Hey everyone! Can anyone recommend a really good marketer who knows the home care industry? I'm looking for someone who can help with Google SEO, Google Ads, getting my agency to rank higher on Google, improving my Google Business Profile, and bringing in more qualified leads. If you've personally used someone and had a good experience, I'd love their contact information. Thanks in advance!"

HOME CARE BUSINESS OWNERS (operator) · ↑22 reactions · <https://www.facebook.com/groups/2156388261379184/posts/291140860921047...>

"Anyone received a call from a place for mom? What are your thoughts they said they found my business on the licensing web and want to see if I need clients"

HOME CARE BUSINESS OWNERS (operator) · ↑13 reactions · <https://www.facebook.com/groups/2156388261379184/posts/273192195049247...>

"Scheduling software for 24/7 home care. I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p). Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑8 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

"Scheduling software for 24/7 home care I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p).* Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) *Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

ATTRACT, DON'T BEG: ADD VALUE TO REFERRAL PARTNERS — 19 buyer documents

"Visit assisted living communities and schedule in-person meetings with the office manager. Bring a small gesture, like drinks and light snacks, to help make a positive first impression. I may be young, but I had to think old school when it came to figuring out how to reach this population. Sometimes the best marketing is simply getting out there and putting in the footwork. I'm not sure how many counties you currently serve, but expanding your service area can also create more opportunities. Don't be afraid to hand out your flyers at places like Goodwill stores and other community locations where your target audience and their families may visit. If you're trying to grow your business while keeping costs low, go above and beyond by doing the legwork yourself. Building relationships face-to-face can make all the difference. I hope this helps someone!"

HOME CARE BUSINESS OWNERS · Priscilla Stone (operator) · ↑29 reactions · <https://www.facebook.com/groups/2156388261379184/posts/288420692193064...>

"We hit the community hard! At EEKC HomeCare Concierge Services, getting our first private-pay client meant getting out where our people are—building relationships, sharing our story, and showing families the heart and quality behind our care. Personal connections, trust, and visibility made all the difference. Sometimes it's not just about marketing—it's about being present, listening, and serving. That's how we started, and it's still our approach today."

Private Pay Clients looking for Affordable Home Care Services. · EEKC HomeCare Concierge Services (operator) · ↑18 reactions · <https://www.facebook.com/groups/2018920001893909/posts/239152230796700...>

"A few things I've learned that may help: 1. Be in an area with a large senior population. If there are more home care agencies than seniors, it will be much harder to grow. 2. Consider building relationships with rehabilitation centers. Many patients need home care before they're discharged, so rehab facilities can be a great source of referrals instead of relying only on agencies. 3. Connect with your local Chamber of Commerce and Council on Aging. They often have referral networks and can introduce you to families looking for services. 4. Keep in mind that assisted living communities are also competing for many of the same clients, so it's important to clearly communicate what makes home care the better option for those who want to remain at home."

HOME CARE BUSINESS OWNERS · Jenae Wilson Frazer (operator) · ↑5 reactions · <https://www.facebook.com/groups/2156388261379184/posts/289670824068051...>

"I see a lot of owners in here asking how to get more private pay clients. Private pay has been the foundation of how I built my agency. We've grown to a multi-million-dollar home care company by learning how to build relationships, position our services correctly, create community referral pipelines, and stop depending on one payer source to grow. I'm opening up some time to help a few home care owners who are serious about building or strengthening their private pay side. If you're tired of guessing and want to learn what has actually worked inside my own agency, comment PRIVATE PAY or message me. I'm not teaching theory. I'm showing you what we actually do."

HOME CARE BUSINESS OWNERS · Louis Rios a week ago (operator) · ↑4 reactions · <https://www.facebook.com/groups/2156388261379184/posts/293898756311924...>

WHAT FINALLY WORKED — 13 buyer documents

"I used place for mom and care in homes. They're not a waste to me. I got my first private pay client that turned into 24HR care. You will def need a system in place for the leads to follow up so you can convert them over. Yes some leads doesn't answer,... See more"

Homecare for newbies · Yasmine Walker (operator) · ↑11 reactions · <https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

"Your agreement should have everything in it. I send an email first & only meet after they've seen it. It has worked for me this way for years.."

Homecare for newbies · Parris Lucille a year ago (operator) · ↑10 reactions · <https://www.facebook.com/groups/706524484973799/posts/1008111701481741>

"Thanks for being so open about this – early days are tough for most of us in domiciliary care. What worked for me was building strong relationships locally rather than relying too much on ads. Start with: *Social workers & hospital discharge teams (visit in person if you can)* Local GP surgeries *Community groups / churches* Also, *word of mouth becomes your biggest asset, so even 1–2 clients done really well can snowball.* I'd also suggest checking your *Google presence (reviews + local SEO)* as families often search there first now. Don't lose momentum – 10 months is still early." — Straightforward + Honest "I'll be honest – Facebook ads and directories rarely bring consistent care clients. Most of our growth came from relationships, not marketing spend. If I were starting again, I'd focus on: Knocking on doors (GPs, pharmacies, community hubs) *Building trust with social workers* Getting your first 5 clients and delivering exceptional care Once your reputation builds, referrals follow. It's slow at first but much more reliable." — Encouraging + Relatable "You're definitely not alone – a lot of us struggled in the first year. What changed things for me was shifting from 'online marketing' to 'being visible in the community.' People need to trust a care provider, and that usually comes from relationships, not ads. Also worth asking – what makes your service different? That clarity really helps when speaking to families and professionals. Stick with it, you're closer than you think." — Engaging (to spark conversation) "Out of curiosity – what area are you based in? A lot of growth strategies depend on your local commissioning setup."

Care Managers Network UK · CQC Registered Professionals · Dewan Choudhury (operator) · ↑7 reactions · <https://www.facebook.com/groups/1971829479945179/posts/250346596678152...>

"Hi, I got my first nursing home gig and I do not understand PPD at all. Can anyone help me?"

Administrator In Training AIT · Anonymous participant (operator) · ↑7 reactions · <https://www.facebook.com/groups/nursinghomeadmin/posts/928920600446643...>

I'D PAY FOR RESULTS, NOT A RETAINER — 9 buyer documents

"I've gotten two hubby and wife couples from them. It is hit and miss however. I'm quick to call customer service and get my money back if I don't make contact. I had one person looking for someone to take his family member to their house on the weekends. Better call the nursing home."

Homecare for newbies · LaKisha Collins a year ago (operator) · ↑7 reactions · <https://www.facebook.com/groups/706524484973799/posts/738060561820191>

"Yes, some are transitioning to that model, charging a \$500 fee that's refundable after placement. Others are shifting towards care management. It's all about disclosure—disclosure—disclosure! Everyone has their own approach. My current no-fee business model is working well, but who knows, maybe I'll consider care management in the future!"

Senior Placement Network · Christine Sevier a year ago (operator) · ↑4 reactions · <https://www.facebook.com/groups/2447813518868667/posts/403065849391748...>

"I tried the service and the leads do come but like mentioned above, you are competing with so many other companies both small and large. I tried it for about three months and it just wasn't worth the money to me because I never got one client from the time I used them. It may work for others, but it just wasn't a good fit for me and I was losing money paying them. I'm about to go the route that was mentioned above via Facebook ads and Google."

Homecare for newbies · Shemeka Doyle-Green a year ago (operator) · ↑1 reactions · <https://www.facebook.com/groups/706524484973799/posts/738060561820191>

"They deducted \$600 from my account after I declined to acknowledge the transaction. My bank refunded my money because it was their fault. The bank went after them and collected the money back. A place for mom started calling us that our account is due and we need to pay the \$600. I just ignored them. They threatened to report us to where I don't know. I still did not say a word and they stopped calling"

HOME CARE BUSINESS OWNERS · Nicole Nde (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

IF SOMEONE WOULD JUST SHOW ME A SYSTEM THAT BRINGS CLIENTS PREDICTABLY — 6 buyer documents

"Hi first, you're not alone. A lot of agency owners go through this phase, especially in the first year. Getting licensed is one thing — building consistent referrals is another. A few things to look at: 1. Clarify your niche. If you're marketing to "everyone," it's harder to stand out. Are you focusing on dementia care, post-hospital recovery, companionship, veterans, private pay families? A clear focus makes referrals easier. 2. Strengthen local relationships. Home care is still a relationship business. Connect with: • Assisted living communities • Rehab centers & discharge planners • Social workers • Senior centers • Churches • Elder law attorneys TEven simple in-person introductions can make a big difference. 3. Review your online presence. When families search, they look for: • A clear website • Google Business profile • Reviews • Clear contact info If it's hard to understand what you offer in 10 seconds, they move on. 4. Follow up consistently. Sometimes it's not about getting more leads — it's about better follow-up. Quick response times and simple intake processes matter a lot. 5. Check your messaging. Are you talking about your agency... or solving the family's problem? Messaging that speaks to "peace of mind," "reliable caregivers," and "safety for mom" tends to connect more. Most importantly, don't take it personally. This business takes patience and consistency. If you're open to sharing what you've tried so far, I'm sure the group can give more specific feedback. You've got this"

Home Care Owners [RNs/ PCA, HHAs in US] · Cassagnol Channel (operator) · ↑3 reactions · <https://www.facebook.com/groups/homecareownersinusa/posts/122625849943...>

"GoDaddy website builder is okay. But not for everyone, you are service provider. You need consistent clients & growth. If you want this you should focus on Wordpress CMS. I'd definitely recommend going with WordPress. It gives you much more control, better SEO potential, and flexibility as your homecare agency grows. It might take a bit more setup, but it's far more effective long-term for getting consistent leads from Google. Are you planning to build it yourself or get help?"

HOME CARE BUSINESS OWNERS · Kabir Mahbub (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/280998794601920...>

"After months of starting the process to get approved by private insurances, we're very close to being approved by the last one we chose. I wanted to get some insight or guidance on if it's normal for referrals from insurances to be slow in the beginning? I've only received 1 referral out of 5 insurance companies we've been approved for. Also, I'd appreciate very much to get some insight on anyone who has run social skills groups and how those worked out or still working out for you. I do not have a center, but that's the goal someday in the near future. I do have a space that I can use for the social skills group. I thought this was a way to get the foot out the door and bring exposure to our company as well since we're not getting much referrals currently. Thank you in advance!! — in New Jersey."

ABA Business Owners (operator) · <https://www.facebook.com/groups/669886889110965/posts/978189904947327>

"Let me ask you something... If you could spend an hour with someone who: Has reached 94/94 occupancy Built a system that keeps all 94 beds constantly full Works with operators managing 1000+ beds ...what would be the first thing you'd ask? That's what this next call is all about. I'm hosting a free "Ask Me Anything" Zoom call for assisted living, personal care home, long term care, and adult family home operators. You can ask about: Increasing occupancy Getting more referrals Getting staffing under control Marketing that actually works Anything else that has you stuck No long presentation. Just bring your questions and get answers. Drop "Q&A" below and I'll send you the link."

Home Care NY - PPL (CDPAP), NHTD, PCA/HHA, Private Pay (operator) · <https://www.facebook.com/groups/1388972261963168/posts/216813766404662...>

THE KEY IS / PREDICTABLE (THE MARKET'S PROMISE) — 4 buyer documents

"Hi first, you're not alone. A lot of agency owners go through this phase, especially in the first year. Getting licensed is one thing — building consistent referrals is another. A few things to look at: 1. Clarify your niche. If you're marketing to "everyone," it's harder to stand out. Are you focusing on dementia care, post-hospital recovery, companionship, veterans, private pay families? A clear focus makes referrals easier. 2. Strengthen local relationships. Home care is still a relationship business. Connect with: • Assisted living communities • Rehab centers & discharge planners • Social workers • Senior centers • Churches • Elder law attorneys TEven simple in-person introductions can make a big difference. 3. Review your online presence. When families search, they look for: • A clear website • Google Business profile • Reviews • Clear contact info If it's hard to understand what you offer in 10 seconds, they move on. 4. Follow up consistently. Sometimes it's not about getting more leads — it's about better follow-up. Quick response times and simple intake processes matter a lot. 5. Check your messaging. Are you talking about your agency... or solving the family's problem? Messaging that speaks to "peace of mind," "reliable caregivers," and "safety for mom" tends to connect more. Most importantly, don't take it personally. This business takes patience and consistency. If you're open to sharing what you've tried so far, I'm sure the group can give more specific feedback. You've got this"

Home Care Owners [RNs/ PCA, HHAs in US] · Cassagnol Channel (operator) · ↑3 reactions · <https://www.facebook.com/groups/homecareownersinusa/posts/122625849943...>

"GoDaddy website builder is okay. But not for everyone, you are service provider. You need consistent clients & growth. If you want this you should focus on Wordpress CMS. I'd definitely recommend going with WordPress. It gives you much more control, better SEO potential, and flexibility as your homecare agency grows. It might take a bit more setup, but it's far more effective long-term for getting consistent leads from Google. Are you planning to build it yourself or get help?"

HOME CARE BUSINESS OWNERS · Kabir Mahbub (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/280998794601920...>

"My first client I received was a 24/7 client within 1 month of opening my business. If you are good at sales and following up... ABSOLUTELY! If you are good at follow ups then your conversion rate can be at least 20%. from each batch."

Homecare for newbies · Tia McCray Pollard (operator) · <https://www.facebook.com/groups/706524484973799/posts/1216180170674892>

"Reviewing a few assisted living businesses today — If you're relying on referrals or inconsistent inquiries, I can usually point out exactly what's missing and how to get more steady inbound. Where are you at right now?"

Residential Assisted Living Homes of Texas (operator) · <https://www.facebook.com/groups/795881234867455/posts/1469163360872569>

Tab 8 — Desired Outcomes

Themes, measured (buyer documents; operators / pre-launch): A full building: 90% occupancy, a wait list 11 (10/1) · Full census / full caseload / every bed filled 6 (6/0) · Predictable clients / full census 4 (3/1) · Referrals and leads I own, not a placement agent's 4 (4/0)

A FULL BUILDING: 90% OCCUPANCY, A WAIT LIST — 11 buyer documents

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions · <https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"Almost exactly 5 years ago, I bought my first assisted living facility. A 10-home, 94-bed CBRF in Wisconsin. I wish I could tell you the last 5 years have been easy and it's been nothing but success stories. That would be a lie, at least partly. The truth? We got our asses kicked lol. Things started out good. We were cashflow positive, starting to better understand the business and get things down. The first thing we knew we had to work on was staffing. This is right around the time the government programs related to Covid kicked in, and many people, not all, realized they could make more money staying home than they would if they went to work. And, well, a lot of people chose to stay home. We tried to combat it by raising the pay for our caregivers. In many cases, we increased caregiver pay by 50%+. But it still didn't solve the problem, so we started working with 3rd party staffing companies. At one point, we were paying around \$60k every single month just in 3rd party staffing. Our profitable business soon turned cash flow negative. At the same time, we were also battling vacancies. We fluctuated from 7 to 14 empty beds on a monthly basis. It seemed like no matter how hard we worked, we couldn't keep up. We would fill three beds and then lose 4. Then we would fill another one and lose two. It was a constant battle of up and down, without ever really getting ahead. Everyone was stressed, including me, and nothing we tried seemed to solve the occupancy problem. Today, as I write this, all 94 of our beds are full and we have a wait list. We are fully staffed without spending a single dollar on 3rd party staffing. Our staff is happier, stronger, and working together better than ever. How did we go from where we were to where we are today? We changed what we were doing. It doesn't matter how hard you work. If you're working on the wrong things, working harder isn't going to fix it. You need a different approach and a different system. And that's exactly what we did. I'm sharing this because I constantly see people in these groups struggling with staffing and keeping their beds full. I'm thinking about doing a Zoom call for this community and walking through exactly what we're doing to keep our beds full and our facilities staffed.. I'll have my team on the Zoom as well so you can ask questions at the end. I'm not going to charge for it and I'm not selling anything on the call. I just want to help people who are struggling with these same issues or know they could improve in these areas. If you're interested, hop on a Zoom call with us on August 20th at 8pm EST and we'll figure it out together a day at a time. Comment "BEDS" and I'll DM you the link. Facebook sometimes sends messages from people you aren't connected with to your Message Requests folder. If you comment "BEDS," keep an eye on your Message Requests in case my message lands there."

Rcfe Owners Network Group (operator) · ↑28 reactions · <https://www.facebook.com/groups/352285274494478/posts/976832502039749>

"Hi! I own a small practice that provides BCBA direct services. We are home & community based and have already acquired a waitlist. I would love to hire on additional BCBAs, but am competing with a hospital system and corporate ABA (why I am anonymous lol). What would make you consider a small company role vs a large clinic? My goal is to hire on BCBAs and take the time to train, work with them, etc so they will autonomy over their own cases. I 'm not looking to micromanage anyone, just support and provide high quality services. Not including pay- which I could compete with others in the area!"

BCBA Careers · ScenicNectarine9995 (operator) · ↑3 reactions · <https://www.facebook.com/groups/7447177152061328/posts/282149757248548...>

FULL CENSUS / FULL CASELOAD / EVERY BED FILLED — 6 buyer documents

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions ·

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I'm not in sales, but I'm a department head and know that in my company there are monthly census quotas, sale quotas and coming off the COVID situation lots of companies really cranked the pressure up on Assisted Living Communities back in May when the president declared the Covid state of emergency in the nation was "over". Almost immediately it was an expectation that everything get back to "business as usual" and they doubled/tripled the quotas for move ins. There are high quotas each months if the building is not fully occupied and even if it is fully occupied there is an expectation of having a sizeable wait list with people who have placed a deposit to hold apartments for the future. A lot of people aren't aware that assisted living communities are mostly owned by real estate investment trusts that have a contract with management companies whose names are on the signs of these properties. So the owners' groups who own the property put a lot of pressure on the management companies and at the community level it just compounds with expectations that are intensified. Our executive director and sales director have weekly conference calls with the corporate office where they are grilled about move ins on the books already confirmed, projected move ins with inquiries that have sounded promising and what it will take to get them to sign and put a deposit down, how many inquiries are in the database that haven't come into the community to tour and why and what can we do to get them in to tour and how many leads are in the database to follow up on as well as any move outs and a lot of details on the factors that led to the move out (death, needing skilled care, financial reasons, dissatisfaction). If you ever make an inquiry at a community that is part of a chain, even a smaller company, you will be logged into the system and the sales team will contact you every month until you formally demand to be taken off the list or until you say your relative has passed away. If you have come into the building for a tour, there will be even more contact until you say you have chosen another community (they will ask why), that your relative has passed or if you formally request no further contact. Edited to add: please also be aware that the deal that has been made is not permanent. The rate increases every January- this is industry wide. Depending on how much of a deal you get now, you may experience a higher percentage increase the next year. Everyone's rent goes up. It may be 3% it may be 12%. 6/8% seems to be the average in our area this year. Also- most communities give you a room rate and that is one thing, but care charges are a completely different thing. If your father begins requiring help with anything or if his needs of what is already being done increases the care charges will increase. If there is a rapid cognitive decline and needs change with dressing, bathing, continence, cueing to meals and activities, cueing to get up in the morning or not being oriented to place and time there will be a need for him to move to a memory care setting and that is generally significantly more expensive. There are also situations in my community where care needs are so personalized that a family hires a third party caregiver as well to be with their loved one and this could be for a variety of reasons. I know it can all be overwhelming and a lot of pressure to move in, feel free to ask all of the questions- staffing ratios, when shift changes are, percentage ratios of staff turnover, resident turnover, other charges that could arise for anything like transportation to doctor appointments to the community providing continence supplies. Just lots of information in a short time. Take your time absorbing the info and ask all the questions you need. Verbalize that you don't like the feeling of being pressured. You can put that in a google review as well (there is a huge push for people just moving in to do online reviews for a lot of these companies). Hoping this info is helpful and wishing you luck and know that so many of the people who work in the communities, like myself, feel so privileged to be trusted to care for people's loved ones each day, we are passionate about what we do, we love sharing time getting to know each resident and work so hard every day to help each person have a wonderful day!"

r/AssistedLiving (operator) · ↑5 upvotes · https://www.reddit.com/r/AssistedLiving/comments/18p1j1x/how_does_the_...

"A place for mom charges quite a lot. Contact social services in every hospital that you can. They keep a list of licensed care homes that APS has cases on and refers to the displaced residents. I was in a shutdown of an unlicensed care home and was sent to the hospital, is how I know. I noticed there was several homes on facebook, is there a way to network them and if someone's has a full house refer to someone that needs clients, in other words, help each other."

Private Pay Clients looking for Affordable Home Care Services. · Patti Wyckoff (operator) · ↑2 reactions · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

PREDICTABLE CLIENTS / FULL CENSUS — 4 buyer documents

"Hi first, you're not alone. A lot of agency owners go through this phase, especially in the first year. Getting licensed is one thing — building consistent referrals is another. A few things to look at: 1. Clarify your niche. If you're marketing to "everyone," it's harder to stand out. Are you focusing on dementia care, post-hospital recovery, companionship, veterans, private pay families? A clear focus makes referrals easier. 2. Strengthen local relationships. Home care is still a relationship business. Connect with: • Assisted living communities • Rehab centers & discharge planners • Social workers • Senior centers • Churches • Elder law attorneys TEven simple in-person introductions can make a big difference. 3. Review your online presence. When families search, they look for: • A clear website • Google Business profile • Reviews • Clear contact info If it's hard to understand what you offer in 10 seconds, they move on. 4. Follow up consistently. Sometimes it's not about getting more leads — it's about better follow-up. Quick response times and simple intake processes matter a lot. 5. Check your messaging. Are you talking about your agency... or solving the family's problem? Messaging that speaks to "peace of mind," "reliable caregivers," and "safety for mom" tends to connect more. Most importantly, don't take it personally. This business takes patience and consistency. If you're open to sharing what you've tried so far, I'm sure the group can give more specific feedback. You've got this"

Home Care Owners [RNs/ PCA, HHAs in US] · Cassagnol Channel (operator) · ↑3 reactions · <https://www.facebook.com/groups/homecareownersinusa/posts/122625849943...>

"GoDaddy website builder is okay. But not for everyone, you are service provider. You need consistent clients & growth. If you want this you should focus on Wordpress CMS. I'd definitely recommend going with WordPress. It gives you much more control, better SEO potential, and flexibility as your homecare agency grows. It might take a bit more setup, but it's far more effective long-term for getting consistent leads from Google. Are you planning to build it yourself or get help?"

HOME CARE BUSINESS OWNERS · Kabir Mahbub (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/280998794601920...>

"Reviewing a few assisted living businesses today — If you're relying on referrals or inconsistent inquiries, I can usually point out exactly what's missing and how to get more steady inbound. Where are you at right now?"

Residential Assisted Living Homes of Texas (operator) · <https://www.facebook.com/groups/795881234867455/posts/1469163360872569>

REFERRALS AND LEADS I OWN, NOT A PLACEMENT AGENT'S — 4 buyer documents

"Thanks for posting videos about this , I searched for a while and have not found much info. I started my agency recently and want to increase my leads"

youtube · @julietdron9144 · The Basics - How To Start A Senior Referral Business! (operator) · ↑5 likes · <https://www.youtube.com/watch?v=DtNU13mT-ZE&lc=UgwdH5S0yHLfx6OlodZ...>

"Hi first, you're not alone. A lot of agency owners go through this phase, especially in the first year. Getting licensed is one thing — building consistent referrals is another. A few things to look at: 1. Clarify your niche. If you're marketing to "everyone," it's harder to stand out. Are you focusing on dementia care, post-hospital recovery, companionship, veterans, private pay families? A clear focus makes referrals easier. 2. Strengthen local relationships. Home care is still a relationship business. Connect with: • Assisted living communities • Rehab centers & discharge planners • Social workers • Senior centers • Churches • Elder law attorneys TEven simple in-person introductions can make a big difference. 3. Review your online presence. When families search, they look for: • A clear website • Google Business profile • Reviews • Clear contact info If it's hard to understand what you offer in 10 seconds, they move on. 4. Follow up consistently. Sometimes it's not about getting more leads — it's about better follow-up. Quick response times and simple intake processes matter a lot. 5. Check your messaging. Are you talking about your agency... or solving the family's problem? Messaging that speaks to "peace of mind," "reliable caregivers," and "safety for mom" tends to connect more. Most importantly, don't take it personally. This business takes patience and consistency. If you're open to sharing what you've tried so far, I'm sure the group can give more specific feedback. You've got this"

Home Care Owners [RNs/ PCA, HHAs in US] · Cassagnol Channel (operator) · ↑3 reactions · <https://www.facebook.com/groups/homecareownersinusa/posts/122625849943...>

"GoDaddy website builder is okay. But not for everyone, you are service provider. You need consistent clients & growth. If yoy want this you should focus on Wordpress CMS. I'd definitely recommend going with WordPress. It gives you much more control, better SEO potential, and flexibility as your homecare agency grows. It might take a bit more setup, but it's far more effective long-term for getting consistent leads from Google. Are you planning to build it yourself or get help?"

HOME CARE BUSINESS OWNERS · Kabir Mahbub (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/280998794601920...>

Tab 9 — Typical Day

Themes, measured (buyer documents; operators / pre-launch): The hands-on licensee-administrator running two facilities 11 (11/0) · The phone / scheduling never stops (24/7) 6 (6/0)

THE HANDS-ON LICENSEE-ADMINISTRATOR RUNNING TWO FACILITIES — 11 buyer documents

"Any insight on Lic nursing home administrator and/ or AIT program. Looking for Pros and Cons. How difficult is it being an admin? I'm looking for a better work/life balance. Plus I'm burned out working clinical. I've worked in SNFs for 11 years as a therapist. 7 years as the DOR. I've worked closely with MDS. I've been a part of a wound care team. I've also worked with nursing to implement fall programs and other collaborations. My point is I've seen a lot of how the back of the house works. Also, my previous career was owner/operator of a high volume family restaurant. I have experience with budgets and P&L. I know this kinda sounds like a resume. But I think it's beneficial offering opinions if one knows my experience. Even when this post becomes old, any advice and comments would be appreciated."

r/healthcareadmin (operator) · ↑20 upvotes · https://www.reddit.com/r/healthcareadmin/comments/sych6p/any_insight_o...

"Don't forget YOU are not the manager, you are the owner/operator. So no.... actually you don't show up when caregivers call out, you're not licensed to do their work, the manager/administrator gets called and fills the shift with another caregiver or does the job themselves. You won't even know about it, because they are handling the day to day."

youtube · @RALAcademy · Residential Assisted Living Is The Number One Real Estate In (operator) · ↑19 likes · <https://www.youtube.com/watch?v=Q9bl7hQL0w8&lc=UgzvNlleUuUMvGqtlYh...>

"Same. My kid was 12 at the time and my corporate job environment had gotten so toxic I missed a volleyball game because I couldn't stop crying that night. I started planning my exit the very next day and used the severance money to fund my first year as a licensee. It seemed crazy at the time because my income/benefits were so good but I just couldn't bear to go into another corporate role. Aside from having to pay more for crappier health insurance, I have zero regrets."

r/realtors (operator) · ↑9 upvotes · https://www.reddit.com/r/realtors/comments/1j449i8/taught_78_years_old...

THE PHONE / SCHEDULING NEVER STOPS (24/7) — 6 buyer documents

"Scheduling software for 24/7 home care. I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p).* Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) *Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑8 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

"Scheduling software for 24/7 home care I'm the financial PoA for a friend of the family and we've hired 24/7 in-home caregivers for her who we are hiring/managing directly rather than going through an external agency. I'm looking for scheduling software to make life for both the lead caregivers who makes the schedule and for myself (to automate detection of overtime, gaps/double-coverage, etc.). There are a bunch that do a lot of what we want, but all the ones I've found so far miss a few things we'd really like, and I'm hopeful someone will know of something that can do what we want. Here are the features I haven't yet managed to find all of in a product so far: *Supports 1-deep 24/7 scheduling. At a minimum, it should flag (though not forbid) gaps or double-covered times. Bonus points if it's easy to change the shift change time (e.g. don't make me change both shifts if a shift change time moves from 3p to 4p).* Allocate overnight shifts to the day/week in which the hours work instead of lumping them all onto the day when the shift starts, to match our timekeeping processes. Without this, the per-week overtime computations will be incorrect for us. (Different agencies may handle allocation of overnight hours differently, but this is how we handle it, and we need the scheduling software to match or the overtime validation isn't useful to us.) *Allow us see the schedule in the time zone in which the caree is located, rather than the local time zone for the person viewing the schedule. (I live in a different time zone, and don't want to have to mentally translate to the caregiver's local time, just show me the schedule in their time zone so I don't have to translate.)* Provide a monthly calendar view that runs Su-Sa even though our pay period runs Th-W. (It's great for the weekly view to match our schedule, but when we go to a monthly view I want to see it on a normal calendar. Or to be able to switch back and forth, that would be ideal.) I've demoed the most promising options I found (ShiftCare, Connecteam, MakeShift, WhenToWork, Rotaville, and I'm waiting on approval for my trial account for Careswitch) and as of right now they're all a miss on the things above. Anyone have a recommendation for something that'll meet our needs?"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...>

"This is a good question. My gut tells me that a nursing home administrator would pay more than an assisted living administrator because it's a higher level of care, but I also think it depends on the size of the facility. One thing you want to be aware of for both, is as the administrator, you are responsible for everything going on at the facility and you need to make sure people (staff and residents) are following all the rules. You'll need to be great at communicating, and also make sure you stay on top of all the rules and regulations. You're also required to cover any shifts that get dropped, and you'll be on call 24/7. It can be a very demanding job. As the administrator though, you are in charge and can have a huge impact on the staff and residents and it can be extremely rewarding. I hope that helps!"

youtube · @assistedlivinginvesting · DAY IN THE LIFE OF AN ASSISTED LIVING FACILITY ADMINISTRATOR (operator) · ↑2 likes · https://www.youtube.com/watch?v=GWgKaNjsG_s&lc=Ugy6dgpU6Qa323zIHS1...

Lead Sources — what they have paid for and what it returned (first 6 voices per source; all 12 are on the page)

Every lead source or vendor an operator names, counted on the buyer corpus. *Worked* / *burned* / *asking* are language matches in the same document — a sort for reading, not a verdict. Dollar figures are the ones written in the sentence that names the source.

lead source	buyer docs	operators	all docs	worked	burned	asking	dollar figures quoted
Hospital / SNF / discharge planner referral relationships	128	115	698	30	10	30	\$500.00, \$10, \$20, \$12, \$25K
Coaches, courses, consultants (RAL Academy, Homecare 101, Mom's House, CSA, SRES)	91	58	860	9	2	29	\$1000, \$484.80, \$750/week, \$30
Networking / BNI / chamber / senior centers / community events	62	59	211	8	6	10	\$250, \$2500, \$500, \$600, \$400
SEO / website / Google Business Profile	58	53	258	8	4	14	\$250, \$2500, \$500, \$600, \$400, \$20,000 per month
A Place for Mom	42	40	351	8	9	9	\$25 per lead, \$600, \$68 per lead
Placement agents / referral agencies (CarePatrol, ALL, Oasis, independents)	39	38	174	6	1	12	\$1.1 m
Insurance panels / pediatricians / schools (ABA)	25	23	104	1	3	9	
Google Ads / LSA / PPC	17	16	49	3	2	2	\$3 a day, \$15, \$2, \$1,700 per month
VA / Medicaid waiver / LTC insurance contracts	16	14	89	1	0	4	\$750/week
Franchise systems (Home Instead, Visiting Angels, Right at Home, CarePatrol...)	13	11	248	1	2	5	
Pay-per-lead / lead-gen vendors	11	11	33	3	5	0	\$6 per lead, \$25 per lead, \$58, \$300 per lead, \$68 per lead
Facebook / Instagram / Meta ads	10	9	39	4	2	1	\$60, \$6 per lead
Caring.com	8	8	33	3	1	2	\$25 per lead
Yelp / Angi / HomeAdvisor / Thumbtack / Nextdoor	6	5	27	0	1	2	\$250, \$2500, \$500, \$600, \$400
CareInHomes / Aging Care / other lead directories	4	4	13	1	2	0	\$25 per lead
Marketing agencies / consultants	4	3	27	2	0	0	
Care.com	1	0	128	0	0	1	

Read: the ledger is a sort, the voices are the evidence. Three things hold across every group. (1) The paid referral sites are the most talked-about and the most resented source: prepaid blocks, per-lead billing regardless of outcome, the same family sent to several agencies, aggressive sales reps who "call every week", and a fight for credits; the minority who defend them say it is a speed and follow-up game. (2) Relationship referrals (discharge planners, social workers, rehabs, elder-law attorneys, senior centers, churches) are the universally recommended route and the one owners say actually produced their first clients; they are also slow ("8–9 months"), gated, and dependent on the owner's own time. (3) Paid digital is where the owner has the least language: Google Ads are "the most expensive and least effective" in one owner's telling and "\$6 a lead with a high close rate" in another's; Facebook ads brought caregivers, not clients, for the one owner who reports the test. Nobody in the buyer voice describes a working owned inbound system with numbers attached, which is the hole the ghost advertisers (A Place for Mom's consumer funnels) and the operator-facing vendors (Occupancy Partners, Plena, the pay-per-lead sellers) both sell into.

Hospital / SNF / discharge planner referral relationships — 128 buyer documents (115 operators)

The recommended route in every group and the one owners credit for first clients. It is described as slow (months to develop), personal (cookies, lunch-and-learns, showing up weekly), gated (hospitals 'say no', social workers keep lists of licensed agencies), and non-scalable (it lives in the owner's calendar). The copy has to respect it, not replace it: 'keep your discharge planners; stop depending on them for every client.'

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life.

Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you.

Case Manager: Good morning. It's nice to meet you.

Administrator: Thank you. Before I leave, may I ask you one quick question?

Case Manager: Of course.

Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations.

Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital.

Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises.

Case Manager: I like that. Thank you for taking the time to stop by.

Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.)

Case Manager: Thank you. I'll definitely keep this for future reference.

Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community.

Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit.

Case Manager: Thank you so much. That's very thoughtful of you.

Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in

touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too! HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions ·

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"Any insight on Lic nursing home administrator and/ or AIT program. Looking for Pros and Cons. How difficult is it being an admin? I'm looking for a better work/life balance. Plus I'm burned out working clinical. I've worked in SNFs for 11 years as a therapist. 7 years as the DOR. I've worked closely with MDS. I've been a part of a wound care team. I've also worked with nursing to implement fall programs and other collaborations. My point is I've seen a lot of how the back of the house works. Also, my previous career was owner/operator of a high volume family restaurant. I have experience with budgets and P&L. I know this kinda sounds like a resume. But I think it's beneficial offering opinions if one knows my experience. Even when this post becomes old, any advice and comments would be appreciated."

r/healthcareadmin (operator) · ↑20 upvotes · https://www.reddit.com/r/healthcareadmin/comments/sych6p/any_insight_o...

"I honestly created my own ad using Canva and pay for the marketing on my Facebook page. My referrals have come from Facebook marketing 50% and I have placed home health brochures at dispensaries here, grocery stores, dollar general, and doctors offices. I have a brochure and a meet the owner postcards at the places I listed above. Everybody's results will be different but there are many options to try."

Homecare for newbies · Nyesha Jennings a year ago (operator) · ↑11 reactions · <https://www.facebook.com/groups/706524484973799/posts/976062864686625>

Coaches, courses, consultants (RAL Academy, Homecare 101, Mom's House, CSA, SRES) — 91 buyer documents (58 operators)

The biggest 'source' by count is not a lead source at all: coaches, consultants, courses and designation programs (policies-and-procedures consultants at \$1,000, RAL Academy, Homecare 101, Mom's House for realtors, CSA/CAPS designations). The pre-launch owner buys knowledge before clients; the operating owner has usually already paid one and is skeptical of the next ('everyone was trying to poach me for money').

"What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency? I recently came across this question on this platform, and I thought it would be helpful to address it since many home care agency owners may have the same question. Before you think about what to send, I want you to change the way you think. Put yourself in the shoes of a hospital case manager, a social worker, or a discharge planner. Every day, they receive calls, emails, brochures, and visits from different home care agencies, all hoping to earn their trust. What would make them remember your agency? It is not because you brought the biggest gift basket or the most expensive promotional item. It is because you presented yourself professionally, built a genuine relationship, clearly communicated the value of your agency, and demonstrated that you are dependable, knowledgeable, and committed to providing quality care. Referral sources place their professional reputation on the line every time they recommend a home care agency. If a patient has a poor experience, it reflects on them as well. That is why trust is one of the most important factors in building referral relationships. When you introduce your agency, your goal should not be to ask for referrals. Your goal should be to introduce yourself professionally, learn what is important to the referral source, understand the challenges they face, and explain how your agency can help meet those needs. Remember, introducing yourself is only the beginning. One visit rarely builds a referral relationship. Stay consistent. Follow up professionally, keep your promises, communicate effectively, and deliver excellent care every time a referral is entrusted to your agency. Over time, people will begin to associate your agency with professionalism, reliability, and quality care. That is what turns an introduction into a trusted referral relationship. Now, let's look at what that conversation might sound like in real life.

Administrator: Good morning. My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC. I know you're very busy, so I won't take much of your time. I just wanted to stop by to introduce myself and our agency. We are a licensed non-medical home care agency providing personal care, companionship, respite care, post-hospital support, transportation, and other in-home support services that help individuals remain safe and independent in the comfort of their own homes. It's a pleasure meeting you.

Case Manager: Good morning. It's nice to meet you.

Administrator: Thank you. Before I leave, may I ask you one quick question?

Case Manager: Of course.

Administrator: When you're selecting a home care agency for your patients, what qualities are most important to you? I always like to understand what our referral partners value so we can better meet their expectations.

Case Manager: Communication is very important to us. We also look for agencies that answer their phones, respond promptly, start care without unnecessary delays, and have dependable caregivers. We want to know that our patients will receive quality care after they leave the hospital.

Administrator: Thank you for sharing that with me. I completely understand why those qualities are important. When you refer a patient to a home care agency, you're placing your trust and professional reputation on the line. Our goal is to provide reliable, compassionate care while maintaining open communication with both your team and the families we serve. We would truly appreciate the opportunity to earn your trust whenever the need arises.

Case Manager: I like that. Thank you for taking the time to stop by.

Administrator: The pleasure is mine. Thank you for taking time out of your busy schedule to meet with me today. I'd like to leave you with our brochure and business card so you'll have our contact information whenever the need arises or if you ever have a patient who could benefit from our services. (Hands the brochure and business card to the case manager.)

Case Manager: Thank you. I'll definitely keep this for future reference.

Administrator: Before I go, I'd also like to leave this small token of appreciation for your office. I know how much dedication it takes to help patients and families every day, and I just wanted to say thank you for everything you and your team do for our community.

Note: I generally do not recommend giving edible items. While they may be appreciated, you never know whether someone has food allergies, dietary restrictions, or personal preferences. In addition, once the food is consumed, the reminder of your agency is gone. Instead, consider practical, professional items such as branded mugs, pens, notepads, journals, calendars, desk organizers, tote bags, reusable water bottles, or other useful office supplies featuring your agency's name and logo. These items provide lasting value, keep your agency visible, and help maintain top-of-mind awareness long after your visit.

Case Manager: Thank you so much. That's very thoughtful of you.

Administrator: You're very welcome. It was truly a pleasure meeting you today. I look forward to staying in

touch and building a professional relationship with you and your team. Have a wonderful day. What Can We Learn From This Scenario? Notice the order of the conversation. The administrator did not walk in asking for referrals. She also did not begin by handing over a brochure, business card, or gift. Instead, she first introduced herself, built rapport, asked a thoughtful question, listened carefully, and addressed the concerns that mattered most to the case manager. Only after building that connection did she present her brochure and business card. By that point, the marketing materials supported the conversation and gave the case manager an easy way to contact the agency when the need arose. Finally, as the meeting came to an end, she presented a small token of appreciation as a simple expression of gratitude for the dedication and service the case manager and the entire discharge team provide to the community. Remember, your brochure introduces your agency. Your business card provides a way to reach you. A thoughtful gift expresses appreciation. But it is your professionalism, communication, consistency, and the quality of care you provide that earn trust. And trust is what leads to long lasting referral relationships."

HOME CARE BUSINESS OWNERS (operator) · ↑48 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>

"Certifications that are worthwhile? I have my BA in Healthcare Admin and currently work as a Patient Access Specialist in a relatively small mental health clinic in New York State. I mostly deal with registering patients, maintaining medical records, verifying insurance benefits, obtaining pre-auths, and referring patients to other levels of care & providers. I really enjoy my job but want to learn more and feel like though my BA in Healthcare Admin was a good foundation, it didn't really go that deep into the everyday tasks I do. Specifically, with Health Insurance and billing, I could really use more in-depth training. Are there any Certifications for Healthcare Administration/Patient Access Professionals that are worthwhile? or even just training courses online without certification? My company does set aside money for us to use for training but all the training resources they seem to have are geared towards Clinicians."

r/healthcareadmin (operator) · ↑13 upvotes · <https://www.reddit.com/r/healthcareadmin/comments/rqk1re/certification...>

"I used to sjust stay quiet and mind my own business but there are some of us on these platforms who are genuinely looking to exchange help, ideas and function as peers. I don't mind paying for coaching or mentorship which I am doing. However, I pay someone I have seen demonstrate that they know what they are talking about and can get me the results I am looking for."

HOME CARE BUSINESS OWNERS · Pamela Styles (operator) · ↑10 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/273224561379344...>

"Is This Even Legal? I'm a direct service professional in California and I think my fellow workers and I have been getting taken advantage of. Let me explain. The company I work for has a policy called "shared staffing". In this company, clients are typically paired up into two bedroom apartments and there's one worker per individual client. So on a typical day there's two people in the house with two clients and everything runs pretty smoothly. But on a day, like today, if somebody calls out then it's only one person working with both clients. Now, I don't mind that so much, because both these clients are fairly easy to work with. But what my company likes to do is charge IHSS the full number of hours for the shift with the client that has IHSS hours and then pay nothing for the client that they would have to pay for (so we're only payed for one client). Is this normal? It feels highly unethical if not illegal and I don't even know if I have any recourse. This has been going on since I started working for this company a few years ago and no one seems to be able to help me with this so I hope that someone here can. Feel free to ask any follow-up questions necessary for clarity. Thank you so much in advance for your time!"

r/homehealthcare (operator) · ↑6 upvotes · https://www.reddit.com/r/homehealthcare/comments/d6ktak/is_this_even_l...

"What are the steps to start a residential care facility in Roseville, CA?. Hi RCFE owners/mgrs, May I please ask for your help and mentorship with getting my first RCFE started in Roseville, CA? I recently purchased a home and will be taking the administrator course soon and then apply for a facility license. I have been unable to find out the following: 1) where to get patients from? 2) how to get paid by the patients? Do patients pay me directly or does insurance pay me? 3) where to hire caretakers from? 4) what type of business or worker comp insurance will I needed? 5) while I am applying for facility license, should I rent out the house with a one year lease, to help with mortgage payments? 6) getting facility licensure may take a while, so can I submit facility license application before I obtain an admistrator license? I am sure there are many more questions I need to ask. Please help me with your expertise and experience. Thank you for your time and support!"

Residential Care Homes Owners/Operators · Van Voong (operator) · ↑6 reactions · <https://www.facebook.com/groups/732719531430807/posts/1399974124705341>

Networking / BNI / chamber / senior centers / community events — 62 buyer documents (59 operators)

"Visit assisted living communities and schedule in-person meetings with the office manager. Bring a small gesture, like drinks and light snacks, to help make a positive first impression. I may be young, but I had to think old school when it came to figuring out how to reach this population. Sometimes the best marketing is simply getting out there and putting in the footwork. I'm not sure how many counties you currently serve, but expanding your service area can also create more opportunities. Don't be afraid to hand out your flyers at places like Goodwill stores and other community locations where your target audience and their families may visit. If you're trying to grow your business while keeping costs low, go above and beyond by doing the legwork yourself. Building relationships face-to-face can make all the difference. I hope this helps someone!"

HOME CARE BUSINESS OWNERS · Priscilla Stone (operator) · ↑29 reactions · <https://www.facebook.com/groups/2156388261379184/posts/288420692193064...>

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"I am still not landing any leads for my Private Home Care Provider Agency. I have given out my business cards, brochures, sent emails, and gone to outreach community functions, but I haven't received even one call. What am I doing wrong?"

HOME CARE BUSINESS OWNERS (operator) · ↑21 reactions · <https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

"I'm feeling frustrated and overwhelmed, and I could really use some advice from those who have been in my shoes. I'm in California and received my home care agency license on March 30, 2026. Since then, I haven't had any luck getting clients. My first thought was to reach out to the assisted living facility where I previously worked. I was hopeful they would add my agency to their referral list, but I've been ignored. I've also contacted other assisted living facilities, but it seems like the wellness directors, administrators, or the people who handle caregiver referrals either don't answer calls, are never available, or won't give me an opportunity to meet with them. I'm blessed to have a wonderful support system at my church. Some of our seniors live in assisted living facilities, and their families have even tried speaking with the facility directors about using my agency. Unfortunately, they were told the facilities already work with five agencies and aren't willing to add any more. I'm honestly heartbroken. I invested a lot of time, money, and faith into building this business, and right now it feels like I'm not making any progress. Thankfully, I still have my full-time job, which helps keep me busy, but I truly want to grow my agency and make it successful. For those of you who have started a home care agency in California, what helped you get your first clients? How did you build referral relationships when doors kept closing? Are there strategies that worked for you besides assisted living facilities? I would truly appreciate any advice, encouragement, or lessons you've learned. Thank you so much"

HOME CARE BUSINESS OWNERS (operator) · ↑11 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/289670824068051...>

"Hi Jenny Jen Congrats on getting licensed — the fact that you've already mailed 4,000 flyers and are working referral leads tells me you're serious, which is more than most. The challenge is that those channels are cold. Your fastest path to a first client is almost always warm referrals — hospital discharge planners, social workers, and senior living advisors who personally hand you a name. Building those relationships takes showing up consistently in person, not just dropping off brochures. Happy to share what's worked for agencies I've helped scale. Feel free to connect."

HOME CARE BUSINESS OWNERS · Albert Raven (operator) · ↑10 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/284819363219864...>

SEO / website / Google Business Profile — 58 buyer documents (53 operators)

"Hey everyone! Can anyone recommend a really good marketer who knows the home care industry? I'm looking for someone who can help with Google SEO, Google Ads, getting my agency to rank higher on Google, improving my Google Business Profile, and bringing in more qualified leads. If you've personally used someone and had a good experience, I'd love their contact information. Thanks in advance!"

HOME CARE BUSINESS OWNERS (operator) · ↑22 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/291140860921047...>

"So for Florida your address needs to be physical not a virtual one. If you need to use your home address then do it. Delayed my application because I used a virtual address. Make sure you file your sunbiz correctly they will be looking at it. Make sure your verbiage is correct. Do not put "home health" anywhere on your website. You can do home care. Don't mention anything about doing ADLS, in Florida it's only supervision for ADLs, hands on personal care isn't allowed and you'd be considered home health at this point. The AHCA homemaker license is like \$100 I believe . The skilled one more expensive and more regulations. There is no survey visit required for home care. I did the 4 week live class which was definitely beneficial. She gives you the steps to properly set your business up."

Homecare for newbies · Dovelene Williams a year ago (operator) · ↑10 reactions ·
<https://www.facebook.com/groups/706524484973799/posts/1016591983967046>

"Thanks for being so open about this – early days are tough for most of us in domiciliary care. What worked for me was building strong relationships locally rather than relying too much on ads. Start with: *Social workers & hospital discharge teams (visit in person if you can)* Local GP surgeries *Community groups / churches* Also, word of mouth becomes your biggest asset, so even 1–2 clients done really well can snowball. I'd also suggest checking your Google presence (reviews + local SEO) as families often search there first now. Don't lose momentum – 10 months is still early." ——— Straightforward + Honest "I'll be honest – Facebook ads and directories rarely bring consistent care clients. Most of our growth came from relationships, not marketing spend. If I were starting again, I'd focus on: Knocking on doors (GPs, pharmacies, community hubs) *Building trust with social workers* Getting your first 5 clients and delivering exceptional care Once your reputation builds, referrals follow. It's slow at first but much more reliable." ——— Encouraging + Relatable "You're definitely not alone – a lot of us struggled in the first year. What changed things for me was shifting from 'online marketing' to 'being visible in the community.' People need to trust a care provider, and that usually comes from relationships, not ads. Also worth asking – what makes your service different? That clarity really helps when speaking to families and professionals. Stick with it, you're closer than you think." ——— Engaging (to spark conversation) "Out of curiosity – what area are you based in? A lot of growth strategies depend on your local commissioning setup."

Care Managers Network UK - CQC Registered Professionals · Dewan Choudhury (operator) · ↑7 reactions · <https://www.facebook.com/groups/1971829479945179/posts/250346596678152...>

"Looking for marketing ideas! . I have started my own home care agency in Alberta, Canada. I am looking for great ideas on how to get my first clients. I am currently doing a lot of in person visits to various places such as independent living facilities, seniors centers, pharmacies, doctors offices,etc. Besides the in person stuff, we have our website, some social media (looking at having it professionally managed). Any helpful insight is appreciated!"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...

"Looking for marketing ideas! I have started my own home care agency in Alberta, Canada. I am looking for great ideas on how to get my first clients. I am currently doing a lot of in person visits to various places such as independent living facilities, seniors centers, pharmacies, doctors offices,etc. Besides the in person stuff, we have our website, some social media (looking at having it professionally managed). Any helpful insight is appreciated!"

r/RunAHomeCareAgency (operator) · ↑7 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...

A Place for Mom — 42 buyer documents (40 operators)

The most-discussed paid source. What operators say it costs: \$50–68 per lead, \$580 prepaid for ten referrals, billed per referral whether or not the family signs, cancellation described as hard, credits contested. What they say it returns: 'nobody ever answered', '25 referrals in two months, 2 onboarded; 15 leads this month, none closed', 'the same client given to 10 other agencies'; the defenders: 'closed 2 out of 10, making more weekly than I paid', 'I got 3 clients through them, it's about how you market yourself', 'it worked out for me in central Nebraska'. Read as: it can work for an owner with instant follow-up and a closer's temperament; the average owner in the group experiences it as paying to compete for a family who never picks up.

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads. Initially, it was to be a few hrs a day. I drove over 30 minutes to complete the assesment, only to be turned away. I sat with our now client and her cousin, gave her some safety tips, and left our information with her. I then followed up with the cousin who initiated the contact via email, thanking her for the opportunity and letting her know that we are only a call away. Fast forward, one month later, on Oct 30, i received two back to back calls from the same cousin telling me that our now client was in the ED and could not go home unless there was a home care agency that would take her. Otherwise, she'd be sent to a TCU or nursing home. Cloent said she I agreed to the case, and confirmed a next day asesment. The cousin stated the client was only interested in a few hrs a day. Upon completing my assessment, I informed the client that she required around the clock. She wasn't happy but she signed up. All that to say, yes APFM is high risk, however as many say, the buy-in is in the followup."

Homecare for newbies · Dumah Darling (operator) · ↑13 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/1130570192569224>

"Has anyone ever had a A Place for Mom actually reach out to them? They contacted me today, signed me up, waived all my fees, and even set me up in the portal to start receiving private pay clients. I usually hear people say 'A Place for Mom ain't it,' so now I'm curious... how's it going for you?"

Homecare for newbies · Anonymous participant (operator) · ↑13 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

"Anyone received a call from a place for mom? What are your thoughts they said they found my business on the licensing web and want to see if I need clients"

HOME CARE BUSINESS OWNERS (operator) · ↑13 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/273192195049247...>

"I used place for mom and care in homes. They're not a waste to me. I got my first private pay client that turned into 24HR care. You will def need a system in place for the leads to follow up so you can convert them over. Yes some leads doesn't answer,... See more"

Homecare for newbies · Yasmine Walker (operator) · ↑11 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

Placement agents / referral agencies (CarePatrol, ALL, Oasis, independents) — 39 buyer documents (38 operators)

Two voices at once: the home care and RCFE owner who is 'way too dependent on the placement agents that have been bringing me clients' and would like to stop paying a month's rent per placement, and the placement agent herself asking how to get families. The Washington State placement-agency list post (a medical-supply salesman's) shows how facilities actually try to reach agents: mass emailing the list, which the poster warns against. For senior living the placement agent is the incumbent channel the ads must displace; for the independent agent she is a second buyer of the same engine.

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

"Finally licensed!. Finally licensed here in Sacramento, Roseville area and hired my first two caregivers. If anyone needs help with anything at all please let me know. I feel like an expert, at least with the licensing and start up stage. I will keep everyone posted as I go along! So many people have messaged me about this, I am creating a free licensing forum on my Skool platform. Please join me there, I am hardly on here. [Update: My Skool community blew up! And I made \$1.1Million in revenue my first year. It is now March and we are already at \$250K for the year. I have since went to paid for Skool to continue providing valuable resourses with the live QAs from industry professionals and future guests. There are 5 modules everything from email domination for placement agents that got me 3 placement agents (I sent it to 36 agent total), how to hire caregivers and all the marketing things I did."

r/RunAHomeCareAgency (operator) · ↑10 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

"Finally licensed! Finally licensed here in Sacramento, Roseville area and hired my first two caregivers. If anyone needs help with anything at all please let me know. I feel like an expert, at least with the licensing and start up stage. I will keep everyone posted as I go along! So many people have messaged me about this, I am creating a free licensing forum on my Skool platform. Please join me there, I am hardly on here. [Update: My Skool community blew up! And I made \$1.1Million in revenue my first year. It is now March and we are already at \$250K for the year. I have since went to paid for Skool to continue providing valuable resourses with the live QAs from industry professionals and future guests. There are 5 modules everything from email domination for placement agents that got me 3 placement agents (I sent it to 36 agent total), how to hire caregivers and all the marketing things I did."

r/RunAHomeCareAgency (operator) · ↑10 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

"That depends on the type of service the referral agency offers . Do they mass fax a potential clients info to 40 facilities blind and have the faculty do ALL the work of setting up tour, following up with family OR do they do a thorough assessment of client, vet each place they refer to and hand hold the client/ family through every step of placement and check in after to make sure it's the right fit ? One in my option deserves payment the other not at all and can turn out to be a burden to families with mass calls from facilities ."

Senior Care Networking · Nichole Casado (operator) · ↑6 reactions · <https://www.facebook.com/groups/seniorcarenetworking/posts/20421835258...>

"Anyone know how to digital market for senior care?. I've become way too dependent on the placement agents that have been bringing me clients. Im looking to see if anyone has tried digital marketing or run ads? Update: I started running ads and even though I have not gotten a single client i got a ton of caregivers and everyone in our community started to recognize us. Because of this by product of running these ads we have started to become known. Win? Sure, I'll take it. I am posting my experience with digital marketing on my Skool platform. The first 7 days are free and then it a monthly subscription to be apart of my community. ["

r/RunAHomeCareAgency (operator) · ↑5 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/1ffa065/anyone_kn...

Insurance panels / pediatricians / schools (ABA) — 25 buyer documents (23 operators)

ABA is a different animal: the constraint is credentialing and panel access, then pediatrician and school referrals, then the waitlist you cannot staff. 'I'm credentialed with all the insurance companies that currently have their panels open and I'm still stuck on referrals' is the ABA version of 'my phone isn't ringing'. Marketing to parents directly is barely discussed in owner voice; the operator's own consumer ad in Vince's feed (Georgia Behavior Associates: 'No Waitlist · Insurance Covered · Katie Beckett Waiver Accepted') shows the hooks that work on the family.

"I started a private practice AMA I'm a BCBA that went fully independent in 2021. I do all the aspects of business and clinical myself - including credentialing, billing, marketing, and provide all clinical services myself. I'm in CA. Ever wanted to go your own way? AMA!"

r/bcba (operator) · ↑66 upvotes · https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...

"I know you're an AIT now, but consider this when you become an Administrator. When a building reaches 100% occupancy, it's a fantastic feeling for the team. Here's what you can do to boost census and enhance patient care: Partner with local hospice and home health agencies for steady referrals. Build strong personal connections with local hospitals to become their provider of choice. Update insurance contracts to accept a wider range of plans. Establish contracts with local prisons for specialized patient care. Add specialized service lines like dementia, HD, trach, and vent units. These strategies have not only increased census but also improved overall service quality. If you're looking to grow your facility's census, these tips might help you too!

HealthcareLeadership#LTC#CensusGrowth#PatientCare"

Administrator In Training AIT · Demetrius Kirk (operator) · ↑45 reactions · <https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

"I have been a BCBA for over 15 years and I have decided to start my own business/clinic. I already have my LLC, and I am credentialed with TriWest and BCBS. I am planning on starting out with just 2 RBT's and myself and grow as appropriate. My plan is to have a relatively small clinic and hope to do the billing and manage referrals/reauthorizations myself. I need mentoring on billing, general business set up, referral/authorization process (I know how to write ITPs & PRs) with Triwest, BCBS, and Aetna in Texas specifically. I have posted before, but I got recommendations for companies that DO the billing, etc. for you -- I need coaching on how to do it myself. Any recommendations would be appreciated!"

ABA Business Owners · Anonymous participant (operator) · ↑5 reactions · <https://www.facebook.com/groups/669886889110965/posts/951864540913197>

"Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

ABA Business Owners · Anonymous participant (operator) · ↑4 reactions · <https://www.facebook.com/groups/669886889110965/posts/942369488529369>

"Struggling to get referrals for ABA company?. Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

Non-BCBA Owners ABA Business Collaborative · Anonymous participant (operator) · ↑3 reactions · <https://www.facebook.com/groups/1081018657046251/posts/143052813209530...>

Google Ads / LSA / PPC — 17 buyer documents (16 operators)

Thin and contradictory in buyer voice: 'the least effective but the most expensive' vs '\$6 per lead and my close rate is pretty high'. The owners asking for 'a marketer who knows home care' want Google Ads plus Google Business Profile plus SEO in one person. The senior-living marketer voices (vendors, kept out of the buyer count) describe local SEO + PPC + Performance Max + Facebook retargeting as the standard stack and warn about Fair Housing targeting limits.

"Hey everyone! Can anyone recommend a really good marketer who knows the home care industry? I'm looking for someone who can help with Google SEO, Google Ads, getting my agency to rank higher on Google, improving my Google Business Profile, and bringing in more qualified leads. If you've personally used someone and had a good experience, I'd love their contact information. Thanks in advance!"

HOME CARE BUSINESS OWNERS (operator) · ↑22 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/291140860921047...>

"Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

ABA Business Owners · Anonymous participant (operator) · ↑4 reactions ·

<https://www.facebook.com/groups/669886889110965/posts/942369488529369>

"Struggling to get referrals for ABA company?. Hi everyone, I could really use some insight from those of you who've been through the early stages of building your ABA company. I've been up and running for about a year now and I'm feeling a bit stuck when it comes to referrals. I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach. I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads. Despite all of that, I'm still sitting at zero clients. I did have one child ready to start, but they ended up choosing a center last minute. At this point I'm paying my BCBA out of pocket with no incoming revenue, so I really want to make sure I'm not missing something obvious or doing this the hard way. For those of you who are getting steady referrals: What actually worked for you in the beginning? Where do most of your referrals come from now? Is there something you did that made a big difference that I might not be thinking of? I'm open to any honest feedback or suggestions. I know this takes time, but I just want to make sure I'm moving in the right direction. Thank you in advance"

Non-BCBA Owners ABA Business Collaborative · Anonymous participant (operator) · ↑3 reactions ·

<https://www.facebook.com/groups/1081018657046251/posts/143052813209530...>

"I feel like they charge too much. Just make yourself a good Google page- Photos and all. Continue running a small ad in Google Ads- say \$3 a day. (Up it to \$15+ when you are about to have a vacancy.) When full, just go back to \$2-3 a day, just to keep your name out there!"

Senior Placement Network · Grandmother's House (operator) · ↑3 reactions ·

<https://www.facebook.com/groups/2447813518868667/posts/439960940368905...>

"I may be able to help, depending on what you're looking for. One thing I've seen working with home care agencies is that many invest heavily in Google Ads before fixing the foundation. If your Google Business Profile isn't fully optimized, your local SEO is weak, or your website isn't built to convert visitors into inquiries, you can end up paying for clicks that never become clients. A good strategy combines: • Google Business Profile optimization • Local SEO so you rank in your service area • Google Ads with the right targeting • A website designed to convert visitors into calls • A referral strategy that complements online marketing If you're open to it, I'd be happy to share what's worked for agencies I've worked with or point you in the right direction."

HOME CARE BUSINESS OWNERS · Chantelle Teasdell (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/291140860921047...>

VA / Medicaid waiver / LTC insurance contracts — 16 buyer documents (14 operators)

"I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources:* Social service agencies *Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones."**

r/RunAHomeCareAgency (operator) · ↑24 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxy/i_put_toge...

"Hey so im located in Michigan and I just got awarded two contracts one with DWIHN and Senior alliance. Which is the medicaid waivers for IDD and AMI and Seniors. What ive learned is that every insurnace payer for contract has a different rate that they will pay. DWIHN pays \$43 you can start your dcw off at \$16-\$17 an hr and make \$27-\$26 an hour or you can hire a CNA and pay them \$20 and still make \$23 an hr Senior alliance pay is lower for CLS They pay \$23 an hour so at most i can pay staff is \$15 no more than \$16 For nursing services they pay \$61 for RN and \$57 for LPN so hiring a RN any where from \$45-50 an hour you can make \$15-10 profit an hr in profit. I havent gottten any private pay clients yet but thats next on my list"

Homecare for newbies · Krys Krystal a year ago (operator) · ↑6 reactions · <https://www.facebook.com/groups/706524484973799/posts/1052048583754719>

"So.... I just started in FL (like 8 weeks 8into this). And I only have 1 private pay client and she's 24 hour care.... but after I pay my staff office expenses and software (I use Ally/CareTime) I make about \$750/week... Of course my goal is 5-10 clients. And I'm stuck on how to become an insurance provider esp managed care. I have my NPI but idk what to do next..."

Homecare for newbies · Traneka Rashell a year ago (operator) · ↑4 reactions · <https://www.facebook.com/groups/706524484973799/posts/1052048583754719>

"How do resident referrals work for ALF owners including individual approval and compensation. ALF owners/operators — I'm a nurse and my next move is becoming an ALF owner, so I'm trying to learn more about how referrals actually work from the owner side. Can an individual person be approved to refer residents to an ALF? If so, what usually qualifies them? If there is compensation, what does that normally look like for private pay vs Medicaid waiver residents? Also, are some referral relationships simply based on networking and maintaining good business relationships with no referral fee involved? Like checking in, bringing flowers, lunch, etc. Just trying to understand how this part of the business actually works. Thank you"

Assisted Living Investment Group (ALIG) · Skye Babay (operator) · ↑3 reactions · <https://www.facebook.com/groups/2886092941628251/posts/451427707880982...>

"Not Oakland County but I think Angel gardens takes the medicaid waiver and I also think there is an American house in Livonia that also takes the medicaid way"

East Meets West Through Networking · Becky Sinai Eizen a year ago (operator) · ↑1 reactions · <https://www.facebook.com/groups/emwtn/posts/1708249326496574>

Franchise systems (Home Instead, Visiting Angels, Right at Home, CarePatrol...) — 13 buyer documents (11 operators)

"I own a non-medial homecare agency. We are in the back 6 months of our 3rd year of operation and reached 7 figures in our 2nd year for gross income. We have found high turnover rates occur at the beginning of operations. Once you're settled with a decent book of business, you're able to find good employees who then refer their friends. Our turnover rates are closer to 50%. We are not certified with Medicaid and we are part of a franchise. I run the business with my wife and one other administrator and we have about 40 employees working part time and full time. Feel free to ask questions."

r/Entrepreneur (operator) · ↑2 upvotes · https://www.reddit.com/r/Entrepreneur/comments/1rqjot3/pe_is_dumping_b...

"I'd be curious what franchise.... They should be able to answer this question and provide the support and resources to get over the lull you are experiencing."

HOME CARE BUSINESS OWNERS · Kerin Staup Zuger a week ago (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/294413039260496...>

"Hi Chelsey. I'd be happy to speak with you more. Feel free to complete our franchise form online and we can schedule a call!"

YouTube · @KrisChana (operator) · ↑1 likes · <https://www.youtube.com/watch?v=Ws6nhEzQvEw&lc=Ugz6l2duHQvQ4jwDwVB...>

"Nice Try - But falls far short of minimum needs for Home Care businesses. We're required to use ClearCare within our home care franchise system. We used to use another very good software called HomeTrak. We embraced ClearCare about 2 years ago when our franchiser told us that they had tested it and that we were going to switch to it because it offered many advantages that were expected to improve our quality and reduce operational costs by automating and simplifying. After having used it for nearly 2 years, my opinion of it is that it isn't yet even good enough to have been released for first use, and that we should have been getting paid to help develop it (rather than paying to use it) for these past 2 years. Our issues with it are numerous and it seems to me that ClearCare prefers to focus on introducing more options for it rather than fixing its problems. Perhaps it's because of the horrible contract that we had to sign that makes us have to use and pay for it for 5 years - with no way out. Clearcare's shortcomings are extensive and I can't begin to list all of them, but I'll share some so readers get a feel - Many of the features promoted as making it unique aren't of value because clients aren't interested in using them, or there is some functional reason why they don't work out to actually be used. For example, we have only a very small percentage of clients that want to use the Family Room and those that do create a lot of extra work for us because they look at it constantly and come-up with questions simply because a change wasn't made quick enough before they logged on, and similar. For those few that use it, it actually causes more problems than what it does good. Other features such as e-mail and text notifications to caregivers require that caregivers pay for cell plans with lots of data capability, but many don't want to, and stop the messaging making the function far less value than promoted. - Cost is much higher than our prior software yet overall, the extra work we must put into making it function okay requires more time, adding more additional costs. (There are some things it does well such as some of the scheduling-related features, and a lot of the report capabilities are nice). But many areas such as invoicing, care plans, etc. are cumbersome with output that isn't in my opinion good enough to be considered acceptable. - Terms of the contract with ClearCare are horrible and unacceptable. No one in their right mind would agree to them. I signed only because of being required by our franchiser and because I trusted our franchiser to do what was right and necessary. - Clearcare has a clause in our contract with them that says that we as customer exclusively own all rights, title, and interest in and to all customer data, yet regardless, they turn-around and hand client protected data over to others without our permission. That is in violation of our State's home care regulations and probably all other states as well. - Our conversion to ClearCare was rushed and support at the time was lacking. We ended-up with significant, costly problems such as over a month period where we had to do our payroll manually because the payroll report output from ClearCare was unintelligible. Subsequently there were ongoing issues with it paying people incorrect OT, and more that were due to the payroll report provided to us, not because of anything we did incorrectly ourselves. - Care plans produced by ClearCare are confusing, overly long, don't have good ways to enter much of the important details... - ClearCare keeps coming-up with new options for sale to us, but seems to do very little to address obvious issues with the base software. - Even little things such as a place to include referrer's telephone extension numbers doesn't exist. There are work-arounds, but these little things add up to a lot of complication, confusion and become ridiculous. With all of this being said, there are some things that ClearCare does very well, including the functionality of scheduling, and many reports it generates. But the overwhelming number of shortfalls far outweigh the good qualities. Between the added cost in time, the monthly cost, and the big problems with start-up, I feel we're in the hole 10's of thousands of dollars vs where we'd be if we would have just kept using our prior software."

trustpilot (operator) · <https://www.trustpilot.com/reviews/56b24d800000ff000937464f>

"Eldercare in this country is one of the nastiest money-making industries since slavery. *Unfortunately – there are stories like this about seniors who reside in nursing homes....assisted livings....and even there are instances of private Caretakers taking advantage of seniors. There are many scams that happen door to door and over the phone that can affect seniors. Unfortunately mistakes can happen and this is sad but there are good facilities out there that care for seniors and Home Instead is a good company. It is important that families be involved in the care of their loved ones so that these instances can hopefully be prevented or these predators can be caught!* My husband has Alzheimer's and I hired a caregiver from an agency. One of the first questions I had for the agency was...."Do you do background checks." Well, to make a long story short, the second time I had the caregiver over to the house, she sent my husband downstairs,(We live in an apartment on the second floor.) by himself to get her coffee!!! She then stole jewelry and gift cards from me!!!! While my husband was out of the apartment!!!! You better believe I called the police and reported it, I also contacted the Human Services agency. I do hope for the best that they catch her and send her to jail. One can only hope! The reason I know this.....I came home early and saw my husband, by himself in the hallway. I will NOT ever get help from that agency again and I will alert everyone of the terrible agency that is. I'm having locks put on ALL bedrooms and I'm getting a surveillance system in the house in the future. I do want to know what goes on when I'm gone. It is definitely the only way to go, sad, but true! * I own a company that cares for seniors. It is a big responsibility and requires owners to work day and night overseeing get their company. The biggest problem is see....when a caregiver who has a squeaky clean background is suspected to have been dishonest (over 15 years I have suspected this to have happened 5 times). The very first thing I do is suspend my caregiver and call the police. I will not tolerate dishonesty! In each instance, the police did not find any illegal behavior. Well, I did not feel comfortable with suspecting my caregiver being dishonest. So I terminated all 5. I had to pay unemployment (which is better than placing them with another client) and one threatened a lawsuit."

unhappyfranchisee (operator) · <https://www.unhappyfranchisee.com/home-instead-senior-care-franchise>

Pay-per-lead / lead-gen vendors — 11 buyer documents (11 operators)

Prices written by owners: \$6, \$25, \$50, \$58, \$68, \$300 per lead. The burn language is the strongest of any source ('run as fast as you can', 'out of 10 paid leads I got one client who needed two visits'). The one structural complaint is shared leads: 'most lead generation companies send referrals out to all the agencies in the area.' Exclusive leads is the phrase the vendors now advertise against it (Occupancy Partners: 'exclusive leads generated and booked onto your calendar').

"A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined their partnership request the gentlemen got so upset with me, it literally felt like I was on the twilight zone!! \$58/per "possible" lead, that is sent to 4 other agencies is a complete scam and is not good business practice and I told him that, he was so stuck on trying to sell me a marketing product that he neglected to even understand our mission as an agency!! They will get their money regardless, roughly \$300 per lead.. WE will not and I spoke on that as well. For kicks and giggles, I also asked how our agency will stand out amongst others?! No response to that question.... Smh, the call was a disaster, in real time. But he had the right one bc lk what questions to ask. Ultimately his approach was not it. And it showed his character and what he was truly after..He did not want to hear about the relationship building or community work that we do... His mind was simply set to sell... We would never do business with any organization that does not put client care needs as priority. They do not care about the agency practices all. They just want your dollars!! Agencies and families should be aware of this!"

HOME CARE BUSINESS OWNERS (operator) · ↑51 reactions · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

"It's hard to make it make sense. You are paying \$60 for a lead that's being shared with 5 or more other agencies at the same time, so your likelihood of closing on any of them is extremely low. A much better approach would be to learn how to generate and nurture your own leads through Google and Meta ads and other lead generation approaches, so the lead is all yours with no competition. My ads typically cost me around \$6 per lead and my close rate is pretty high. Hands down the best decision I've made for my agency. Hope this helps."

HOME CARE BUSINESS OWNERS · Edward Davis (operator) · ↑9 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/288229603545506...>

"Another question for my fellow agency owners: Have any of you used a referral source or lead generation company for private pay clients that you found to be effective and reasonably priced? I'm looking for recommendations that don't cost an arm and a leg. I'd love to hear what has worked for you and what you'd avoid."

HOME CARE BUSINESS OWNERS (operator) · ↑7 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/286769206024879...>

"I'm sorry you experienced this. Unfortunately, a lot of lead companies sell the same lead to multiple home care agencies, so you end up paying to compete with several companies for the same family. I wouldn't let this discourage you, especially since you've already proven you can find a client on your own. At only 4 months in, I would focus less on buying leads and more on building your own referral pipeline. Start developing relationships with senior communities, rehab/SNF social workers, elder law attorneys, geriatric care managers, hospice companies, physicians, churches, senior centers, and other professionals who regularly encounter families needing care. Also make sure your Google Business Profile and social media clearly show what makes your agency different and exactly who you serve. Private-pay marketing takes consistency and follow-up, it's usually not one visit, one flyer, or one phone call. You want referral sources to start recognizing your name and trusting you enough to recommend you. If you'd like, I help newer home care owners build a step-by-step marketing and referral strategy so they know exactly who to target, what to say, and how to follow up without wasting money on shared leads. Feel free to message me."

Home Care Business Owners Tools · Tina Hemmings (operator) · ↑2 reactions ·
<https://www.facebook.com/groups/homecareownersunite/posts/107527837501...>

"I stop using a place for mom and caring.com their leads are expensive and no results. I am just using one at the moment Care In Homes their leads are \$25 per lead. But try using other method to get clients."

HOME CARE BUSINESS OWNERS · Sher Lee (operator) · ↑2 reactions ·
<https://www.facebook.com/groups/2156388261379184/posts/266158858419248...>

Facebook / Instagram / Meta ads — 10 buyer documents (9 operators)

The one owner-run test on record produced caregiver applicants and community recognition, not a client ('I have not gotten a single client, I got a ton of caregivers'). Another owner says half her referrals come from her own Facebook page marketing plus brochures in dispensaries, grocery stores and doctors' offices. Read as: Meta reaches the family and the caregiver in the same feed; the offer and the landing page decide which one calls.

"I honestly created my own ad using Canva and pay for the marketing on my Facebook page. My referrals have come from Facebook marketing 50% and I have placed home health brochures at dispensaries here, grocery stores, dollar general, and doctors offices. I have a brochure and a meet the owner postcards at the places I listed above. Everybody's results will be different but there are many options to try."

Homecare for newbies · Nyeshia Jennings a year ago (operator) · ↑11 reactions · <https://www.facebook.com/groups/706524484973799/posts/976062864686625>

"It's hard to make it make sense. You are paying \$60 for a lead that's being shared with 5 or more other agencies at the same time, so your likelihood of closing on any of them is extremely low. A much better approach would be to learn how to generate and nurture your own leads through Google and Meta ads and other lead generation approaches, so the lead is all yours with no competition. My ads typically cost me around \$6 per lead and my close rate is pretty high. Hands down the best decision I've made for my agency. Hope this helps."

HOME CARE BUSINESS OWNERS · Edward Davis (operator) · ↑9 reactions · <https://www.facebook.com/groups/2156388261379184/posts/288229603545506...>

"Thanks for being so open about this – early days are tough for most of us in domiciliary care. What worked for me was building strong relationships locally rather than relying too much on ads. Start with: *Social workers & hospital discharge teams (visit in person if you can)* Local GP surgeries *Community groups / churches* Also, word of mouth becomes your biggest asset, so even 1–2 clients done really well can snowball. I'd also suggest checking your Google presence (reviews + local SEO) as families often search there first now. Don't lose momentum – 10 months is still early." — Straightforward + Honest "I'll be honest – Facebook ads and directories rarely bring consistent care clients. Most of our growth came from relationships, not marketing spend. If I were starting again, I'd focus on: Knocking on doors (GPs, pharmacies, community hubs) *Building trust with social workers* Getting your first 5 clients and delivering exceptional care Once your reputation builds, referrals follow. It's slow at first but much more reliable." — Encouraging + Relatable "You're definitely not alone – a lot of us struggled in the first year. What changed things for me was shifting from 'online marketing' to 'being visible in the community.' People need to trust a care provider, and that usually comes from relationships, not ads. Also worth asking – what makes your service different? That clarity really helps when speaking to families and professionals. Stick with it, you're closer than you think." — Engaging (to spark conversation) "Out of curiosity – what area are you based in? A lot of growth strategies depend on your local commissioning setup."

Care Managers Network UK - CQC Registered Professionals · Dewan Choudhury (operator) · ↑7 reactions · <https://www.facebook.com/groups/1971829479945179/posts/250346596678152...>

"We've seen a lot of success for our Senior Living clients with a combined strategy of local SEO, PPC, Performance Max and a combination of Facebook Ads and Facebook Retargeting! One thing to keep in mind is Fair Housing guidelines on Facebook when running ads- your targeting will be restricted to 25 miles around the city the facility is located in and you can't have any additional targeting. We have still seen these ads perform exceptionally well!"

r/SeniorLivingMarketing (operator) · ↑1 upvotes · https://www.reddit.com/r/SeniorLivingMarketing/comments/1l4a6ip/how_to...

"I tried the service and the leads do come but like mentioned above, you are competing with so many other companies both small and large. I tried it for about three months and it just wasn't worth the money to me because I never got one client from the time I used them. It may work for others, but it just wasn't a good fit for me and I was losing money paying them. I'm about to go the route that was mentioned above via Facebook ads and Google."

Homecare for newbies · Shemeka Doyle-Green a year ago (operator) · ↑1 reactions · <https://www.facebook.com/groups/706524484973799/posts/738060561820191>

Caring.com — 8 buyer documents (8 operators)

Named far less than A Place for Mom; the tone is the same ('washed up leads', 'too many calls') with one owner reporting CareInHomes at \$25 a lead producing 4 private-pay clients and one LTC client. One group comment dates the decline: 'lead quality dropped a lot in the last 18 months and the cost per converted client stopped pencilling out; most owners shifted into discharge planner relationships.' Caring.com was acquired by SilverAssist (Oasis Senior Advisors' parent) per an advisor's post.

"CreativeLychee1297 sometimes it takes a little bit longer to get private pay clients. Most people do pay for leads at Places like A Place For Mom or caring.com. But, if you're looking for private pay, I suggest you think outside the box."

HOME CARE BUSINESS OWNERS · Real Is to CreativeLychee1297's reply (operator) · ↑2 reactions · <https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

"I stop using a place for mom and caring.com their leads are expensive and no results. I am just using one at the moment Care In Homes their leads are \$25 per lead. But try using other method to get clients."

HOME CARE BUSINESS OWNERS · Sher Lee (operator) · ↑2 reactions · <https://www.facebook.com/groups/2156388261379184/posts/266158858419248...>

"For those of you working in senior placement or senior living, have any of you had success using Caring.com, A Place for Mom, or similar referral services? We're a small independent shared living home in Idaho and have been building relationships with local placement agencies, but I want to make sure I'm not overlooking other effective referral sources. Are there any platforms, directories, or lead sources that you've found particularly valuable? Thank you in advance for any insight!"

Senior Placement Network (operator) · <https://www.facebook.com/groups/2447813518868667/posts/439960940368905...>

"I got 3 clients through a place for mom. They send you leads so it's all about how YOU market yourself. These people need help, how else are you going to find leads in the beginning? I believe it's worth the money, it's pricey but that is what you have to take with any business it takes money to make money. Now I'm signing up with caring.com and they do marketing as well"

Home Health Care Networking · Sandra Williams (operator) · <https://www.facebook.com/groups/2503674136547361/posts/319690059389137...>

"Using Caring.com now. I definitely get leads, I'd say it's 60/40 on whether they answer or even show interest. It's almost like why did you call of you weren't going to be receptive to entertaining calls? I can't say it isn't worth it... but it is a challenge."

Home Health Care Networking · Nikkia Shantel (operator) · <https://www.facebook.com/groups/2503674136547361/posts/319690059389137...>

Yelp / Angi / HomeAdvisor / Thumbtack / Nextdoor — 6 buyer documents (5 operators)

"I got my clients by calling nursing homes and posting in local Facebook groups/ Nextdoor."

HOME CARE BUSINESS OWNERS · Ja Nay (operator) · ↑19 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/284819363219864...>

"We have a total of 6 clients so putting it in percentages will not be helpful. I'll give you some hard numbers. Outlay: \$250 on logo creation \$2500 to build website (including photo shoot and video) \$500 on SEO and Google maps setup (currently with 3 reviews) \$600 for design and printing of 500 brochures \$400 to join the local Chamber of Commerce for networking \$0 for Facebook, Nextdoor, passing out brochures, visiting care facilities Conversion: 3 clients from Facebook group post 1 client from Nextdoor post 1 from networking 1 from word of mouth It's important to note that each of the above conversions from free activities was undoubtedly strongly enhanced by a good website and Google presence. Finding high-quality caregivers has not been easy as well. Our website has been particularly helpful with that. For reference, we have been in business for 6 months and are grossing \$20,000 per month now. We have 4 caregivers, not including my partner, myself, and my son, who all work with clients, too."

r/RunAHomeCareAgency (operator) · ↑3 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dj6i94/marketing...>

"Our neighborhood has a Facebook group, and I am a member. I put a post up to the group saying I was starting a new home care agency and would offer a discount to anybody in the neighborhood. Nextdoor is also organized by neighborhoods, so I copied the same post for it."

r/RunAHomeCareAgency (operator) · ↑1 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dj6i94/marketing...>

"What do you mean by Facebook group post or Nextdoor post? Eg like are you posting in there saying hey all try us we are a home are agency or what Facebook groups are you targeting?"

r/RunAHomeCareAgency (operator) · ↑1 upvotes · <https://www.reddit.com/r/RunAHomeCareAgency/comments/1dj6i94/marketing...>

"Mattie Gaffney I've with a place for mom , yelp and other places just fake leads"

Homecare for newbies · Felicia Oke to Mattie Gaffney's comment a year ago (operator) ·

<https://www.facebook.com/groups/706524484973799/posts/879065094386403>

CareInHomes / Aging Care / other lead directories — 4 buyer documents (4 operators)

"I used place for mom and care in homes. They're not a waste to me. I got my first private pay client that turned into 24HR care. You will def need a system in place for the leads to follow up so you can convert them over. Yes some leads doesn't answer,... See more"

Homecare for newbies · Yasmine Walker (operator) · ↑11 reactions ·

<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

"I stop using a place for mom and caring.com their leads are expensive and no results. I am just using one at the moment Care In Homes their leads are \$25 per lead. But try using other method to get clients."

HOME CARE BUSINESS OWNERS · Sher Lee (operator) · ↑2 reactions ·

<https://www.facebook.com/groups/2156388261379184/posts/266158858419248...>

"I do careinhomes. I got 4 private pay clients and one long term care"

HOME CARE BUSINESS OWNERS · PattyLabelle Lozier (operator) · ↑1 reactions · <https://www.facebook.com/groups/2156388261379184/posts/283561427678990...>

"HOME CARE BUSINESS OWNERS · Lotus Senior Care · July 7 · So, I was just on the phone with Careinhomes asking about the referral program they offer, and the guy told me that I can start with the lowest tier to get started but then wants to tell me how many zip codes I need to use for it to work. If you're only starting with 5 leads you don't need 15 zip codes and asked if I worked with a place for mom before because they are more expensive and I won't get far with tier 1. I said yes and I didn't use a lot of zip codes with them also and generated a lead that I closed. He said well you should go with them then. I was so taken back by his response. I couldn't believe him. You can't determine one's destiny only God knows that"

HOME CARE BUSINESS OWNERS | So, I was just on the phone with Careinhomes asking about the referral program they offer, and the guy told me that I can start with the lowest tier t... · 70.0K members · Join group (operator) · <https://www.facebook.com/groups/2156388261379184/posts/288757708159362...>

Marketing agencies / consultants — 4 buyer documents (3 operators)

"Yes, I did, I went through Legal Zoom which was pretty easy but you have to stay on top of it. A year later, I hired a consultant who happened to look up my LLC and saw that one of my forms had expired. You can try legal Zoom but bear in mind that you are responsible for maintaining it. Or hire someone to do it for a fee. I have a guy who helped me dissolve my other LLCs if you want I can get you his info."

r/RunAHomeCareAgency (operator) · ↑2 upvotes · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

"I'm going to say something that might rub a few people the wrong way. If your assisted living still has empty beds month after month, there are things you're either not doing or not doing consistently. I know this because we made just about every mistake you can make. I'm not a marketing company. I'm not a placement agency. I own a CBRF with 10 homes and 94 beds. For years, we consistently had anywhere from 5-15 empty beds, regardless of how hard we worked. Now we're sitting at 94/94 occupied, we have a wait list, and our team is stable. On August 20th, I'm hosting a free Zoom with my RN Administrator and one of our ownership partners. We're going to walk through exactly what we do today and the mistakes we made during our first 4 years that kept us stuck. Nothing for sale. Just real conversation. If you want to join us, comment "ZOOM" below and I'll message you the link. Facebook sometimes sends messages from people you aren't connected with to your Message Requests folder. If you comment "ZOOM," keep an eye on your Message Requests in case my message lands there."

Home Health Care Agency Owners & Entrepreneurs (operator) · <https://www.facebook.com/groups/1154838852454654/posts/171456237648229...>

"Check this out!!!! 8 referrals in one day. 4 referrals in less than 24 hours from a single email. A fax to SNFs that produced a referral in 2 hours. Brand new home licensed... referrals already coming in. This isn't hype. I currently operate 94 beds. All 94 are full. And what's happening inside these screenshots is the same system we use to keep them full. No paid ads. No fancy marketing agency. No magic. No crazy marketing budget needed. Just consistent execution: • Identify referral sources • Build the list • Follow up weekly • Track the numbers Next Wednesday at 7 central on zoom we're breaking down exactly how we do it. This is the last time I am going to do this free training for a while. If your occupancy isn't where you want it to be, you need to be on this Zoom. Comment FULL and we'll send the link."

ADULT FAMILY HOME GROUP OF WASHINGTON (operator) · <https://www.facebook.com/groups/448686879807645/posts/1642422813767373>

"I lost my marketing job in Covid and started in Senior Care I was left wondering what I was truly good at... ..and if I could ever succeed when the world was ending. I was 26 and had no job. My friends ridiculed me for even considering starting a small business during COVID. But then, life threw me a curveball that changed everything. I had to find a senior care facility for my aunt, who had severe dementia. The world was in lockdown, and finding a caring, loving place online was almost impossible. I researched for 15 hours... ..and I came across Judson Senior Living. The team, the services, the culture—it all radiated positive energy. It was exactly what I wanted for my aunt. As we talked, she asked me a question that sparked something inside me: "Why don't you help people like me and facilities find each other?" That question changed everything. I realized there was a gap in the market... ..and a need to bridge families searching for quality care with the facilities that provide it. I decided to start my own senior care marketing company and help them find residents. It took me 2 months to find all the senior care facilities across the states and put them in spreadsheets. I started calling 50 facilities every day offering them a powerful digital service to attract more residents Since that moment... I've never looked back. I got energized and pumped every day. I worked NON-STOP. By 2021, I became a founder I look back and cannot remember the old me. Today, I'm proud to help senior care facilities connect with the people who need them most. And through it all, I've discovered what I'm truly good at: Making meaningful connections that change lives through marketing. Moral of the story? Find an intersection of something, some people, some vibe, and some place you love. 3 out of 4 will do. But when they all intersect... Well...? That's when you get powerful. You forget the OLD you. assistedliving MarketingSuccess assistedliving seniorliving growyourcommunity"

Assisted Living Facility And Home Care Services! (pre-launch) ·

<https://www.facebook.com/groups/304213569931127/posts/2264973593855105>

Care.com — 1 buyer documents (0 operators)

"Has anyone had experience hiring hourly caregivers through Care.com? Care recommends that families start communicating via the messaging tool on their website before taking the conversation offline with a caregiver-I've messaged 2 caregivers who either asked for my number right away or gave me theirs to communicate via text-wondering if others have had this experience? Is this because they're busy or dont want to use Care's messaging tool-maybe it doesnt matter-since they have Care profiles, I should still be able to run a paid background check on them. Has anyone paid to use Care's background checks? I know all caregivers get a basic check when making their profile I believe, but I'm looking into the most extensive one called Premium check, as well as possibly the Continuous check for ongoing monitoring. I think the caregiver must agree to these checks in advance. Has anyone had experience checking credentials (such as if the caregiver says that are a CNA or HHA)? Care apparently doesnt check/verify credentials and says the family must do this. I dont even know what a valid certificaion looks like! If anyone has hired thru Care, what were your steps? Did you ask for references and/or a resume (not sure how many folks have a formal resume made up for home caregiver roles?) before asking for a phone/video or in person interview? Did you do the background checks after checking (how many?) references and conducting one interview? Just trying to get a sense of the best steps/process for hiring as it's all on the family if not going thru an Agency. Did anyone add specific insurance (such as bodily injury protection) to their home owners (apartment renters insurance for us) insurance, to protect yourself from liability if the caregiver got hurt while on the job? We also have cameras set up and those will be running-but it's a new experience for us to hire a stranger from online. We'd much prefer word of mouth recommendations (& didnt like going thru agencies due to quality of care we experienced & agencies promising one type of skill/experience but us not getting it-like hoyer lift experience)-but havent had luck with this. Thanks!"

agingcare (pre-launch) · <https://www.agingcare.com/questions/carecom-reviewsexperiences-with-pr...>

Why They Buy — the five answers an ad needs (Ad Brief)

Every line below points at evidence on this page: a measured theme count (buyer-voice documents), a verbatim voice with its URL, or an ad / landing page from the Competitor Ads tab. Nothing here was assumed; where a line is a recommendation it says so.

1. Why they buy

- **The phone is not ringing after the licence. Owners open, hand out cards, visit facilities, and sit at zero clients for months; the first two years are described as the hardest.**
- ‘I am still not landing any leads for my Private Home Care Provider Agency. I have given out my business cards, brochures, sent emails, and gone to outreach community functions...’ (HOME CARE BUSINESS OWNERS, 21 reactions) — <https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>
- ‘Your first two years will be your hardest. I know agencies that opened 2 years now and still no clients.’ — <https://www.facebook.com/groups/2156388261379184/posts/283021973066269...>
- Theme ‘No clients / can’t land the first one’ and ‘The phone isn’t ringing’ on Tab 1
- **They are renting clients from someone else and want to own the inquiry: placement agents, A Place for Mom, a referral partner who goes quiet. Dependence is the fear; ‘families finding me’ is the wish.**
- ‘I’ve become way too dependent on the placement agents that have been bringing me clients.’ (r/RunAHomeCareAgency) — https://www.reddit.com/r/RunAHomeCareAgency/comments/1ffa065/anyone_kn...
- Frequency: ‘Referral sources & placement agents: dependence, fees, how to get them’ is the #3 buyer-voice theme
- Occupancy Partners’ page sells exactly this: ‘You don’t need more leads. You need more move-ins... Exclusive families, never shared’
- **They have already paid for shared leads and been burned, so ‘exclusive’ and ‘never shared’ are the words that move them.**
- ‘\$58 per “possible” lead, that is sent to 4 other agencies is a complete scam’ (51 reactions, 39 comments) — <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>
- ‘Out of 25 referrals the last two months we were able to onboard 2. This month 15 leads and not one closed.’ — <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>
- 25 of 241 operator-facing ads already lead with ‘exclusive / never shared / vs A Place for Mom’ (Competitor Ads → hook patterns)
- **An empty unit or an empty caseload has a number on it, and the vendors who win say the number. Facilities: \$4,000–6,000 a month per empty unit, \$120K lifetime value of a move-in. Home care: ‘five clients last month, this month nothing’.**
- Occupancy Partners feed ad: ‘EVERY EMPTY UNIT is \$4,000 to \$6,000 a month GONE’ (Vince’s feed, IMG_2806)
- PatientsPipeline landing page: ‘You signed five clients last month. This month? Nothing.’
- ‘Today, as I write this, all 94 of our beds are full and we have a wait list’ — the desired state, in an RCFE owner’s words (28 reactions) — <https://www.facebook.com/groups/352285274494478/posts/976832502039749>

- **They want the relationship route (discharge planners, social workers) kept, not replaced; they will buy the thing that fills the gaps between referrals.**
- Lead Sources: ‘Hospital / SNF / discharge planner referral relationships’ is the most-named source and the one owners credit for first clients
- ‘Most owners shifted into discharge planner relationships at the local hospitals, that still works.’ — <https://www.facebook.com/groups/2156388261379184/posts/283561427678990...>

2. Recurring problems and frustrations

- **Paid referral sites: prepaid blocks, billed per referral whether or not the family signs, same family sent to several agencies, aggressive reps, credits refused, hard to cancel.**
- ‘They send you referrals and bill you per referral whether you sign them up or not; that bill grows quickly. Never again.’ — <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>
- Lead Sources ledger: A Place for Mom — burned language in 9 of 42 buyer documents, worked in 8
- **Marketing spend with nothing to show: ads that produced caregivers instead of clients, agencies and coaches that took money, ‘the most expensive and least effective’.**
- ‘I started running ads and even though I have not gotten a single client I got a ton of caregivers.’ — https://www.reddit.com/r/RunAHomeCareAgency/comments/1ffa065/anyone_kn...
- Tab 6 theme ‘Google Ads / Facebook ads / SEO / website’ and Tab 5 ‘Leads & referral sources that wasted money or ghosted’
- **Feast or famine: a referral partner sends two families then goes quiet; three or four big clients carry the agency and one passes away.**
- PatientsPipeline page (the vendor wrote the owner’s story): ‘a referral partner who sends two families, then goes quiet for a quarter... running on word of mouth and three or four big clients — not a system’
- Tab 7 theme ‘If someone would just show me a system that brings clients predictably’
- **The owner is the only one selling and doing everything else too: scheduling, compliance, the 24/7 phone, payroll.**
- Tab 9 themes ‘The hands-on licensee-administrator’ and ‘The phone / scheduling never stops’
- OBB’s self-diagnosis section names it back to them: ‘I’m the only one selling — owner as bottleneck’
- **Vertical-specific versions: ABA owners are credentialed and still have no referrals, or have a waitlist they cannot staff; facilities carry tour quotas and ratios; placement agents ask how to get their first families.**
- ‘I’m credentialed with all the insurance companies that currently have their panels open... I’m still sitting at zero clients.’ (ABA Business Owners) — <https://www.facebook.com/groups/669886889110965/posts/942369488529369>
- Tab 5 theme ‘ABA: insurance won’t credential me, authorizations, the waitlist I can’t serve’

3. Common questions and objections that stop them buying

- **‘Has anyone actually used this? Is it worth it?’ — every vendor gets asked in the group before the owner replies to the rep. Proof from a named peer beats any claim.**
- Tab 4 theme ‘Has anyone used A Place for Mom / Caring.com / Care.com for clients?’ and ‘What lead companies actually work for private pay?’

- ‘Anyone received a call from a place for mom? What are your thoughts’ (13 reactions, 48 comments) — <https://www.facebook.com/groups/2156388261379184/posts/291614524540347...>
- **‘The leads won’t answer / they’re shared / they’re old.’ The owner assumes any lead is a shared lead until told otherwise.**
- ‘They give “fake clients”, nobody ever answered.’ (9 likes) — <https://www.facebook.com/groups/2156388261379184/posts/288131654888634...>
- Home Care Pipeline ad: ‘if you’ve tried ads before and got nothing but clicks and form fills from people who never answer...’ — the objection written into the ad
- **‘I’m not paying for something I can do myself’ and ‘no ad or marketing is guaranteed’ — the DIY and the fatalist. Both are answered by a result-based term (booked assessments, guarantee floor), not by a retainer.**
- ‘I’m sorry but I’m not paying anyone for something I can do myself.’ — <https://www.facebook.com/groups/2156388261379184/posts/288757708159362...>
- ‘You must spend money to make money; no ad or marketing is guaranteed.’ — <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>
- Tab 7 theme ‘I’d pay for results, not a retainer’
- **‘Do you only run Facebook ads?’ ‘What if we already get referrals?’ ‘How long before we see results?’ — the three questions the vendors put in their own FAQs because they hear them on every call.**
- PatientsPipeline FAQ (landing page) and OBB ‘How fast will I see results?’ / ‘See the 90-day climb’
- **‘I can’t afford it / I’m too small’ — the vendors pre-empt it with a floor (‘owners above \$70k/month’, ‘\$1M+/yr’, ‘20 to 80 unit communities’); the smaller owner reads that as ‘not for me’.**
- Plena feed ad: ‘Home care agency owners above \$70k/month’; HomecareGrow ads: ‘Home care owners \$1M+/yr’; Occupancy Partners: ‘built for 20 to 80 unit communities’
- 39 of 241 operator-facing ads open with a named-audience qualifier (hook patterns)
- **Cancellation and credit fear: ‘they make cancelling so hard’, ‘I had to block them’, ‘get my money back through my bank’. A visible pause / cancel term removes it.**
- ‘I was with A Place for Mom and wasn’t getting anywhere with them. I had to block them paying for something...’ — <https://www.facebook.com/groups/2156388261379184/posts/283328209368979...>
- VERVE landing page answers it: ‘Pause or Resume Anytime · Zero Setup Fees’

4. Common hooks and visual formats

- **Named-audience qualifier as the first words: ‘Home care agency owners...’, ‘Read this if you run an assisted living facility with apartments sitting empty’, ‘ABA owners...’. The most common opening in the operator-facing ads (39 of 241).**
- Home Care Breakthrough Solutions (103 days): ‘Home care agency owners doing \$1M to \$3M+ in annual revenue often hit the same frustrating wall...’
- Wisdom First Marketing (77 days): ‘Read this if you run an assisted living facility with apartments sitting empty.’
- **Question hooks (41 of 241): ‘Are you paying a salesperson who’s not even bringing in \$10K a week?’, ‘If your sales rep left tomorrow, would your referrals actually drop?’**
- Home Care Breakthrough Solutions, two ads
- **Number + timeframe + guarantee: ‘60 qualified assessments in 90 days. Guaranteed.’, ‘\$150,000 in new revenue in 90 days. Or we work for free.’, ‘20 booked, confirmed tours in 90 days’, ‘Araceli added 19**

clients last week with 1 simple ad campaign’.

- Plena / PatientsPipeline feed ad (451 reactions); OBB landing page; Occupancy Partners feed ad; HomecareGrow ads (86 days)
- **Against-the-directory positioning: ‘A shared lead isn’t a lead. It’s a race you already lost.’, ‘Exclusive families, never shared’, ‘Stop paying for leads. Stop sharing them with 5 competitors.’**
- OBB landing page; Occupancy Partners; CareXroads group post — <https://www.facebook.com/groups/2156388261379184/posts/288445237857276...>
- **Speed-to-lead as the proof mechanic: ‘75% of families choose the first community that responds — 47 hours vs five minutes’, ‘we call every inquiry within 2 min’.**
- Occupancy Partners landing page; OBB stat bar
- **Visual formats that carry these hooks: talking-head founder video on location (Occupancy Partners, Plena, Advanced Agent Marketing in the feed), a stat bar or ‘90-day climb’ chart, a calendar filled with booked assessments/tours, a named-client before/after, and the plain ‘Fill Your Assisted Living Units’ headline card. 102 of the 241 operator-facing ads are video, 139 static.**
- Competitor Ads → formats and hook patterns; Vince’s feed section (IMG_2787, 2794, 2806)

5. Features and benefits we can and should demonstrate in an ad

- **Exclusive inquiries on the owner’s own brand, never shared. Demonstrate it: show the inquiry arriving with the agency’s name on it and no other agency on the thread; contrast with the ‘sent to 4 other agencies’ voice.**
- Owner voice: ‘sent to 4 other agencies’; vendor proof that it sells: OBB ‘100% run on your brand, never shared’, VERVE ‘1-to-1 lead ownership’
- **Booked, confirmed assessments or tours on the calendar, not ‘leads’. Demonstrate it: a calendar view filling up, the confirmation text to the family, the count per week.**
- Occupancy Partners ‘20 booked, confirmed tours in 90 days’; Plena ‘60 qualified in-home assessments in 90 days’; OBB ‘we book qualified assessments onto your calendar’
- **Speed to lead: the call-back within minutes, at night and on weekends. Demonstrate it: a stopwatch or timestamped screen of an inquiry answered in under five minutes, against the ‘47-hour’ industry number.**
- Occupancy Partners page; owners’ own complaint that paid leads ‘never answered’ is the same problem seen from the family’s side
- **Intake handled, not just leads delivered: qualification (private pay, hours, location) before the owner sees it. Demonstrate it: the qualification questions on screen, the disqualified inquiry that never reached the owner.**
- Owners: ‘some are just looking, curious, or only wanted info’ (Private Pay Clients group); PatientsPipeline ‘from inquiry to the booked in-home assessment’
- **Result-based terms: a guarantee floor, pay on booked assessments, pause any time, no setup fee. Demonstrate it: the one-line term on screen, and the credit/cancel policy the referral sites refuse.**
- Tab 7 ‘I’d pay for results, not a retainer’; OBB ‘or we work for free’; VERVE ‘Pause or Resume Anytime · Zero Setup Fees’
- **Reporting that reads as move-ins, clients or admissions, not impressions. Demonstrate it: a one-screen report with cost per booked assessment and cost per client.**

- Occupancy Partners: ‘We report move-ins, not impressions’; owners’ question ‘what did you spend that you felt you got a good value for?’ (r/RunAHomeCareAgency) — <https://www.reddit.com/r/RunAHomeCareAgency/comments/1chu7jm/marketing...>
- **Keeps their referral relationships and adds a second channel. Demonstrate it: the discharge planner is still in the picture; the engine fills the weeks they go quiet.**
- Lead Sources: hospital / discharge planner relationships are the most-named source; ‘that still works’ — <https://www.facebook.com/groups/2156388261379184/posts/283561427678990...>
- **By-product the owner already noticed: the same ads bring caregiver applicants. Demonstrate it as a bonus, not the promise.**
- ‘I have not gotten a single client, I got a ton of caregivers and everyone in our community started to recognize us.’ — https://www.reddit.com/r/RunAHomeCareAgency/comments/1ffa065/anyone_kn...

Caveat: Hypotheses are the ‘demonstrate it’ clauses; the facts are the counts, the voices and the ads. Two things the research does not yet show: how an operator reacts to a price, and which of the vendors above actually deliver (no owner in the corpus reports a result from HomecareGrow, OBB, Plena or Occupancy Partners; the only outcome voices are about A Place for Mom, CareInHomes and Caring.com).

Economics, Valuation & Exit — what the established buyer scores by

What this buyer scores by. The established owner runs the business by the numbers a buyer will pay for.

Sources: Breakwater M&A valuation guide 2026, Carevoyant / Activated Insights benchmarking, HomeCarePulse, Home Health Care News M&A.

\$9–12M	\$2,600–5,000	12–24 mo	Record
6–8× exit at \$1.5M EBITDA	Cost to replace one caregiver	Census stability buyers require	PE home-care acquisitions 2024–25

Established pain hierarchy (measured across the corpus)

Ordered by document count over the 2026-09-08 ladder corpus (7,401 documents, all ten verticals); the number in parentheses is the same pattern on the morning home-care-only corpus (3,679 documents). A document is counted once per pattern.

1. **Caregiver retention at scale — 329 docs (home-care-only baseline 139).** The dominant pain. Turnover both *caps growth* (can't staff the cases you win) and *destroys valuation* (buyers discount hard for high turnover). Industry turnover runs **60–80%**; agencies below **50%** command premium multiples. Losing one caregiver costs **\$2,600–\$5,000** in recruit + train (Carevoyant/Activated Insights).
2. **Owner-operator treadmill — 228 docs (home-care-only baseline 71).** Can't step off the day-to-day; no management layer, no systems. The business can't run — or sell — without them. *"One major task in being a business owner is HR and payroll... wearing so many hats."*
3. **Exit / valuation / EBITDA — 87 docs (home-care-only baseline 29).** What's my agency worth, how do I sell, what raises the multiple.
4. **Census stability & growth — 76 docs (home-care-only baseline 28).** Predictable client flow to *scale*, not survive; volatile census = valuation discount.
5. **PE roll-ups & consolidation — 32 docs (home-care-only baseline 12).** Addus, BrightSpring, Help at Home are buying up their markets; compete, partner, or sell.
6. **Scaling past the revenue plateau — 28 docs (home-care-only baseline 7).** Stuck at \$Xm, can't break to the next tier.
7. **Referral / payor concentration risk — 21 docs (home-care-only baseline 13).** Over-reliant on one payor or a few referral feeders = fragile revenue + valuation discount.

The economics that matter to this buyer (industry benchmarks)

- **Valuation multiples (Breakwater M&A, 2026):** a small **owner-dependent** non-medical agency at ~\$300K EBITDA sells **3–4× (\$900K–\$1.5M)**; one with **\$1.5M+ EBITDA, diversified referrals + professional management commands 6–8× (\$9M–\$12M)**. Medicare-certified home health commands **1.5–2× higher** than non-certified at similar EBITDA. Small owner-operated deals often valued on **SDE** (adds back owner salary).
- **What drives the multiple UP:** low caregiver turnover (<50%), stable/growing census over 12–24 months, diversified payor mix, diversified referral sources, strong star ratings, clean survey history, **management that runs it without the owner**.
- **What drives it DOWN:** high owner dependency, declining/volatile census, single-payor or single-referral concentration, past deficiencies/sanctions.

- **Deal structure:** typically cash + **10–20% seller note** (2–4 yrs) + **10–20% earnout** tied to *census retention and key referral-relationship retention* post-sale.
- **The market:** PE-backed consolidators completed record home care acquisitions in 2024–25; hospitals are pushing patients out of facilities into home-based care (demand tailwind). "PE is dumping billions into home care despite 79% caregiver turnover" (r/Entrepreneur, ↑314).

Retention at Scale — the growth ceiling and the valuation lever

For the established owner, caregiver retention is not an HR nicety — it is the growth ceiling and the valuation lever. **329 corpus docs** (ladder corpus; 139 on the home-care-only baseline) touch it; the M&A data says agencies below **50% turnover** (vs. the **60–80%** industry norm) command premium multiples, and each lost caregiver costs **\$2,600–\$5,000** to replace (Carevoyant / Activated Insights). You cannot scale census you cannot staff.

The killer insight (why they actually leave)

"Home care isn't a medical business; it's a logistics and HR business... The moat isn't your branding or your bedside manner — it's your scheduling density. If you can give a caregiver 40 hours a week within a 5-mile radius, they stay. If you give them 20 hours with a 30-minute commute between clients, they go to Target. The 79% turnover is usually a symptom of fragmented schedules rather than just low hourly."

r/Entrepreneur operator · ↑7

"Places are offering \$25 an hour and still can't find staff. Part of the problem is the pay but also how they are treated... Many are just leaving healthcare altogether no matter how much they raise the wages."

r/nursing · ↑6

"Wages for home care workers are abysmal, fast food now pays better."

r/nursing · ↑120

The retention levers (from operators + the coach playbooks)

1. **Scheduling density** — cluster hours near the caregiver's home; full-time-equivalent hours in a tight radius beat a higher wage spread across a 30-minute commute.
2. **The first week** — *"a disorganized first week actively erodes their confidence in your agency's competence."* Structured onboarding/orientation is a retention act, not paperwork.
3. **PRN / float team** — a texted, first-come pool for coverage so a call-off doesn't burn your full-timers (and doesn't cost you 40 guaranteed hours).
4. **A real scheduler** — *"the scheduler position is very critical... has to be on top of everything."* At scale, scheduling IS retention.
5. **Pay + daily pay** — competitive wage floor plus earned-wage-access (Tapcheck/daily pay) to beat the fast-food alternative.
6. **Recognition & matching** — caregiver-client matching and being treated as more than "a number" (the recurring VOC complaint).

The retention/recruiting vendors competitors sell to owners (inLeap's adjacent set)

- **Training / onboarding:** CareAcademy, Nevvon, learn2care
- **Earned-wage-access / daily pay:** Tapcheck, and payroll add-ons
- **Recruiting / RPO / job boards:** Aploi, myCNAjobs, Indeed, ZipRecruiter, Hireline
- **AI / matching / monitoring:** Sensi.AI, scheduling-density tooling inside WellSky / AxisCare / AlayaCare

- **Coaches with retention programs:** Home Care Evolution (Hurricane), Justin Currie, Candyce Slusher, Jedi Calventas ("Stop Losing Caregivers")

inLeap's angle: the same acquisition engine that fills census also fills the caregiver pipeline — and pairing acquisition with scheduling-density + retention analytics is what turns turnover (the valuation killer) into the differentiator that raises the multiple.

Competitor Ads across the operator ladder (cleaned 2026-09-10)

Cleanup 2026-09-10 (Vince's review: "some of the ads aren't B2B — ads looking for caregivers, nurses, employees, and ads looking for clients"). Every card is now classified by WHO it talks to before any product word counts: an ad that names the operator as its audience sells to operators; an ad that recruits workers ("now hiring", "\$21–23/hr", "join our team") or talks to the family ("is it the right time for memory care?", "no waitlist", "your loved one") is the operator's own ad whatever else it says; a B2B ad that names no senior-care vertical (doctors, vets, therapists, PT clinics, generic hourly hiring) is an adjacent-practice ad kept aside as a format reference; a lead-gen vendor's own family-facing ad is its ghost funnel. Thirteen residual cards are hand-read overrides (`raw/ads_overrides.json`, each with its reason). Before the cleanup the "sells to operators" set was 264 and contained ~30 consumer or recruiting ads and ~25 adjacent-practice ads; three real B2B ads (Census Home x2, Wisdom First) were hiding in the own-ads bucket. Precision after the cleanup, random 30 read one by one: 30 of 30 B2B-to-senior-care-operators on a fresh random 30 (seed 911) read one by one after the 2026-09-10 audience-first cleanup; the 09-08 rules had scored 28 of 30 on their own sample while still admitting ~30 consumer/recruiting ads and ~25 adjacent-practice ads, which is why the sample number alone was not enough (Vince caught it by reading the tab).

Ordering rules: advertiser table by number of distinct ads then longest run; ads on the page by longevity, running businesses first, then would-be owners; the impressions position stated on each card is the card's rank inside its phrase's impressions-sorted Library result. Every advertiser name and card links to its landing page (or Facebook page when no destination link was captured) and to the Ad Library entry; creatives are cached in `creatives/ads/`.

Counts. 154 phrases + advertiser names across the ladder (ADLIB_owner_side.json, ADLIB_ladder_names.json, ADLIB_ladder_phrases.json, ADLIB_owner3.json, ADLIB_vince.json), **2,828 raw cards → 1,795 distinct ads → 233 selling to senior-care operators from 95 advertisers** (203 to running businesses, 30 to would-be owners: start-up coaches, licensing consultants, franchise recruiters); 27 adjacent-practice ads set aside; 45 lead-seller consumer funnels (ghost advertisers); 625 operators' own ads (556 to families, 64 recruiting staff, 5 to referral partners; collapsed on the page); 865 off-topic keyword noise dropped. Before the 2026-09-08 ladder expansion: 19 home-care phrases, 337 raw ads, 49 advertisers.

Ads selling to operators, by vertical addressed (an ad can address several): Home care 101 · Home health 25 · Hospice 13 · Assisted living / RAL 74 · Memory care 4 · Independent living 0 · Senior living / CCRC / SNF 39 · Adult day 1 · Placement / referral agency 16 · Multi-location medical 17 · Independent referral agents 11 · In-home ABA 13 · Cross-vertical 24.

Vince's feed (first source, D1l): 29 advertisers screenshotted from his feed; **21 found in the Library** by advertiser-name search, **8 feed-only:** Dr. Mark Stevens, Nathan Mummert, Nextdoor (Advertise on Nextdoor), Elias Benedith – Senior Care Growth Consultant (CareIntake AI), Michael Porche, Advanced Agent Marketing (Instagram), BAS Renovations (roofing), Advanced Agent Marketing · Brett And Ethan · 'Annuity Producers' page. Every one appears in the Competitor Ads tab with its screenshot (`creatives/feed/`), its verified site where one was captured, and its source marked.

Feed advertiser	Screenshot	Site	Vertical addressed	Hook (OCR)	Found in the Library?
-----------------	------------	------	--------------------	------------	-----------------------

Phoebe	creatives/feed/IMG_2782.jpg	https://jobs.phoebe.org	home care agency	Scheduling headaches holding you back? Home Care Agencies: Don't Hire Another Scheduler. We built you a better option.	yes — 9 ads, longest 210d, Ad Library (name search)
Dr. Mark Stevens	creatives/feed/IMG_2786.jpg	—	assisted living	Assisted living owners — quick question... Assisted Living Owners with 30+ Residents	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)
Wisdom First Marketing	creatives/feed/IMG_2784.jpg	https://go.wisdomfirstmarketing.com/assisted/	assisted living / senior living	For assisted living community operators... 6 MONTH WAIT LIST. We help you build a six month waitlist. Need more booked tours?	yes — 7 ads, longest 77d, Ad Library (name search)
Whatsnap.ai	creatives/feed/IMG_2784.jpg	—	marketers / operators	\$40 Leads Became \$9 Leads When I Deleted The Landing Page	yes — 2 ads, longest 12d, Ad Library (name search)
Occupancy Partners	creatives/feed/IMG_2788.jpg	https://go.occupancypartners.io/opt-in	assisted living / independent living	(video) "you know every empty unit..."; comment "What about independent living?" — "yes!"	yes — 21 ads, longest 22d, Ad Library (name search)
Nathan Mummert	creatives/feed/IMG_2789.jpg	—	hospice	"Some months, 90 Hospice admissions, some months, 45." Hospice Owners & Administrators trying to grow.	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)
ureDocs	creatives/feed/IMG_2790.jpg	https://www.7figuredocs.co/guaranteed-patients	medical practices (adjacent)	Hey Doctors! I'm Sophie, an AI... 30 Patients in 30 Days with AI	yes — 6 ads, longest 8d, Ad Library (name search)
Home Care Breakthrough Solutions	creatives/feed/IMG_2795.jpg	https://go.homecarebreakthrough.com/register	home care agency	Home care owners — are you paying a sales rep under \$10K/week in revenue results? Our system guarantees ROI or we work for free	yes — 15 ads, longest 464d, Ad Library (name search)

Lukrah	creatives/feed/IMG_2791.jpg	https://start.lukrah.com/book-call-page-2	multi-location medical care agencies	Multi-Location Medical Care Agencies... (talking head, "consultation")	yes — 4 ads, longest 23d, Ad Library (name search)
Home Care Pipeline	creatives/feed/IMG_2793.jpg	https://www.homecarepipeline.com/b	home care agency	Home care agency owners — if you've... Take the HomeCare Assessment	yes — 3 ads, longest 72d, Ad Library (name search)
Mint CRO	creatives/feed/IMG_2793.jpg	https://get.mintcro.com	marketers (adjacent)	Mint CRO Conversion Rate	yes — 11 ads, longest 65d, Ad Library (name search)
Searchlift AI	creatives/feed/IMG_2792.jpg	https://liftsearch.ai/booking-wf	assisted living	15 more family inquiries a month... Assisted Living Owners Get More Move-Ins From Google & AI WITHOUT ADS For \$297/mo. No retainers, no ad spe	yes — 6 ads, longest 8d, Ad Library (name search)
Premiere Destiny Home Care Success	creatives/feed/IMG_2798.jpg	https://premierhomecareuccess.com/referral-guidev2	home care agency	If your referrals are inconsistent... Your Referrals Aren't Random, They're Broken	yes — 3 ads, longest 135d, Ad Library (name search)
Talroo	creatives/feed/IMG_2799.jpg	https://www.talroo.com/start	home care / staffing (50+ caregivers)	If you hire 50+ caregivers, CNAs, or h... How Finish'd Filled Caregiver Roles Faster	yes — 29 ads, longest 13d, Ad Library (name search)
Nextdoor (Advertise on Nextdoor)	creatives/feed/IMG_2799.jpg	—	local business	Most of my new clients now come straight from Nextdoor.	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)
Elias Benedith — Senior Care Growth Consultant (CareIntake AI)	creatives/feed/IMG_2743.jpg	—	senior living directors + home care	Senior Living Directors & Home C... How CareIntake AI Works: family calls → booked appointment → reminder call/text → logs conversation & noti	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)

Michael Porche	creatives/feed/IMG_2746.jpg	—	residential assisted living investing	I almost didn't do this because I thought I wasn't ready. No cash. No plan B. Still closed two senior care homes. I got my plan here — THE O	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)
ClearDesk	creatives/feed/IMG_2742.jpg	https://usa.cleardesk.com/book-discovery-call	home care owners	FOR HOME CARE OWNERS: Schedulers, recruiters, built for Home Care. HIPAA-trained, WellSky-fluent placements at \$2,250–\$2,500/mo all-in, abou	yes — 4 ads, longest 92d, Ad Library (name search)
HomecareGrow.io	creatives/feed/IMG_2722.jpg	https://homecaregrow.io/150k-guarantee	home care agency (\$1M+/yr)	Home care owners doing \$1M+/yr... We Pay For Ads. You Get Home Care Clients	yes — 23 ads, longest 86d, Ad Library (name search)
Age Safe America	creatives/feed/IMG_2722.jpg	—	home safety training (adjacent)	Professional training in home safety, made simple. Equip yourself with the skills to create safer homes and support independent living.	yes — 3 ads, longest 35d, Ad Library (name search)
VERVE Care Partners	creatives/feed/IMG_2741.jpg	https://vervecarepartners.online/opt-in-page	in-home senior care agencies	PAY PER LEAD = GET 5+ IN HOME ELDER CARE LEADS IN 7 DAYS. No ad spend. No setup fee. High-intent leads.	yes — 19 ads, longest 106d, Ad Library (name search)
(YouTube search: "home care leads")	creatives/feed/IMG_2780.jpg	—	coaches	Justin Currie — Secrets for Home Care Agencies; Aaron Bogle — How To Get 22 Private Pay Home Care Leads FAST (Case Study)	yes — 1 ads, longest 15d, Ad Library (phrase search)
Occupancy Partners	creatives/feed/IMG_2787.jpg	https://go.occupancypartners.io/opt-in	assisted living	20 booked, confirmed private-pay tours... 'money down the drain' (video subtitle) · Fill Your Assisted Living Units	yes — 21 ads, longest 22d, Ad Library (name search)
Occupancy Partners	creatives/feed/IMG_2806.jpg	https://go.occupancypartners.io/opt-in	assisted living / senior living	EVERY EMPTY UNIT is \$4,000 to \$6,000 a month GONE. We fill them for you by giving you exclusive leads generated and booked onto your calenda	yes — 21 ads, longest 22d, Ad Library (name search)

Plena: PatientsPipeline	creatives/feed/IMG_2794.jpg	https://get.patientspipeline.com/	home care agency	Home care agency owners above \$70k/month: we'll book you 60 qualified in-home assessments in 90 days. Guaranteed.	yes — 1 ads, longest 14d, Ad Library (phrase search)
Georgia Behavior Associates	creatives/feed/IMG_2802.jpg	—	in-home ABA (operator's own consumer ad)	In-home ABA therapy is available for... Ages 18 months to 21 years · No Waitlist · Insurance Covered · Katie Beckett Waiver Accepted · Persona	yes — 2 ads, longest 184d, Ad Library (phrase search)
Advanced Agent Marketing (Instagram)	creatives/feed/IMG_2800.jpg	—	annuity / IUL agents — NOT senior care; format reference only	Can You Handle 15-20 Qualified... 'Take the consultation' (talking head over a Zoom-grid B-roll)	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)
BAS Renovations (roofing)	creatives/feed/IMG_2796.jpg	—	off-topic	Which roof fails?	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)
Advanced Agent Marketing · Brett And Ethan · 'Annuity Producers' page	RPreplay_Final1788919449 / 1788970736 / 1788970801 (screen recordings)	—	annuity / IUL agents — NOT senior care; format reference only	Can You Handle 15-20 Qualified Appointments · Attention Annuity Advisors... your pipeline · Annuity Producers — [income disclaimer burned in]	FEED ONLY — not findable by keyword or by advertiser name in the Library (2026-09-08)

Advertiser (sells to operators)	Site	Ads	Longest run	Sells to	Verticals addressed	In Vince's feed
HomecareGrow.io	https://homecaregrow.io/150k-guarantee	22	86d	operators	Home care, Cross-vertical, Placement / referral agency	yes
Occupancy Partners	https://go.occupancypartners.io/opt-in	20	22d	operators	Assisted living / RAL, Senior living / CCRC / SNF, Cross-vertical, Placement / referral agency	yes
AxisCare	https://axiscare.com/white-papers/understanding-evv-2	14	166d	operators	Home care, Assisted living / RAL	
Home Care Breakthrough Solutions	https://go.homecarebreakthrough.com/register	10	184d	operators	Home care	yes

Residential Assisted Living Academy	https://residentialassistedlivingacademy.com/how-to-start-a-residentia...	10	44d	both	Assisted living / RAL, Senior living / CCRC / SNF	
Wisdom First Marketing	https://go.wisdomfirstmarketing.com/assisted/	7	77d	operators	Assisted living / RAL, Senior living / CCRC / SNF	yes
Census Home	http://censushome.com/	6	23d	operators	Home care, Cross-vertical	
Assisted Living Locators Franchise	https://www.assistedlivinglocatorsfranchise.com/	5	70d	operators	Assisted living / RAL, Independent referral agents, Placement / referral agency	
Grow Senior Care Marketing	https://growseniorcaremarketing.com/grow-your-home-care-agency/	4	162d	operators	Home care	
Welton Hong	https://www.seniorcaremarketingmax.com/learning-center/	4	58d	operators	Home care	
Sensi.AI	https://www.sensi.ai/demo/	4	40d	operators	Home care, Senior living / CCRC / SNF	
KRIBA.co	https://www.facebook.com/61591814430984/	4	22d	operators	Assisted living / RAL, Memory care, Senior living / CCRC / SNF	
Calhoun Bhella Hospice Lawyers	https://calhounbhellahospice.com/	4	9d	operators	Hospice	
Medbridge	https://www.medbridge.com/care/remote-therapeutic-monitoring	3	215d	operators	Home health, Multi-location medical, Hospice	
Premiere Destiny Home Care Success	https://premierehomecaresuccess.com/referral-guidev2	3	135d	operators	Home care	yes
OBB - Home Care Growth	https://go.onlinebizbuilders.com/home-care-hero-wp	3	100d	operators	Home care	
Phoebe	https://www.phoebe.work/clock-in-out-landing	3	31d	operators	Home care	yes
Modern Business Marketing - MBM	https://www.facebook.com/modernbusinessmarketing1/	3	25d	operators	Assisted living / RAL, Placement / referral agency	
HH Assist	https://www.facebook.com/61590780969293/	3	22d	operators	Home health	
Homecare 101	https://www.homehealthcare101.com/free-class	3	9d	both	Home care, Home health, Multi-location medical	

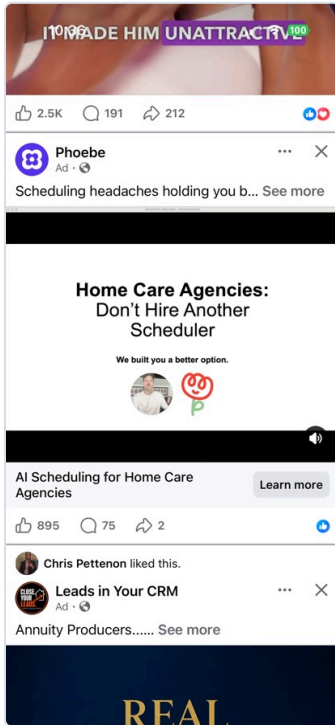
The RAL Room: Assisted Living Mastermind	https://offers.theralroom.com/webinar	2	208d	would-be owners	Assisted living / RAL	
LaKeysha Cobbs Hayes - Coach Key	https://practicereadyaba.com/free-resources	2	141d	operators	In-home ABA, Multi-location medical	
ABA Business Coach	https://ababusinesscoach.com/start-up-blueprint	2	138d	operators	Multi-location medical, In-home ABA	
Zach Pevnick, PT, DPT - Home Health Leaders	https://www.facebook.com/ZachPevnickPT/	2	105d	operators	Home health	
Group Home Masterminds	https://www.facebook.com/grouphomemasterminds/	2	104d	both	Assisted living / RAL, Placement / referral agency	
Home Care Pipeline	https://www.homecarepipeline.com/b	2	72d	operators	Home care	yes
Florida Assisted Living Consulting LLC with alfcaregivercon	https://www.floridaassistedlivingconsulting.com/assisted-living-and-ca...	2	68d	both	Assisted living / RAL, Senior living / CCRC / SNF, Home health, Placement / referral agency	
WholesaleTablets.com	https://www.facebook.com/wholesaletablets/	2	61d	operators	In-home ABA, Multi-location medical	
Caring Senior Service Franchise	https://www.facebook.com/CaringFranchise/	2	41d	would-be owners	Home care, Cross-vertical	
Hospice Care Owners Network	https://moonsethealth.com	2	36d	operators	Hospice	
Hospice Care Marketing	https://www.hospicecaremarketing.com/schedule	2	35d	operators	Hospice	
Kiro Farid	https://grow.caregenius.co	2	22d	operators	Multi-location medical, In-home ABA	
Patheown	https://www.facebook.com/61584957968788/	2	18d	operators	Home health, Hospice	
Alora Home Health Software	https://alorahealth.com	2	15d	operators	Home health, Home care	
HCPA	https://www.facebook.com/hcpa.official/	2	14d	would-be owners	Home care, Home health	
Optic Growth	https://www.facebook.com/61553310292857/	2	14d	operators	Home care, Cross-vertical	
JT Media	https://info.carestaffpro.com/p/veteran-generation	2	9d	operators	Home care	
Home Care Evolution	http://homecareevolution.com/bootcamp	2	9d	operators	Home care	

Searchlift AI	https://liftsearch.ai/booking-wf	2	8d	operators	Assisted living / RAL, Senior living / CCRC / SNF, Cross-vertical	yes
Grow Your Occupancy	https://go.growyouroccupancy.com/crm-move	2	7d	operators	Senior living / CCRC / SNF	
CarePatrol	https://www.facebook.com/CarePatrolCares/	2	7d	operators	Independent referral agents	
Homerun Health	https://homerun.health/book	2	5d	operators	Home health	
Rilla	https://www.rilla.com/lp/all	2	5d	operators	Assisted living / RAL, Senior living / CCRC / SNF	
Horst Construction	http://www.horstconstruction.com/	1	1822d	operators	Senior living / CCRC / SNF	
ELITE HealthCare Consulting	https://www.homecareconsultancy.com/	1	1411d	operators	Home care, Assisted living / RAL	
Assisted Living Investing	https://www.facebook.com/assistedlivinginvesting/	1	722d	would-be owners	Assisted living / RAL	
ABA Leaders Network	https://www.facebook.com/61581814976219/	1	322d	operators	Multi-location medical, In-home ABA	
Certified Homecare Consulting	https://www.certifiedhomecareconsulting.com/	1	245d	would-be owners	Home care	
Empowerment Consulting Agency	https://www.facebook.com/61579789084469/	1	184d	would-be owners	Assisted living / RAL	
stevetryethemortgageguy	https://l.facebook.com/l.php?u=https%3A%2F%2Fwww.instagram.com%2F_u%2F...	1	152d	operators	Assisted living / RAL, Senior living / CCRC / SNF	
Scale My Niche	https://go.scalemyniche.com/learn-more-senior	1	135d	operators	Independent referral agents	
First Day Homecare	https://www.facebook.com/firstdayhomecare/	1	133d	would-be owners	Home care	
Cory Boldroff- The Real Estate Planner	https://forms.gle/9qMRW43RqhaK8CxBA	1	125d	operators	Independent referral agents	
Avida Personal Home Care	https://www.facebook.com/avida.wi/	1	107d	would-be owners	Home care, Multi-location medical	
Home Care Sales	https://www.facebook.com/homecaresales/	1	107d	operators	Home care, Home health, Hospice, Multi-location medical	

VERVE Care Partners	https://www.facebook.com/61589396376519/	1	106d	operators	Assisted living / RAL, Home care	yes
Accushield	https://accushield.com/take-a-tour/	1	103d	operators	Senior living / CCRC / SNF	
Phillip Vincent	http://www.momshouse.com/start	1	99d	would-be owners	Independent referral agents, Senior living / CCRC / SNF	
ClearDesk	https://usa.cleardesk.com/book-discovery-call	1	92d	operators	Home care	yes
Treasured Hearts ABA, LLC	https://www.facebook.com/treasuredheartsa/ba/	1	78d	operators	Multi-location medical, In-home ABA	

Read of the field across the ladder (from the ads, hypotheses labeled): Across the ladder the vendors sell four things: (a) occupancy / move-ins / booked tours to assisted living and senior living operators — Wisdom First ('6 MONTH WAIT LIST', '50+ facilities trust us', 30-second quiz), Occupancy Partners ('How much of your census depends on referral sources you don't own? One discharge planner moves on...'), Searchlift ('15 more family inquiries a month'), Grow Your Occupancy (sales-leader training), Rilla (mystery-shop AI for sales counselors); (b) caregiver / CNA recruiting to home care, home health and senior living — Talroo ('If you hire 50+ caregivers, CNAs, or home health aides a month, the big job boards can't keep your pipeline full'), JT Media, Hireline, ClearDesk (offshore schedulers and recruiters 'built for Home Care', HIPAA); (c) AI intake / scheduling / referral processing — Phoebe ('Don't Hire Another Scheduler'), CareIntake (feed only), Homerun Health ('intake in minutes, not days... AI agents that process referrals'), Integrated Hire ('you don't need another \$90K RN'); (d) the pay-per-lead / we-pay-for-ads guarantee lane — VERVE ('5+ in-home elder care leads in 7 days, no ad spend'), HomecareGrow ('We Pay For Ads. You Get Home Care Clients', owners doing \$1M+/yr), Home Care Pipeline, Premiere Destiny ('Your Referrals Aren't Random'). The starter lane is its own market: RAL Academy, Assisted Living Investing, The RAL Room, Adult Day Care Academy, hospice-license consultants, Florida ALF consulting — courses and licensing, not growth. **Gap for inLeap (hypothesis):** nobody in the 267 operator-facing ads sells one engine that fills census AND the caregiver pipeline across more than one rung; the occupancy vendors stop at AL/senior living, the recruiting vendors stop at job ads, and the pay-per-lead vendors stop at home care. The words that recur in the winners: 'empty', 'wait list', 'booked tours', 'referral sources you don't own', 'we pay for ads', 'don't hire another scheduler'.

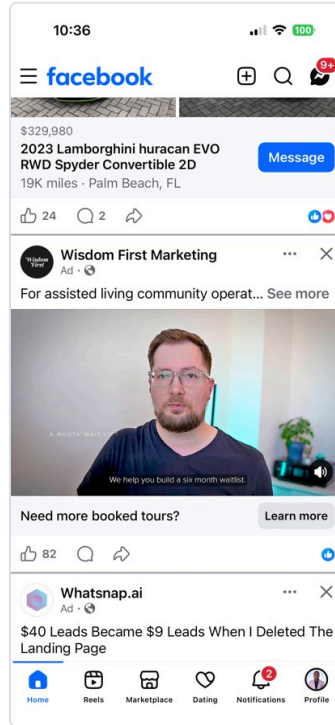
Vince's feed — the screenshots



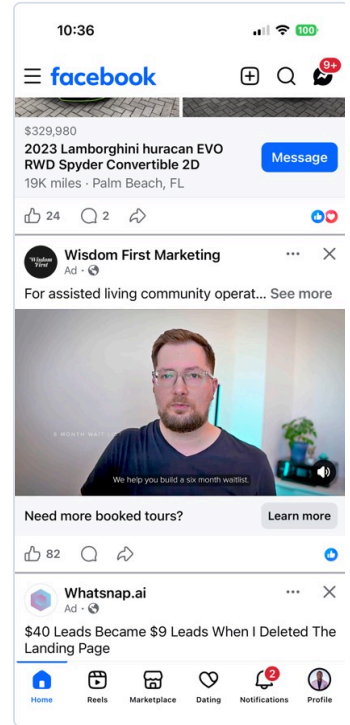
Phoebe — home care agency



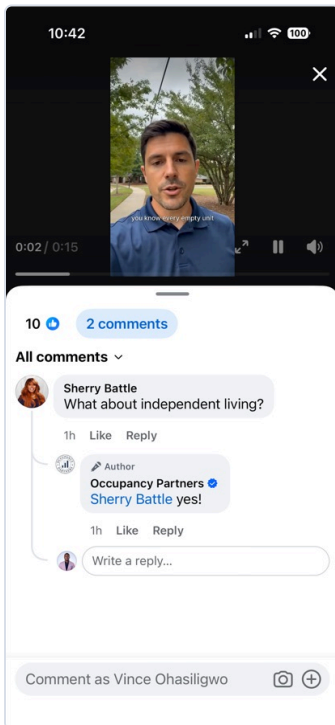
Dr. Mark Stevens — assisted living



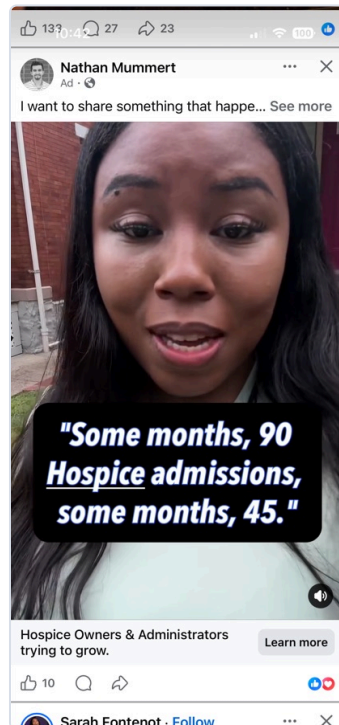
Wisdom First Marketing — assisted living / senior living



Whatsnap.ai — marketers / operators



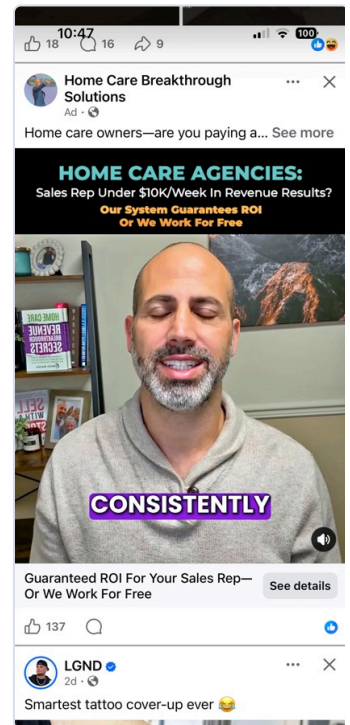
Occupancy Partners — assisted living / independent living



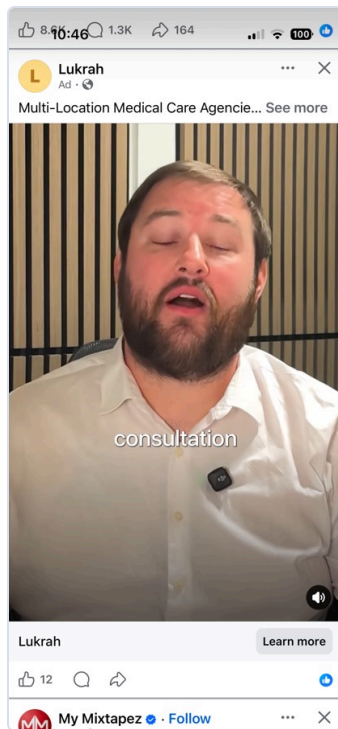
Nathan Mummert — hospice



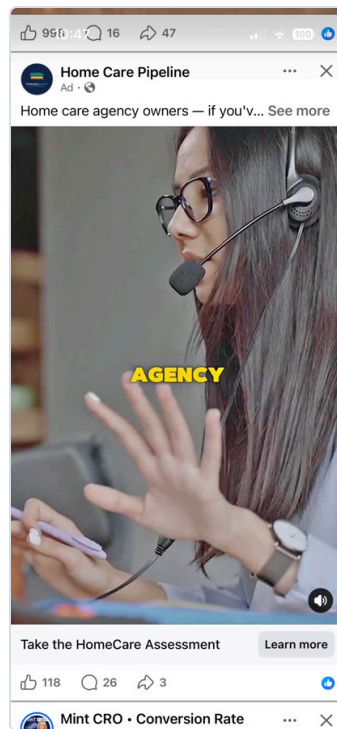
ureDocs — medical practices (adjacent)



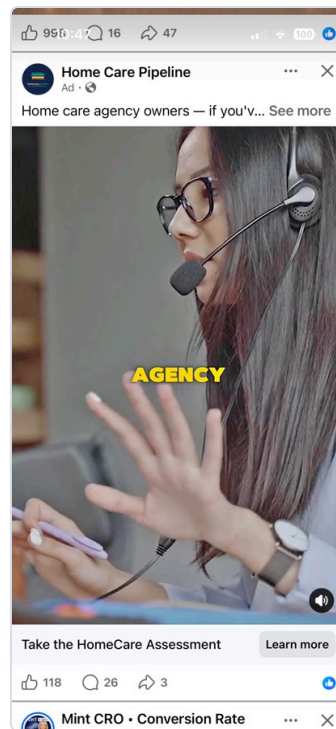
Home Care Breakthrough Solutions — home care agency



Lukrah — multi-location medical care agencies



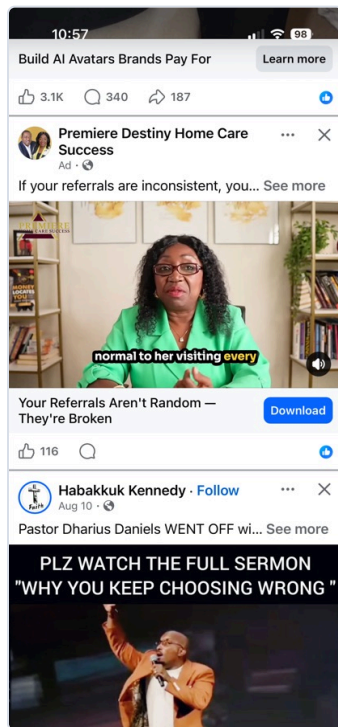
Home Care Pipeline — home care agency



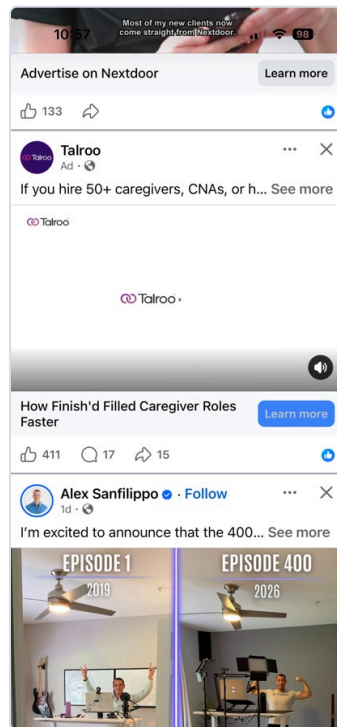
Mint CRO — marketers (adjacent)



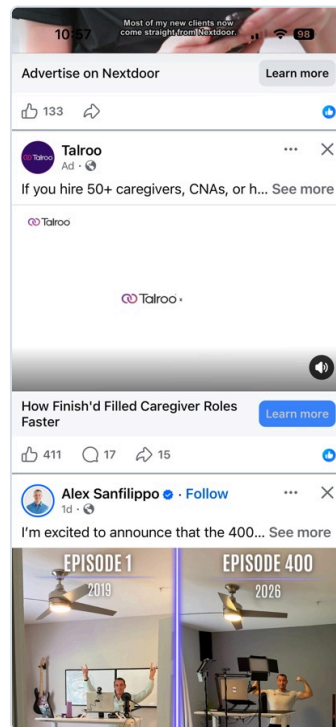
Searchlift AI — assisted living



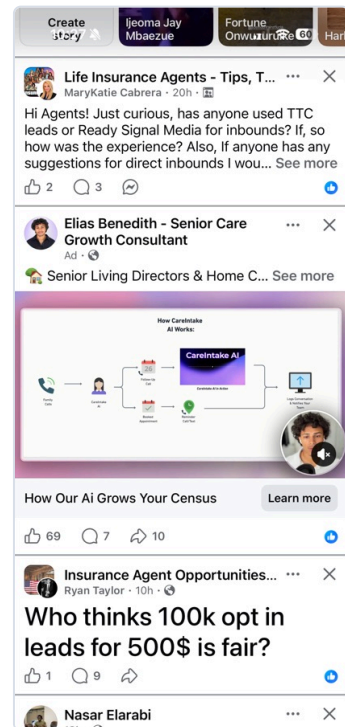
Premiere Destiny Home Care Success — home care agency



Talroo — home care / staffing (50+ caregivers)



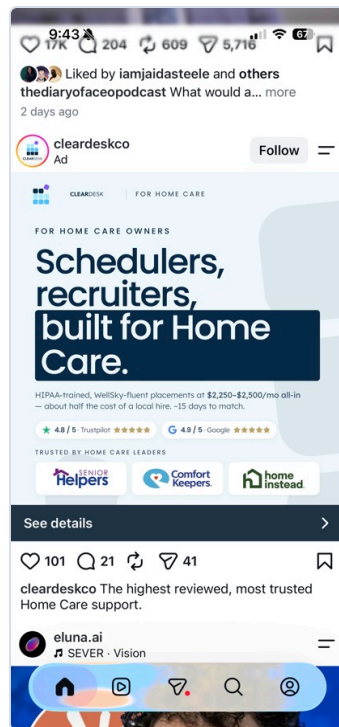
Nextdoor (Advertise on Nextdoor) — local business



Elias Benedith – Senior Care Growth Consultant (CareIntake AI) — senior living directors + home care



Michael Porche — residential assisted living investing



ClearDesk — home care owners



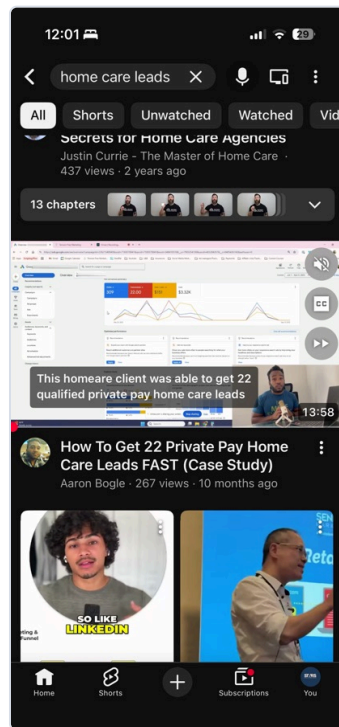
HomecareGrow.io — home care agency (\$1M+/yr)



Age Safe America — home safety training (adjacent)



VERVE Care Partners — in-home senior care agencies



(YouTube search: "home care leads") — coaches



Occupancy Partners — assisted living



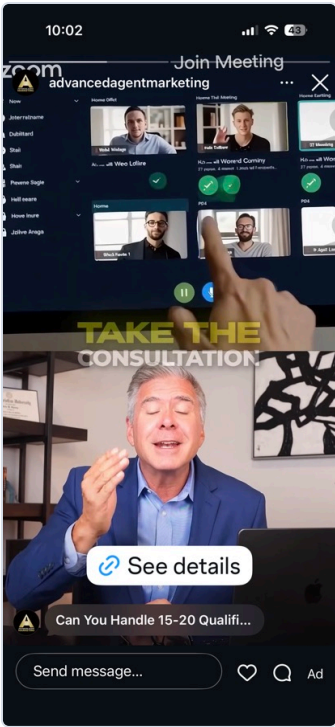
Occupancy Partners — assisted living / senior living



Plena: PatientsPipeline — home care agency



Georgia Behavior Associates — in-home ABA (operator's own consumer ad)



Advanced Agent Marketing (Instagram) — annuity / IUL agents — NOT senior care; format reference only



BAS Renovations (roofing) — off-topic

Landing pages (B2B only)

Eye reviews of the pages that matter (full-page screenshots are in [creatives/lp/](#) and on the page's Competitor Ads tab):

- **<https://homecare.aplaceformom.com>** — Eye review: the provider-side door is a five-field form (name, company, email, phone, state) under 'Trusted by families. Built for home care growth.' with 'How our program works' and 'Included in Your Partnership' below, no price on the page; 'a team member will reach out within 1–2 business days' — the sales call the owners in the groups describe (\$50–68 per lead, \$580 prepaid for 10) happens after this form, not on it. A portal login sits top-right: the relationship is a managed account, not self-serve.
- **<https://www.caring.com/partners>** — Eye review: Caring.com's provider page is 'Claim Your Free Caring.com Listing' — the directory model: free listing first, 'Get Referrals' as a nav CTA, separate 'Home Care Partner Sign In' and 'Senior Living Partner Sign In'. The page sells the listing, not the lead price; owners learn the per-lead cost inside.
- **<https://homecaregrow.io>** — Eye review: the purest owner-facing pitch in the set. H1 'Home Care Agencies: Add \$150,000 New Revenue In 16 Weeks With Exclusive Leads', a guarantee ('or we work with you for free until you do'), a named client proof ('Araceli from Love N Care adds 12–19 new clients per week'), a 20-minute 'Agency Growth Call' as the only CTA, 'proud partner of franchisees'. It sells exactly against the group complaint: 'Are low quality leads limiting your agency's growth?' → exclusive, fresh, Meta-ads system.
- **<https://onlinebizbuilders.com>** — Eye review: OBB / Home Care Hero — '\$150,000 in new revenue in 90 days. Or we work for free.' 'A shared lead isn't a lead. It's a race you already lost.' 'We don't sell you leads. We book qualified assessments onto your calendar.' Stat bar: 2-minute call-back, 100% run on your brand never shared, 0.22% dispute rate since 2019. A self-diagnosis section ('Where are you stuck? feast or famine / owner as bottleneck / my leads don't book') mirrors the group threads word for word. CTA is APPLY (qualification), not buy.
- **<https://occupancypartners.io>** — Eye review: senior living version of the same offer, 'for independently owned senior living communities' (20–80 units). H1 'You don't need more leads. You need more move-ins.' Argument: 75% of families choose the first community that responds, 47-hour average response vs five minutes, \$120K lifetime value of one move-in, 88% industry occupancy. 'We book the tours. We report move-ins, not impressions. Exclusive families, never shared.' CTA: book a strategy call.
- **<https://get.patientspipeline.com>** — Eye review: 'Built exclusively for non-medical home care agencies.' H1 'More in-home assessments. More new clients. Less guesswork.' The problem section is the owner's own story: 'You signed five clients last month. This month? Nothing... a referral partner who sends two families, then goes quiet for a quarter... running on word of mouth and three or four big clients — not a system.' Disqualifier ('isn't for every home care agency'; the feed ad says owners above \$70k/month), FAQ answers 'Do you only run Facebook ads?' and 'What if we're already getting referrals?'. CTA: free growth assessment.
- **<https://www.momshouse.com>** — Eye review: the referral-training vendor for the independent agent. H1 sells 'the Silver Tsunami as your endless revenue source for referral fees and real estate deals'; three certifications (Senior Living Placement 6 weeks, Real Estate Investing 8 weeks, Downsizing Specialist 6 weeks), 'Get Certified to Unlock Access', bundle pricing on the page (\$47 entry, \$6,000 / \$18,000 / \$216,000 income framing). The buyer it creates is the realtor-turned-placement-agent who then needs families — the same acquisition gap, one rung down.

- <https://vervecarepartners.com> — Eye review: pay-per-lead for placement agents and providers: 'Connect with Local Families Actively Seeking Senior Care', 'Launch in <7 days · 1-to-1 Lead Ownership · Zero Setup Fees', 'Every lead is yours — no sharing, no overlap', phone number as the CTA. The 'exclusive' claim is the direct answer to the A Place for Mom complaint; no price on the page.
- <https://www.phoebe.work> — Eye review: not a lead vendor — the scheduling AI from Vince's feed (895 reactions). H1 'The AI teammate that keeps your home care agency running 24/7', '75% of shifts filled in under 15 minutes', 'Book a Demo'. It sells the other job (staffing) to the same owner; useful as the proof that the owner audience buys from Meta ads when the pain is named precisely.
- <https://www.careinhomes.com> — Eye review: the directory the owners rate as cheaper (\$25 a lead in the groups): family-facing matching page with a 'Provider Login' and 'Featured Home Care Near [zip]' listings; the provider offer is not on the page.
- <https://www.seniorly.com/agents> — Eye review: Seniorly (now CareScout) sells the family the agent: 'Seniorly Partner Agents are commissioned by the senior living community you choose only when you move in' — the placement-agent economics the RCFE owners resent, stated to the family as '100% free'.

Advertiser / page	URL	H1	CTAs	Forms
A Place for Mom — home care partner page	https://homecare.a.placeformom.com/	Trusted by families. Built for home care growth.	Get started	1
Caring.com — partners	https://www.caring.com/partners/get-listed	Claim Your Free Caring.com Listing	The Free Senior Living Advisors, Start a Free Consultation, Starting The Convers	3
CareInHomes	https://www.careinhomes.com/	Need help? Use our home care matching service, check out options here	Contact Us, See More Listings, Get Started	1
Seniorly — partners	https://www.seniorly.com/partners/communities		Sign In, Learn more, Contact	0
HomecareGrow.io	https://homecaregrow.io/	Home Care Agencies: Add \$150,000 New Revenue In 16 Weeks With Exclusive Leads		0
Phoebe AI scheduling (feed)	https://withcoral.com/	The data engine for enterprise AI	Contact, See the full methodology and results., Contact us	0
Home Care Breakthrough Solutions	https://homecarebreakthrough.com/	HOME CARE BUSINESS COACHING	Get More Clients, Find and Keep More Caregivers, START HERE	0
Mom's House (referral training)	https://www.momshouse.com/	...And How The Silver Tsunami Can Be Your Endless Revenue Source For Referral Fees And Real Estate Deals Over The Next 2	Contact, Free Webinar, Get Certified	1
CarePatrol franchise	https://carepatrol.com/franchising/research-carepatrol/	CarePatrol Senior Care Advisors	Facebook (opens in new tab), Request Info, Referral Partners	1
Care.com for business	https://www.care.com/benefits/		Get benefits, Learn more, Talk to our team	0
Occupancy Partners (feed)	https://occupancypartners.io/	You don't need more leads. You need more move-ins.	Occupancy Partners, BOOK A CALL, BOOK A STRATEGY CALL	0

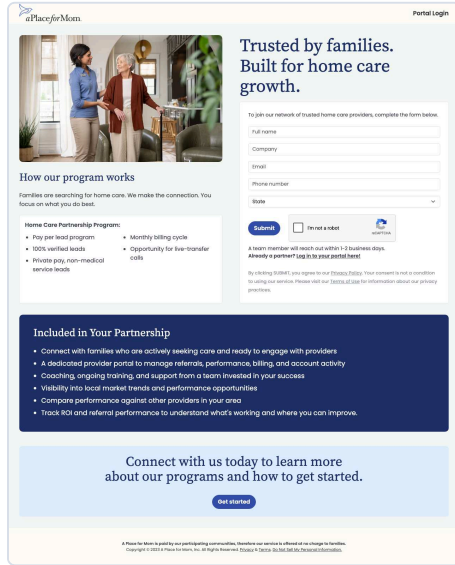
Plena / PatientsPipeline (feed)	https://get.patientspipeline.com/	More in-home assessments. More new clients. Less guesswork.	Get My Free Growth Assessment, I'm ready to get more patients., Do you only run	0
Phoebe AI scheduling (feed)	https://www.phoebework/	The AI teammate that keeps your home care agency running 24/7.	Get Started, Hear Phoebe Talking, Book a Demo	0
Assisted Living Locators franchise	https://www.assistedlivinglocatorsfranchise.com/	Assisted Living Locators Franchise Opportunity	Request Info & Download, GET STARTED!, Disclaimer	2
VERVE Care Partners pay-per-lead (feed)	https://vervecarepartners.com/	Connect with Local Families Actively Seeking Senior Care	Contact Us, Call or Text: 385-402-8683, Book Consult Now	0
Wisdom First Marketing (feed)	https://wisdomfirstmarketing.com/	Better Leads Start With Trust.	LET'S TALK, Call Tracking, Free Resources	0
Seniorly — for placement agents	https://www.seniorly.com/agents	Find your local senior living agent	Sign In, Learn more, Contact	1
OBB Home Care Growth (Online Biz Builders)	https://onlinebizbuilders.com/	The go-to marketing company for home care agencies.	APPLY NOW, APPLY FOR THE HOME CARE HERO PROGRAM, SEE HOW IT WORKS	0
Talroo caregiver hiring (feed)	https://www.talroo.com/	Candidates You Can't Get Anywhere Else.	Apply Intelligence, Channel Partners, Learn more	0
A Place for Mom — partners portal	https://partnercentral.aplaceformom.com/		Contact Us, OUR PARTNERSHIP, GETTING STARTED	2
Home Care Pulse (Activated Insights) — operator vendor	https://activatedinsights.com/	Uplift the Care Experience	Contact Us, CONTACT US, Learn More	0
AxisCare (software vendor ads found 09-08)	https://axiscare.com/	Home Care Is Complex. How You Manage It Shouldn't Be.	Vital Signs, AI Call Transcription, Learn More	1
Approved Senior Network / Hurricane Marketing (09-08 ads)	https://www.approvedseniornetwork.com/	Approved Senior Network Find Care Anywhere	Providers Contact Us, Senior Care Businesses - Get Listed Today, Join Our Site T	2
ChoiceLocal home care marketing	https://choicelocal.com/home-care-marketing/home-care-advertising-idea...	ChoiceLocal Home Care Advertising Ideas Built Around Better Customer Decisions	Contact Us, Schedule A Consultation, Get Started	2
corecubed home care marketing	https://corecubed.com/	Elevate Your Home Care Marketing With corecubed	Core Partners, Website Design, Request a Consultation	2
Senior Living Mastery lead gen	https://seniorlivingmastery.com/services/senior-living-lead-generation...	Senior Living Lead Generation	Book A Call, Contact Us, 1-On-1 Booking Call	0
Waypoint Converts senior living leads	https://www.waypointconverts.com/		Register free →, CallRail Integration, Learn	0
Further (senior living AI leads)	https://www.talkfurther.com/	The leading AI Sales & Marketing platform for Care Communities	Tour SchedulingSeamlessly schedule tours., AnalyticsGet actionable insights., Le	0

SageCare AI intake	https://www.sagecare.ai/	The modern home care agency front office.	Schedule a demo, Get access, Learn more	0
Horst Construction	https://www.horstconstruction.com/	BUILDING WHAT MATTERS TO YOU®	PARTNERS, DESIGN-BUILD, AUDITORIUM DESIGN & THEATER CONSTRUCTION	2
BrightStar Care Bryan / College Station	https://www.brightstartcare.com/blog/senior-care-after-surgery/	Home Health Care for Seniors After Surgery	Contact Us, Call Us 24/7 844.518.0420, FIND CARE NEAR YOU	1
ELITE HealthCare Consulting	https://homecareconsultancy.com/	Welcome to ELITE HealthCare Consulting	STARTUP SERVICES, Non-Medical Home Care Business Startup, Home Health Care Busin	1
The Kensington	https://tools.roobrik.com/agemark/kensingtoncumberland/seniorliving/st...	Is it the right time for senior living?		1
Kingston Bay Senior Living	https://tools.roobrik.com/agemark/kingstonbay/memorycare/start?roobrik...	Is it the right time for memory care?		1
Applaud Autism Services - ABA Therapy	https://www.applaudautism.com/	A child with autism is a full-time occupation.	Contact Us, Schedule a callback, See all ABA services	6
Familiar Surroundings Home Care	https://asnjobs.com/FamiliarSurroundingsHomeCare		Apply Now	0
Colorado Behavior and Learning Group	https://www.coloradobehavior.com/	Licensed ABA Therapy, On-Site Experts: Your Child's Growth Starts Here	Join Us, Referral Partners, Contact	1
Comfort Keepers Home Office	https://www.comfortkeepers.com/b2b-referral/	Meaningful in-home care for your patients	Apply	1
Certified Homecare Consulting	https://www.certiifiedhomecareconsulting.com/		Licensing & Startup, Start a Home Care Business, Start a Home Health Care Busine	2
Nathan Littauer Hospital and Nursing Home	https://secure7.saa shr.com/ta/6101323.careers?full_apply=&jobid=55...	Apply for Job		0
Nicole McCance	https://mccancemethod.com/the-psychology-today-template/	The Psychology Today Template	Free Masterclass, Book A Free Call, Download Now!	1
Medbridge	https://www.medbridge.com/care/remote-therapeutic-monitoring	REMOTE THERAPEUTIC MONITORING	Contact sales about something else., contact page, Contact Sales	1

The RAL Room: Assisted Living Mastermind	https://offers.theraloom.com/webinar-b	High Income Professionals: Invest in the Nation's Most In-Demand Housing Sector:	Reserve Your Seat - Free Live Webinar, Sign me up!	0
Home Care Breakthrough Solutions	https://go.homecarebreakthrough.com/register	Revealed Live: The 3-Step Home Care Revenue Breakthrough System Agencies Use To Scale To 10M+ In Revenue	Save My FREE Seat... Tuesday at 1pm EST	0
AxisCare	https://axiscare.com/white-papers/understanding-evv-2/	Understanding Electronic Visit Verification (EVV)	Vital Signs, AI Call Transcription, Learn More	1
Grow Senior Care Marketing	https://growseniorcaremarketing.com/grow-your-home-care-agency/	Watch The Video Below To Learn The Digital Marketing Secrets We Used To Grow My Choice By Over 400%	SCHEDULE A FREE STRATEGY SESSION, SCHEDULE YOUR STRATEGY SESSION	0
LaKeysha Cobbs Hayes - Coach Key	https://practicereadyaba.com/free-resources	Pick Your Free Guide.	GET FULL ACCESS, GET MY FREE GUIDE →, FIND MY TOP OPPORTUNITIES →	0
ABA Business Coach	https://ababusinesscoach.com/start-up-blueprint	Start-Up Blueprint	GET THE ABA START-UP BLUEPRINT!, keyboard_arrow_right I WANT TO JOIN!, settings	2
Premiere Destiny Home Care Success	https://premierehomecaresuccess.com/referral-guidev2	7 Hidden Breakdowns Costing You Clients (and How to Fix Them)		0
Scale My Niche	https://go.scalemy niche.com/learn-more-senior			1
GreenOak Accounting	https://www.greenoakaccounting.com/webinar		FREE CONSULTATION, Free Consultation, Contact Us	0
Cory Boldroff- The Real Estate Planner	https://docs.google.com/forms/d/e/1FAIpQLSd2BNOw7Zwi2e0NhYLza1_LEnWigd...		Sign in to Google, Learn more	1
RN Pad	https://www.rnpad.com/	RN Pad is the solution to fast and efficient nursing assessments	Free Tools, Contact Us, Schedule a demo	0
Griffin Mallas	https://www.grouppracticebuilders.com/growth-levers/get-free-gp-growth...		Contact	0
Accushield	https://accushield.com/take-a-tour/	Your Personalized Tour Awaits	LEARN MORE, Get Started, Sign-in Options	1
MTCS Business Finance	https://tools.mtcsbusinessfinance.com/	Grow Your Mental Health Practice's Impact And Income.	Get My Free Access, Use the Free Calculator	1

OBB - Home Care Growth	https://go.onlinebizbuilders.com/home-care-hero-wp	We'll Implement Our 'Home Care Hero' Program & Guarantee You \$150,000 in New Revenue in 90 Days or We Work For Free Until	See if your agency qualifies →, See If You Qualify	0
Phillip Vincent	https://www.momshouse.com/start	...And How The Silver Tsunami Can Be Your Endless Revenue Source For Referral Fees And Real Estate Deals Over The Next 2		2
Evolve Healthcare Marketing	https://ehmresults.com/	Full Funnel Patient Acquisition for Mid-Market Healthcare Practices	Contact Us, Let's Talk, call attribution	0
CarDon Senior Living	https://cardon.us/communities/bell-trace/rehabilitation/	Rehabilitation	Start Now, Contact, Facebook Search	0
ClearDesk	https://usa.cleardesk.com/book-discovery-call/	"My VA made a huge impact in just three months." — Kayla S.	Book discovery call →, Book my discovery call →	0
HomecareGrow.io	https://homecaregrow.io/150k-guarantee	Home Care Agencies: Add \$150,000 New Revenue In 16 Weeks With Exclusive Leads		0
Carla Brown, EA - Accounting & Tax Solutions for Physicians	https://browntax.co/7loopholes	Download Your Free	YES! Give Me My Free eBook!	0
Bigageenergy	https://caregivingmasterclass.com/	You don't have to figure caregiving out alone	JOIN NOW, Join the Masterclass now, SIGN UP TODAY	0
Nurse Next Door	https://www.facebook.com/nursenextdoor/	Nurse Next Door	See all photos	2
Wisdom First Marketing	https://go.wisdomfirstmarketing.com/assisted/		See how it works	0
Home Care Pipeline	https://www.homecarepipeline.com/b	3–5 New Private-Pay Clients	SEE IF I QUALIFY →, SEE IF I QUALIFY	0
Nomia HQ	https://nomiahq.com/contact-us/	Get in touch	Contact, Contact Us	0
Florida Assisted Living Consulting LLC with alfcaregivercon	https://www.floridassistedlivingconsulting.com/assisted-living-and-ca...	The #1 Conference for Caregivers, Assisted Living Operators & Senior Housing Professionals in Florida!	Get Your Ticket Now, Get Your Access Ticket Now, See More Reviews	1
TruBlue Franchise Opportunities	https://www.trubluefranchise.com/first-time-business-owner-segment	Why Franchising is the Best Choice for First-Time Business Owners	Schedule Now, Schedule a brief 10-minute call, Download Now	4

The eye-reviewed landing pages



Trusted by families. Built for home care growth.

How our program works

- Pay per lead program
- 100% verified leads
- Private pay, non-medical service leads
- Monthly billing cycle
- Opportunity for low-regular calls

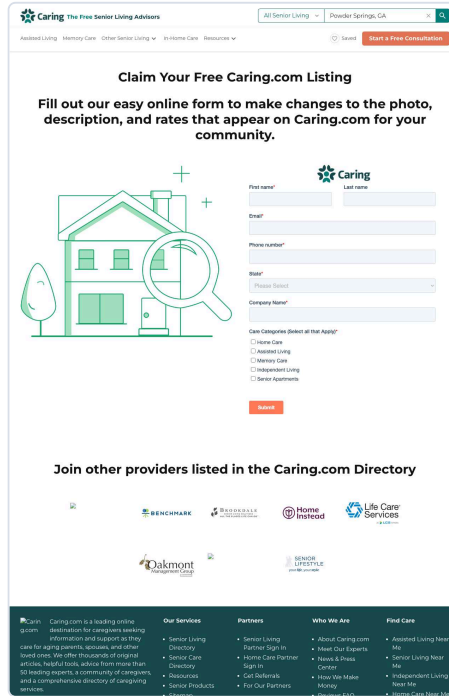
Included in Your Partnership

- Connect with families who are actively seeking care and ready to engage with providers
- A dedicated provider portal to manage referrals, performance, billing, and account activity
- Coaching, ongoing training, and support from a team invested in your success
- Visibility into local market trends and performance opportunities
- Compare performance against other providers in your area
- Track ROI and referral performance to understand what's working and where you can improve.

Connect with us today to learn more about our programs and how to get started.

[Get started](#)

<https://homecare.aplaceformom.com>



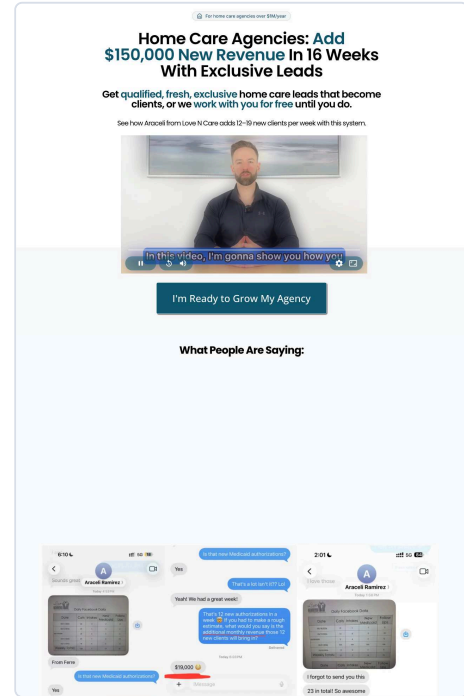
Claim Your Free Caring.com Listing

Fill out our easy online form to make changes to the photo, description, and rates that appear on Caring.com for your community.

Join other providers listed in the Caring.com Directory

[Home](#)

<https://www.caring.com/partners>



Home Care Agencies: Add \$150,000 New Revenue in 16 Weeks With Exclusive Leads

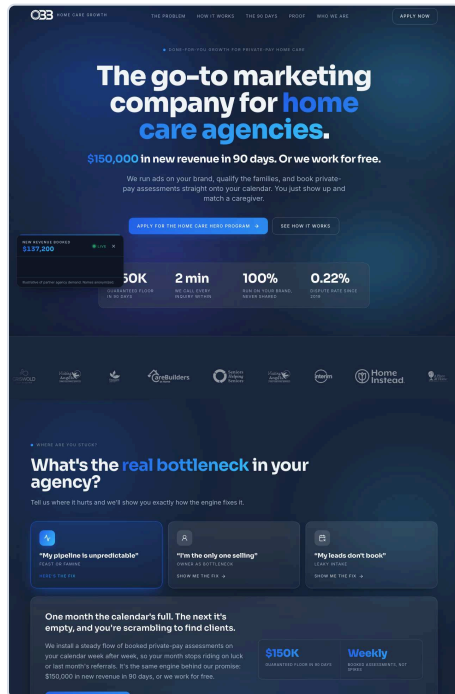
Get qualified, fresh, exclusive home care leads that become clients, or we work with you for free until you do.

See how Ancestral from Love N Care adds 12-15 new clients per week with this system.

[I'm Ready to Grow My Agency](#)

What People Are Saying:

<https://homecaregrow.io>



The go-to marketing company for home care agencies.

\$150,000 in new revenue in 90 days. Or we work for free.

APPLY FOR THE HOME CARE MARK PROGRAM

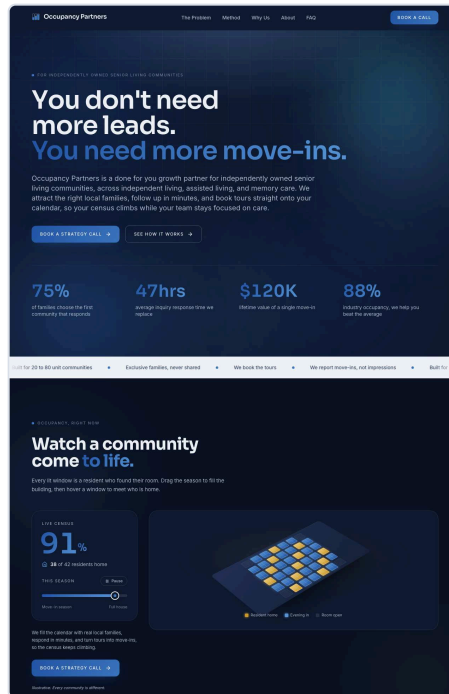
30K 2 min 100% 0.22%

What's the real bottleneck in your agency?

One month the calendar's full. The next it's empty, and you're scrambling to find clients.

[Book a Strategy Call](#)

<https://onlinebizbuilders.com>



You don't need more leads. You need more move-ins.

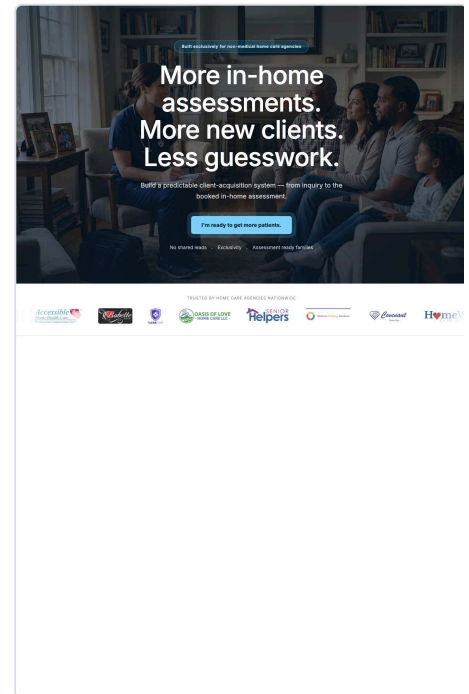
Occupancy Partners is a done for you growth partner for independently owned senior living communities, across independent living, assisted living, and memory care. We attract the right local families, follow up in minutes, and book tours straight into your calendar, so your census climbs while your team stays focused on care.

75% 47hrs \$120K 88%

Watch a community come to life.

[Book a Strategy Call](#)

<https://occupancypartners.io>

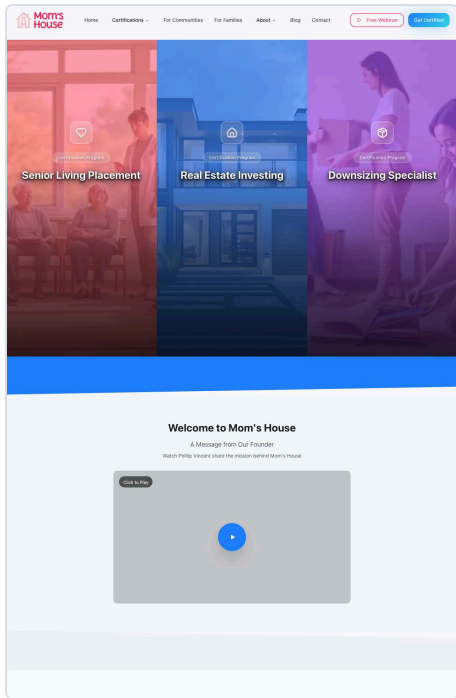


More in-home assessments. More new clients. Less guesswork.

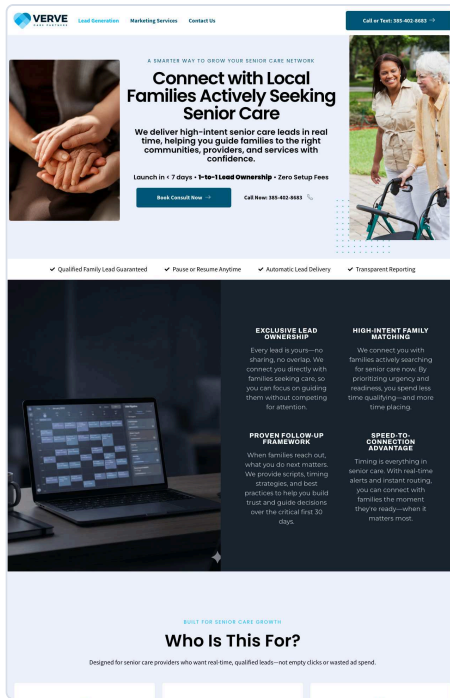
Build a predictive client-acquisition system — from inquiry to the booked in-home assessment.

[I'm ready to get more patients.](#)

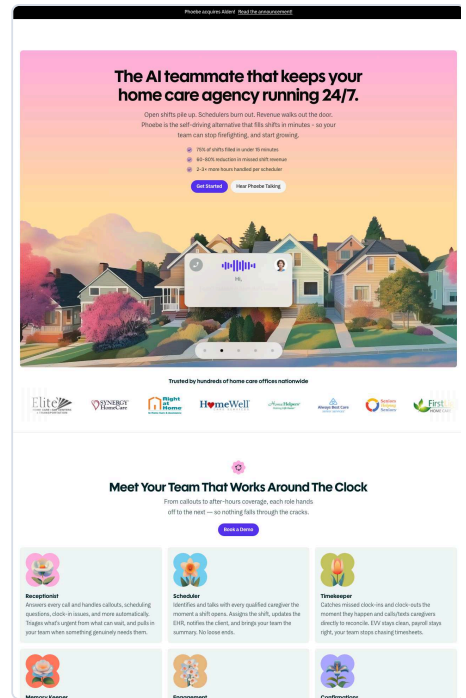
<https://get.patientspipeline.com>



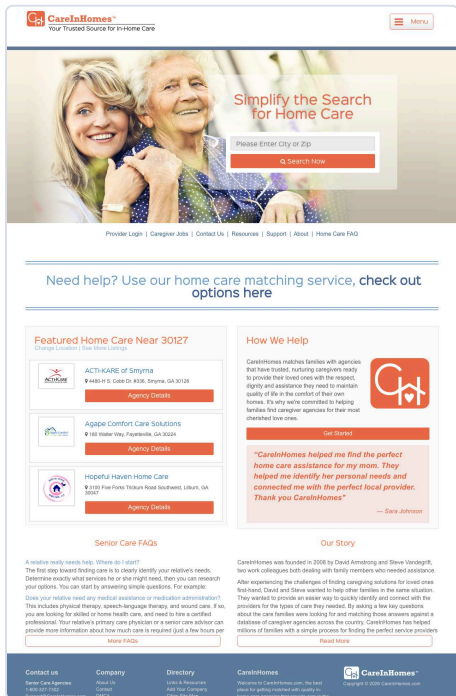
<https://www.momshouse.com>



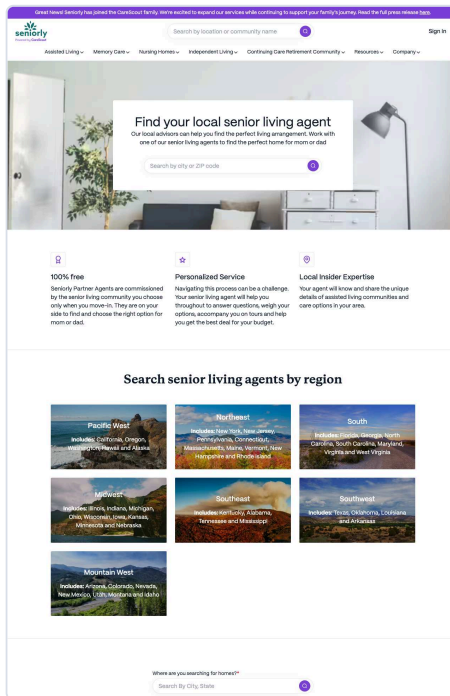
<https://vervecarepartners.com>



<https://www.phoebe.work>



<https://www.careinhomes.com>



<https://www.seniorly.com/agents>

Reviews by Star — buyer-side voice

Cleanup 2026-09-10 (Vince: 'your Reviews by Star are comparing the B2C brands'). The 487 TrustPilot reviews of A Place for Mom, Care.com, Home Instead, Right at Home, Comfort Keepers and Visiting Angels were re-read one by one: 4 are written by an agency owner or franchisee and stand below; the rest are families and workers and are excluded from this B2B page (they belong to the family-caregiver research page). What replaces them is the owner voice about each lead source from the Lead Sources ledger, shown beside the provider-side reviews.

Provider-side reviews on TrustPilot (hand-read out of 487)

- **A Place for Mom · 1★ · home care agency owner, prepaid \$580 for 10 referrals** — "I recently started a home care agency.... I recently started a home care agency and came across A Place for Mom. During our initial Zoom meeting, I shared my concerns, and they explained that I would need to prepay a total of \$580, which they described as covering 10 client referrals. My understanding was that \$58 would be deducted from this prepaid balance only for each referral that resulted in a signed client. I had done my research on them, and some revi" — <https://www.trustpilot.com/reviews/6a679ac4a98e3c28c37ba05d>
- **A Place for Mom · 1★ · provider invoiced for a referral service never activated** — "Never done business with this company. Never done business with this company, and now I have an invoice and threats to damage my credit. I received an email to sign up and activate the referral service, but I haven't signed it or sent the information back. PLEASE STAY AWAY FROM THIS FRAUD SCAMMING COMPANY. I will be starting a class action suit." — <https://www.trustpilot.com/reviews/69e26818219ff73542c4d654>
- **A Place for Mom · 1★ · provider: unqualified leads, two from another state** — "Not Worth the Time – Unqualified Leads and Bad Reputation. This company doesn't properly vet its leads. The people I was connected with were mostly just looking to talk or vent, not seriously interested in the service. Two of the leads I received were even from another state. After doing my own research, I found that the company has a poor reputation, with numerous complaints and even lawsuits filed against them." — <https://www.trustpilot.com/reviews/6807b819c805f0ccd15f63f5>
- **Right at Home · 1★ · Right at Home franchisee on franchisor support** — "If you're considering a Right at Home.... If you're considering a Right at Home franchise expecting real partnership and meaningful support to help you succeed as a business owner, be cautious. You are largely paying for the brand name and will be left mostly on your own."

I spoke directly with Mallory Hoskinson, Senior Director of Growth and Engagement, who leads the business coaching team and sets the tone for how franchisees are supported. God bless her he" — <https://www.trustpilot.com/reviews/6a30e3492e68a2feb38365d9>

What operators say about each lead source (from the Lead Sources ledger; operators first)

A Place for Mom — 42 buyer docs (40 operators) · worked 8 · burned 9 · asking 9

> "A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined the" — HOME CARE BUSINESS OWNERS · owner · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

> "LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads. Initially, it was to be a few hrs a day. I drove over 30 minutes to complete the assesment, only to be turned away. I sat with our now client and her cousin, gave her some safety tips, and I" — Homecare for newbies · Dumah Darling · owner · <https://www.facebook.com/groups/706524484973799/posts/1130570192569224>

> "Has anyone ever had a A Place for Mom actually reach out to them? They contacted me today, signed me up, waived all my fees, and even set me up in the portal to start receiving private pay clients. I usually hear people say 'A Place for Mom ain't it,' so now I'm curious... how's it going for you?" — Homecare for newbies · Anonymous participant · owner · <https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

> "Anyone received a call from a place for mom? What are your thoughts they said they found my business on the licensing web and want to see if I need clients" — HOME CARE BUSINESS OWNERS · owner · <https://www.facebook.com/groups/2156388261379184/posts/273192195049247...>

Caring.com — 8 buyer docs (8 operators) · worked 3 · burned 1 · asking 2

> "CreativeLychee1297 sometimes it takes a little bit longer to get private pay clients. Most people do pay for leads at Places like A Place For Mom or caring.com. But, if you're looking for private pay, I suggest you think outside the box." — HOME CARE BUSINESS OWNERS · Real Is to CreativeLychee1297's reply · owner · <https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>

> "I stop using a place for mom and caring.com their leads are expensive and no results. I am just using one at the moment Care In Homes their leads are \$25 per lead. But try using other method to get clients." — HOME CARE BUSINESS OWNERS · Sher Lee · owner · <https://www.facebook.com/groups/2156388261379184/posts/266158858419248...>

> "For those of you working in senior placement or senior living, have any of you had success using Caring.com, A Place for Mom, or similar referral services? We're a small independent shared living home in Idaho and have been building relationships with local placement agencies, but I want to make sur" — Senior Placement Network · owner · <https://www.facebook.com/groups/2447813518868667/posts/439960940368905...>

> "I got 3 clients through a place for mom. They send you leads so it's all about how YOU market yourself. These people need help, how else are you going to find leads in the beginning? I believe it's worth the money, it's pricey but that is what you have to take with any business it takes money to mak" — Home Health Care Networking · Sandra Williams · owner · <https://www.facebook.com/groups/2503674136547361/posts/319690059389137...>

Care.com — 1 buyer docs (0 operators) · worked 0 · burned 0 · asking 1

> "Has anyone had experience hiring hourly caregivers through Care.com? Care recommends that families start communicating via the messaging tool on their website before taking the conversation offline with a caregiver-lve messaged 2 caregivers who either asked for my number right away or gave me theirs" — agingcare · aspirant · <https://www.agingcare.com/questions/carecom-reviewsexperiences-with-pr...>

CareInHomes / Aging Care / other lead directories — 4 buyer docs (4 operators) · worked 1 · burned 2 · asking 0

> "I used place for mom and care in homes. They're not a waste to me. I got my first private pay client that turned into 24HR care. You will def need a system in place for the leads to follow up so you can convert them over. Yes some leads doesn't answer,... See more" — Homecare for newbies · Yasmine Walker · owner · <https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

> "I stop using a place for mom and caring.com their leads are expensive and no results. I am just using one at the moment Care In Homes their leads are \$25 per lead. But try using other method to get clients." — HOME CARE

BUSINESS OWNERS · Sher Lee · owner ·

<https://www.facebook.com/groups/2156388261379184/posts/266158858419248...>

> "I do careinhomes. I got 4 private pay clients and one long term care" — HOME CARE BUSINESS OWNERS · PattyLabelle Lozier · owner · <https://www.facebook.com/groups/2156388261379184/posts/283561427678990...>

> "HOME CARE BUSINESS OWNERS · Lotus Senior Care · July 7 · So, I was just on the phone with Careinhomes asking about the referral program they offer, and the guy told me that I can start with the lowest tier to get started but then wants to tell me how many zip codes I need to use for it to work. If y" — # HOME CARE BUSINESS OWNERS | So, I was just on the phone with Careinhomes asking about the referral program they offer, and the guy told me that I can start with the lowest tier t... · 70.0K members · Join group · owner · <https://www.facebook.com/groups/2156388261379184/posts/288757708159362...>

Pay-per-lead / lead-gen vendors — 11 buyer docs (11 operators) · worked 3 · burned 5 · asking 0

> "A Place For Mom.... We have never used their services and never will. We do not pay for referrals. Never will. That is a non negotiable for our agency... I am not sure how our name/number got on their list of agencies to call, but we received yet another call from them today and when we declined the" — HOME CARE BUSINESS OWNERS · owner · <https://www.facebook.com/groups/2156388261379184/posts/282822399752893...>

> "It's hard to make it make sense. You are paying \$60 for a lead that's being shared with 5 or more other agencies at the same time, so your likelihood of closing on any of them is extremely low. A much better approach would be to learn how to generate and nurture your own leads through Google and Met" — HOME CARE BUSINESS OWNERS · Edward Davis · owner · <https://www.facebook.com/groups/2156388261379184/posts/288229603545506...>

> "Another question for my fellow agency owners: Have any of you used a referral source or lead generation company for private pay clients that you found to be effective and reasonably priced? I'm looking for recommendations that don't cost an arm and a leg. I'd love to hear what has worked for you and" — HOME CARE BUSINESS OWNERS · owner · <https://www.facebook.com/groups/2156388261379184/posts/286769206024879...>

> "I'm sorry you experienced this. Unfortunately, a lot of lead companies sell the same lead to multiple home care agencies, so you end up paying to compete with several companies for the same family. I wouldn't let this discourage you, especially since you've already proven you can find a client on yo" — Home Care Business Owners Tools · Tina Hemmings · owner · <https://www.facebook.com/groups/homecareownersunite/posts/107527837501...>

Placement agents / referral agencies (CarePatrol, ALL, Oasis, independents) — 39 buyer docs (38 operators) · worked 6 · burned 1 · asking 12

> "I put together a pretty extensive list of potential referral partners. Check it out.. Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities* In" — r/RunAHomeCareAgency · owner · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxy/i_put_toge...

> "Finally licensed!. Finally licensed here in Sacramento, Roseville area and hired my first two caregivers. If anyone needs help with anything at all please let me know. I feel like an expert, at least with the licensing and start up stage. I will keep everyone posted as I go along! So many people hav" — r/RunAHomeCareAgency · owner · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

> "Finally licensed! Finally licensed here in Sacramento, Roseville area and hired my first two caregivers. If anyone needs help with anything at all please let me know. I feel like an expert, at least with the licensing and start up

stage. I will keep everyone posted as I go along! So many people have" — r/RunAHomeCareAgency · owner · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_l...

> "That depends on the type of service the referral agency offers . Do they mass fax a potential clients info to 40 facilities blind and have the faculty do ALL the work of setting up tour, following up with family OR do they do a thorough assessment of client, vet each place they refer to and hand hol" — Senior Care Networking · Nichole Casado · owner · <https://www.facebook.com/groups/seniorcarenetworking/posts/20421835258...>

Franchise systems (Home Instead, Visiting Angels, Right at Home, CarePatrol...) — 13 buyer docs (11 operators) · worked 1 · burned 2 · asking 5

> "I own a non-medial homecare agency. We are in the back 6 months of our 3rd year of operation and reached 7 figures in our 2nd year for gross income. We have found high turnover rates occur at the beginning of operations. Once you're settled with a decent book of business, you're able to find good em" — r/Entrepreneur · owner · https://www.reddit.com/r/Entrepreneur/comments/1rqjot3/pe_is_dumping_b...

> "I'd be curious what franchise.... They should be able to answer this question and provide the support and resources to get over the lull you are experiencing." — HOME CARE BUSINESS OWNERS · Kerin Staup Zuger a week ago · owner · <https://www.facebook.com/groups/2156388261379184/posts/294413039260496...>

> "Hi Chelsey. I'd be happy to speak with you more. Feel free to complete our franchise form online and we can schedule a call!" — YouTube · @KrisChana · owner · <https://www.youtube.com/watch?v=Ws6nhEzQvEw&lc=Ugz6l2duHQvQ4jwDwVB...>

> "Nice Try - But falls far short of minimum needs for Home Care businesses. We're required to use ClearCare within our home care franchise system. We used to use another very good software called HomeTrak. We embraced ClearCare about 2 years ago when our franchiser told us that they had tested it and " — trustpilot · owner · <https://www.trustpilot.com/reviews/56b24d800000ff000937464f>

The employer side (Indeed + Glassdoor) — condensed to the per-brand synthesis; every review is on the page

How pulled: Indeed through the research browser at human pace (3 pages per brand: default, lowest-rated, highest-rated — Indeed's bot wall closes after ~3 fast loads, so star-filter pages were not available; Indeed truncates review text at ~300 characters with a link to the full review) and Glassdoor (URL resolved through Google; the first page's 3 reviews plus the aggregate rating — page 2 is a login wall). 572 employer reviews across 11 brands. Star = the reviewer's own rating.

Brookdale Senior Living (Senior living / CCRC / SNF) — 65 read · 1★ 28 · 2★ 5 · 3★ 5 · 4★ 4 · 5★ 23 · Glassdoor aggregate 3.1 over 5418 reviews

- **Claude suggestion / what should change:** The operator-side lesson is in the 1★ and 4★ columns together: census drives staffing decisions and the scheduler turns over first. A growth engine sold to a Brookdale-type operator has to promise hiring ahead of move-ins and a stable scheduler/intake seat, or it just adds move-ins to a building that will 'get rid of good employees' at the next low-census month (feasibility aside).

A Place for Mom (Placement / referral agency) — 62 read · 1★ 21 · 2★ 3 · 3★ 1 · 4★ 1 · 5★ 36 · Glassdoor aggregate 3.8 over 1429 reviews

- **Claude suggestion / what should change:** For the operator buyer this is the referral source seen from inside: APFM advisors are on quota with 'not enough leads' and pitch 'places blind', which is why operators on the

ladder say the placement lead is 'the same lead sent to 50 agencies' and Five Star reports APFM move-ins under 5%. An offer that gives an operator inquiries it owns competes with this machine on the operator's side, not the advisor's.

Atria Senior Living (Senior living / CCRC / SNF) — 59 read · 1★ 26 · 2★ 3 · 3★ 3 · 4★ 6 · 5★ 21 · Glassdoor aggregate 2.7 over 1646 reviews

- **Claude suggestion / what should change:** Atria's 1★ 'All about sales' and 3★ 'belong in a skilled nursing facility' are the corporate-sales-quota culture the r/AssistedLiving sales manager described; the operator buyer here is regional/corporate, and copy that promises 'more move-ins' will be heard as more quota unless it promises qualified inquiries and staffing.

Sunrise Senior Living (Assisted living / RAL) — 56 read · 1★ 20 · 2★ 4 · 3★ 1 · 4★ 4 · 5★ 27 · Glassdoor aggregate 3.4 over 3023 reviews

- **Claude suggestion / what should change:** Sunrise's own 1★ LPN names the occupancy trap ('bring people in that aren't qualified... becomes more of skilled nursing'): move-ins beyond the care level are a staffing problem next quarter. For inLeap this is a proof point that an occupancy offer must qualify inquiries by acuity, not just book tours.

Amedisys (Home health) — 55 read · 1★ 21 · 2★ 6 · 3★ 2 · 4★ 3 · 5★ 23 · Glassdoor aggregate 3.2 over 2246 reviews

- **Claude suggestion / what should change:** Home health pays clinicians per visit, so low census is a pay cut and turnover follows; the admissions nurse under a patient quota is the intake bottleneck the trade press measures (referral acceptance under 35%). An intake/admissions offer to a home-health operator lands on both the census and the retention pain at once.

LHC Group (Home health) — 55 read · 1★ 21 · 2★ 6 · 3★ 4 · 4★ 2 · 5★ 22 · Glassdoor aggregate 3.5 over 2111 reviews

- **Claude suggestion / what should change:** Read LHC as 'branch good, corporate far away': the growth AE told no is the operator-side signal that at a consolidated agency the buyer of growth is the branch/regional, and the objection is corporate approval, not need.

VITAS Healthcare (Hospice) — 53 read · 1★ 20 · 2★ 2 · 3★ 4 · 4★ 4 · 5★ 23 · Glassdoor aggregate 3.5 over 1622 reviews

- **Claude suggestion / what should change:** The hospice operator's growth lever in these reviews is the admissions/liaison layer: the 3★ sales rep describes messaging that changes with every manager, the 1★ nurses describe caseloads that cannot absorb more admissions. A hospice growth offer that adds admissions without adding intake/caseload capacity lands in the 1★ column (hypothesis from the reviews).

Home Instead (Home care) — 52 read · 1★ 22 · 2★ 3 · 3★ 1 · 4★ 4 · 5★ 22 · Glassdoor aggregate 3.3 over 3508 reviews

- **Claude suggestion / what should change:** Home Instead's columns are the franchise map: the same brand is 1★ or 5★ depending on the owner. For inLeap the independent owner's recruiting pitch competes with a

franchise office that can offer 'plenty of hours' — hours, not applicants, are what the 3★ and 4★ caregivers ask for, which is a scheduling-density problem, i.e., a census problem.

BrightStar Care (Home care) — 50 read · 1★ 22 · 2★ 1 · 3★ 3 · 4★ 4 · 5★ 20 · Glassdoor aggregate 3.2 over 1299 reviews

- **Claude suggestion / what should change:** BrightStar's 1★ liaison and recruiter describe the two seats an owner outsources first (business development and recruiting) turning over because targets move daily; the 5★ recruiter is doing the owner's calendar. The operator buyer's hiring pain here is process, not applicant volume — a recruiting offer sold as 'more applicants' answers the wrong review.

Silverado (Memory care) — 39 read · 1★ 12 · 2★ 5 · 3★ 3 · 4★ 10 · 5★ 9 · Glassdoor aggregate 4 over 540 reviews

- **Claude suggestion / what should change:** Memory care's employer reviews repeat the senior-living pattern (sales seat under pressure, acuity creep at admission, ratios) with a memory-care-specific number: 20:1 to 40:1. An occupancy offer to a memory-care operator must carry the acuity qualification, or the 5★ nurse's 'admitted that shouldn't' becomes the 1★ caregiver's ratio.

SarahCare Adult Day Services (Adult day) — 26 read · 1★ 5 · 2★ 4 · 3★ 6 · 4★ 1 · 5★ 10 · Glassdoor aggregate 2.5 over 13 reviews

- **Claude suggestion / what should change:** 26 rows across ~10 franchised centers; the adult-day operator's staffing problem is drivers and activities staff plus transport, which the caregiver-recruiting vendors in the Ad Library do not address (gap, hypothesis).

AlayaCare (software vendor, TrustPilot) — 1 read · 1★ 1 · 2★ 0 · 3★ 0 · 4★ 0 · 5★ 0 · site score 3.2 over 1 reviews

- **Claude suggestion / what should change:** one review; not enough to synthesize beyond 'billing/invoicing is where agency software fails owners'.

ClearCare Online (software vendor, TrustPilot) — 71 read · 1★ 16 · 2★ 7 · 3★ 1 · 4★ 7 · 5★ 40 · site score 1.6 over 73 reviews

- **Claude suggestion / what should change:** The star columns are two eras: ClearCare 2015–16 (5★) vs WellSky 2021–26 (1–2★). The operator's unmet asks are the ones Vince's feed advertisers now sell around: scheduling that works on the caregiver's phone, billing/payroll math that is right, sales-rep tracking and reporting — the AI-scheduler and intake vendors (Phoebe, ClearDesk, CareIntake) are selling into WellSky's 1★ column.

No TrustPilot page: AxisCare, WellSky (Personal Care / ClearCare is the only WellSky product with a page), Eldermark, Axxess, CareVoyant, Home Care Pulse / Activated Insights — no TrustPilot page; yardi.com resolves to CondoCafe (unrelated, dropped).

Trade press & owner surveys (numbers with URLs)

- **Home care** — Results from investments in experience management and staff engagement have also shown a positive trend, with turnover rates dropping to 75%, the lowest level reported in the past five years. — Activated Insights (Home Care Pulse) 2025 Benchmarking Report press release · <https://activatedinsights.com/latest-news/activated-insights-releases-...>
- **Home care** — Agencies offering at least eight hours of onboarding and 12 hours of ongoing training, including compliance-focused content, reported an average annual revenue increase of nearly \$350,000 — Activated Insights 2025 Benchmarking Report press release · <https://activatedinsights.com/latest-news/activated-insights-releases-...>
- **Home care** — Home-based care had an aggregate 12.9% median customer growth rate in 2024 - its highest in six years. Home care client turnover dropped to 45.5% in 2024, a seven-year low. Median revenues reached \$2.3 million, an increase of more than \$291,000 (14%) compared — McKnight's Home Care on the Activated Insights 2025 report (paywalled to WebFetch; figures as shown in search summary) · <https://www.mcknightshomecare.com/news/home-care-revenues-rise-as-clie...>
- **Senior living / CCRC / SNF** — The occupancy rate for senior housing rose 0.4 percentage points in the second quarter for the 31 NIC MAP Primary Markets to reach 89.9%. The current level of 89.9% was last reached at the end of 2015, more than ten years ago. — NIC (NIC MAP) Q2 2026 occupancy · <https://www.nic.org/blog/senior-housing-occupancy-climbs-in-second-qua...>
- **Independent living** — the occupancy rate for independent living (IL) communities rose 0.3 percentage points in the second quarter to 91.3% — NIC (NIC MAP) Q2 2026 occupancy · <https://www.nic.org/blog/senior-housing-occupancy-climbs-in-second-qua...>
- **Assisted living / RAL** — the occupancy rate for assisted living (AL) communities rose 0.4 percentage points to 88.4%. The gap between AL and IL occupancy rates narrowed to only 2.9 percentage points in the second quarter. Fifteen of the 31 Primary Markets had occupancy rates at or above — NIC (NIC MAP) Q2 2026 occupancy · <https://www.nic.org/blog/senior-housing-occupancy-climbs-in-second-qua...>
- **Home health** — Referral conversion rates have declined 13 percent since 2018, dropping from 77 percent to 64 percent by Q2 2025 — Homecare Homebase, 'Home Healthcare in 2026: Demand Isn't the Problem. Capacity Is.' · <https://hchb.com/home-healthcare-in-2026-demand-isnt-the-problem-capac...>
- **Home health** — The median time from referral entry to start of care exceeds 69 hours, with more than 13 hours spent inside intake processes alone — Homecare Homebase 2026 · <https://hchb.com/home-healthcare-in-2026-demand-isnt-the-problem-capac...>
- **Home health** — CMS finalized a 1.3 percent reduction in aggregate Medicare payments to home health agencies for CY 2026. More than 11,000 Medicare-certified home health agencies serve approximately 3 million Medicare fee-for-service beneficiaries each year — Homecare Homebase 2026 (CMS CY2026 HH PPS final rule) · <https://hchb.com/home-healthcare-in-2026-demand-isnt-the-problem-capac...>
- **Home health** — As of October 2023, less than 35% of home health referrals were accepted by agencies, representing a significant decline from pre-pandemic levels when home health agencies were accepting almost half of all patients referred to them for post-acute care. In a 20 — Luna, 'Why Home Health Referral Acceptance Rates Are Collapsing' (search summary; WellSky 2023 figures) · <https://www.getluna.com/blog/home-health-referral-acceptance-rates>

- **Senior living / CCRC / SNF** — Argentum's workforce projections call for senior care to need three million-plus occupational openings by 2032, with 1.4 million workers needed in 2025 alone (281,000 new plus 1.1 million replacement). — Argentum workforce projections (search summary) · <https://www.argentum.org/wp-content/uploads/2023/03/2023-Workforce-Pro...>
- **Senior living / CCRC / SNF** — Flexible scheduling and work-life balance ranked among the most important job satisfaction factors, while burnout and understaffing were among the most frequently cited challenges in open-ended responses. — Argentum 2026 Perceptions of Careers in Senior Living (with Activated Insights) press release · <https://activatedinsights.com/latest-news/argentum-activated-insights-...>
- **Home health** — 63.3% of home health providers reported turning down referrals. Referral rejections tied to staffing shortages remain about twice as high as they were before the COVID-19 pandemic. Staffing has ranked as the industry's greatest challenge for three years runnin — Home Health Care News / Homecare Homebase 2026 outlook survey (search summary; 103 professionals, 64% C-suite/owners/VPs/directors) · <https://homehealthcarenews.com/survey-ebook/home-based-care-outlook-20...>
- **Hospice** — Average daily census was up 2.2% to 22,723, and admissions increased to 19,394 - a 6.9% jump in Q1 2026. In Florida, hospital referrals accounted for 43.8% of admissions during the first quarter. — Hospice News, 'VITAS Banks on Length of Stay, Referral Mix Management' (search summary) · <https://hospicenews.com/2026/04/27/vitas-banks-on-length-of-stay-refer...>
- **Hospice** — Referring providers value timely response to a referral for hospice care, with Hosparus' goal being to respond to referrals within 30 minutes of receipt and arrange a same-day visit. — Hospice News, 'Technology, Collaboration Pivotal to Speedy Hospice Admissions' (search summary) · <https://hospicenews.com/2026/05/06/technology-collaboration-pivotal-to...>
- **Senior living / CCRC / SNF** — Brookdale Senior Living's weighted average occupancy across communities in the second quarter was 82.4%, up 230 basis points year-over-year. Brookdale has seen measurable changes in key sales leading indicators to include conversion ratios, sales yields and im — McKnight's Senior Living, Brookdale Q2 2026 earnings (search summary) · <https://www.mcknightsseniorliving.com/news/brookdale-senior-living-sec...>
- **Placement / referral agency** — Five Star Senior Living reported that move-ins from A Place for Mom were down to less than 5% of their total move-ins. — McKnight's Senior Living (search summary; Five Star Senior Living on A Place for Mom) · <https://www.mcknightsseniorliving.com/news/a-place-for-mom-is-sold-to-...>
- **Assisted living / RAL** — A Place for Mom released a 2026 Costs of Long-Term Care and Senior Living Report based on a December survey of 820 family caregivers who had moved a loved one into a senior living community or hired in-home care. The report showed that assisted living rates in — McKnight's Senior Living on A Place for Mom 2026 Costs of Long-Term Care and Senior Living Report (search summary; 820 family caregivers surveyed) · <https://www.mcknightsseniorliving.com/news/providers-must-educate-pros...>

16 full articles were read through the research browser (press_articles.json); their numbered paragraphs are in the corpus as `trade_press` (151 entries).

Competitor marketing agencies — inLeap's direct set

These are the senior-care / home-care marketing agencies that sell lead-gen to agency owners. inLeap competes with THESE, not with the lead directories.

Agency	Services	Positioning / Guarantee
Approved Senior Network (Hurricane)	SEO, website, social media, content, reputation, review	
Grow Senior Care Marketing	SEO, Google Ads, website, social media, content, reputation	#1 In Your Area !
corecubed	SEO, Google Ads, content, review, branding	Women-Owned and Operated ## Simplify Success With c
uforocks.com	SEO, PPC, website, content, branding, email	
Sagapixel	SEO, PPC, website, social media, content	
ChoiceLocal	SEO, website, social media, content, reputation, review	\$18 in new customer revenue for every \$1
Cardinal Digital	SEO, Google Ads, PPC, website, content	

Also in the set (discovered, not yet profiled): homecaremarketing.com, turnthepagenational.com, curisdigital.com, seniorcaremarketingmax.com, carezano.com, approvedseniornetwork.com.

Read of the field: the incumbents (corecubed — women-owned, award-winning; Approved Senior Network / Steve 'the Hurricane' Weiss) sell *coaching + done-for-you channels*; the performance players (ChoiceLocal — "\$18 revenue per \$1" ROI guarantee, franchise-focused; Cardinal, Sagapixel, Grow Senior Care) sell SEO/PPC. **Gap for inLeap:** almost all sell *channels* (SEO/PPC/website); few sell an **exclusive-lead + intake-conversion system that also produces caregiver applicants**, and the ROI-guarantee lane (ChoiceLocal) is franchise-tilted — leaving independent owners underserved.

Vendor / competitor map

What owners spend on (vendor ledger, docs): Referral marketing 126 · Coaches/consultants/franchises 82 · SEO/website/GMB 33 · Lead directories (Care.com/APFM/Caring.com) 18 · Agency-management/scheduling software 18 · Recruiting/job boards 14 · Payroll/billing/EVV 12 · Caregiver training/retention 10 · Google Ads/PPC 6.

inLeap's competitive set (the "who sells to owners" landscape):

- **Coaches / communities:** Justin Currie (Master of Home Care, Skool), Steve "The Hurricane" Weiss (Home Care Evolution/HME), Homecare Owners Corner, Aaron Bogle, Coach Michele, HomeCarePulse/Activated Insights. *Angle: teach owners to do it themselves.*
- **Marketing agencies:** Sagapixel (SEO/PPC, "\$950–4k/mo"), corecubed, generic Google Ads shops. *Angle: done-for-you channels.*
- **Lead sellers:** A Place for Mom, Care.com, Caring.com, ElderCareLink, HomeAdvisor. *Angle: buy leads (resented — non-exclusive, upfront, low convert).*
- **Franchises:** Home Instead, Visiting Angels, Comfort Keepers, Right at Home, Senior Helpers. *Angle: buy the brand + playbook for \$80–150k.*
- **AI up-and-comers:** SilverCare AI and similar (the "lead-gen problem" framing in Homecare Owners Corner).

Where inLeap wins (positioning gap): owners are burned by (a) non-exclusive bought leads and (b) traffic that doesn't convert. The open lane = **exclusive, owned lead flow + an intake/conversion system that books assessments**, positioned against referral dependence — *and* a caregiver-recruiting engine (same ad machine already produces caregiver applicants: "I ran ads and got a ton of caregivers"). Two-in-one (clients + caregivers) is the differentiator no single competitor above bundles.

Competitor AD-CREATIVE sweep: DEFERRED to 09-23 (Apify caps). Backfill: Meta Ad Library for Home Instead / Visiting Angels / A Place for Mom / Care.com + the coaches, ranked by longevity, with transcripts — per D1a.

Go-deeper thread cards

One card per lead source the buyer names at least 5 times; quotes are cut from the corpus (verbatim by construction); 'love' = worked / worth-it language, 'hate' = burned language, both inside the buyer voice. The implication line is a hypothesis, labeled.

Thread: Hospital / SNF / discharge planner referral relationships (depth 1 · came up 128× in buyer voice, 115 operators · 698 docs corpus-wide)

Why it came up: "What Should You Send to Hospitals, Social Workers, Case Managers, Discharge Planners, and Physician Offices When Introducing Your Home Care Agency?" —

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...> · "Build strong personal connections with local hospitals to become their provider of choice." —

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

Love (baseline to match): "Your fastest path to a first client is almost always warm referrals — hospital discharge planners, social workers, and senior living advisors who personally hand you a name." —

<https://www.facebook.com/groups/2156388261379184/posts/284819363219864...> · "Start with: *Social workers & hospital discharge teams (visit in person if you can)* Local GP surgeries * Community groups / churches Also, word of mouth becomes your biggest asset, so even 1–2 clients done really well can snowball." —

<https://www.facebook.com/groups/1971829479945179/posts/250346596678152...>

Hate (the opening): "I've worked in SNFs for 11 years as a therapist." —

https://www.reddit.com/r/healthcareadmin/comments/sych6p/any_insight_o... · "What are your top 2 referral sources?" — <https://www.facebook.com/groups/706524484973799/posts/976062864686625>

Paid / effort (figures written next to the name): \$500.00, \$10, \$20, \$12, \$25K

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Hospital / SNF / discharge planner referral relationships..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: Coaches, courses, consultants (RAL Academy, Homecare 101, Mom's House, CSA, SRES) (depth 1 · came up 91× in buyer voice, 58 operators · 860 docs corpus-wide)

Why it came up: "Case Manager: Of course." —

<https://www.facebook.com/groups/2156388261379184/posts/290982876936845...> · "or even just training courses online without certification?" — <https://www.reddit.com/r/healthcareadmin/comments/rqk1re/certification...>

Love (baseline to match): "Selena Mack you need a coach/mentor to help you get back on track." —

<https://www.facebook.com/groups/2156388261379184/posts/279719584063175...>

Paid / effort (figures written next to the name): \$1000 (3×), \$484.80 (2×), \$750/week, \$30

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Coaches, courses, consultants..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: Networking / BNI / chamber / senior centers / community events (depth 1 · came up 62x in buyer voice, 59 operators · 211 docs corpus-wide)

Why it came up: "Don't be afraid to hand out your flyers at places like Goodwill stores and other community locations where your target audience and their families may visit." —

<https://www.facebook.com/groups/2156388261379184/posts/288420692193064...> · "Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources: Social service agencies Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones.**" —

https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

Love (baseline to match): "I'm blessed to have a wonderful support system at my church." —

<https://www.facebook.com/groups/2156388261379184/posts/289670824068051...> · "Hi Jenny Jen Congrats on getting licensed — the fact that you've already mailed 4,000 flyers and are working referral leads tells me you're serious, which is more than most." —

<https://www.facebook.com/groups/2156388261379184/posts/284819363219864...> · "Start with: *Social workers & hospital discharge teams (visit in person if you can) Local GP surgeries* * Community groups / churches Also, word of mouth becomes your biggest asset, so even 1–2 clients done really well can snowball." —

<https://www.facebook.com/groups/1971829479945179/posts/250346596678152...>

Hate (the opening): "You're better off going into hospitals and nursing homes, handing out business cards to social workers or nurses on each floor." —

<https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

Paid / effort (figures written next to the name): \$250, \$2500, \$500, \$600, \$400

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Networking / BNI / chamber / senior centers / community events..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: SEO / website / Google Business Profile (depth 1 · came up 58x in buyer voice, 53 operators · 258 docs corpus-wide)

Why it came up: "Do not put "home health" anywhere on your website." —

<https://www.facebook.com/groups/706524484973799/posts/1016591983967046> · "Besides the in person stuff, we have our website, some social media (looking at having it professionally managed)." —

https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...

Love (baseline to match): "I'm looking for someone who can help with Google SEO, Google Ads, getting my agency to rank higher on Google, improving my Google Business Profile, and bringing in more qualified leads." —

<https://www.facebook.com/groups/2156388261379184/posts/291140860921047...> · "I'd also suggest checking your Google presence (reviews + local SEO) as families often search there first now." —

<https://www.facebook.com/groups/1971829479945179/posts/250346596678152...> · "Does your website focus on SEO?" — <https://www.facebook.com/groups/2156388261379184/posts/284819363219864...>

Paid / effort (figures written next to the name): \$250, \$2500, \$500, \$600, \$400, \$20,000 per month, \$30m

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried SEO / website / Google Business Profile..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: A Place for Mom (depth 1 · came up 42x in buyer voice, 40 operators · 351 docs corpus-wide)

Why it came up: "LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads." — <https://www.facebook.com/groups/706524484973799/posts/1130570192569224> ·

"Has anyone ever had a A Place for Mom actually reach out to them?" —

<https://www.facebook.com/groups/706524484973799/posts/1061369272822650>

Love (baseline to match): "Don't use a place for mom!" —

<https://www.facebook.com/groups/2156388261379184/posts/284819363219864...> · "I am a home Care agency in central Nebraska and I do use a place for Mom they do help find clients it's worked out for me" —

<https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

Hate (the opening): "A Place For Mom...." —

<https://www.facebook.com/groups/2156388261379184/posts/282822399752893...> · "I used place for mom and care in homes." — <https://www.facebook.com/groups/706524484973799/posts/1061369272822650> · "We have wasted time and money with A Place for Mom." —

<https://www.facebook.com/groups/706524484973799/posts/976062864686625>

Paid / effort (figures written next to the name): \$25 per lead, \$600, \$68 per lead

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried A Place for Mom..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: Placement agents / referral agencies (CarePatrol, ALL, Oasis, independents) (depth 1 · came up 39x in buyer voice, 38 operators · 174 docs corpus-wide)

Why it came up: "Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources: Social service agencies Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living* Hair salons **Don't forget past/current clients and their loved ones.**" —

https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge... · "There are 5 modules everything from email domination for placement agents that got me 3 placement agents (I sent it to 36 agent total), how to hire caregivers and all the marketing things I did." —

https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_I...

Love (baseline to match): "As the Director of Sales and Marketing, I use my network of healthcare personnel and work closely with the community, senior placement agencies, CCA, social workers, discharging nurses, case managers, Administrators/Executive Directors, Skill Nursing Facilities, Hospitals, and other healthcare industry organizations to bring in new ALWP and private pay residents to Senior Living / Assisted Living and Memory Care Facilities and increase facility's census." —

<https://www.facebook.com/groups/assistedlivingfacilities/posts/3896882...>

Paid / effort (figures written next to the name): \$1.1 m

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Placement agents / referral agencies..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: Insurance panels / pediatricians / schools (ABA) (depth 1 · came up 25x in buyer voice, 23 operators · 104 docs corpus-wide)

Why it came up: "I do all the aspects of business and clinical myself - including credentialing, billing, marketing, and provide all clinical services myself." —

https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra... · "Update insurance contracts to accept a wider range of plans." —

<https://www.facebook.com/groups/nursinghomeadmin/posts/779910030014368...>

Hate (the opening): "I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach." —

<https://www.facebook.com/groups/669886889110965/posts/942369488529369> · "I'm credentialed with all the insurance companies that currently have their panels open, and I've tried to be proactive with outreach." —

<https://www.facebook.com/groups/1081018657046251/posts/143052813209530...> · "This list is vendors and commercial companies that actually send you clients once you're credentialed with them." —

<https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Insurance panels / pediatricians / schools..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: Google Ads / LSA / PPC (depth 1 · came up 17x in buyer voice, 16 operators · 49 docs corpus-wide)

Why it came up: "Continue running a small ad in Google Ads- say \$3 a day." —

<https://www.facebook.com/groups/2447813518868667/posts/439960940368905...> · "Will be opening my non-medical home care business in a month and will start with Facebook advertising and targeted google ads in my area." — https://www.reddit.com/r/HomeCareOwners/comments/1i6q4vl/tips_on_how_t...

Love (baseline to match): "I'm looking for someone who can help with Google SEO, Google Ads, getting my agency to rank higher on Google, improving my Google Business Profile, and bringing in more qualified leads." — <https://www.facebook.com/groups/2156388261379184/posts/291140860921047...> · "One thing I've seen working with home care agencies is that many invest heavily in Google Ads before fixing the foundation." —

<https://www.facebook.com/groups/2156388261379184/posts/291140860921047...> · "We've seen a lot of success for our Senior Living clients with a combined strategy of local SEO, PPC, Performance Max and a combination of Facebook Ads and Facebook Retargeting!" —

https://www.reddit.com/r/SeniorLivingMarketing/comments/1l4a6ip/how_to...

Hate (the opening): "I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads." —
<https://www.facebook.com/groups/669886889110965/posts/942369488529369> · "I've gone in person to doctor's offices with brochures and small goodie bags, sent out emails wherever I could find contacts, and I've been running Instagram, Facebook, and Google ads." —

<https://www.facebook.com/groups/1081018657046251/posts/143052813209530...>

Paid / effort (figures written next to the name): \$3 a day, \$15, \$2, \$1,700 per month

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Google Ads / LSA / PPC..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: VA / Medicaid waiver / LTC insurance contracts (depth 1 · came up 16x in buyer voice, 14 operators · 89 docs corpus-wide)

Why it came up: "Healthcare referral sources: *Home health agencies Hospices Assisted living facilities Continuing care retirement communities Hospital discharge planners Doctors offices Skilled nursing facilities Independent living facilities House call physicians Rehab centers (outpatient) Rehab facilities (inpatient) Pharmacies Other home care agencies* Diagnosis-specific support groups *Neurologists Other non-healthcare professional referral sources: Social service agencies Workers compensation providers/case managers Bank trust officers Real estate agencies CPAs VA programs Financial planners Geriatric care managers Adult day care centers Churches/clergy Occupational therapists Funeral directors Fiduciaries Elder law attorneys Estate planners Country clubs House cleaning services Senior communities Social workers Care management agents Placement agencies Independent living Hair salons* **Don't forget past/current clients and their loved ones.**" —

https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge... · "Which is the medicaid waivers for IDD and AMI and Seniors." —

<https://www.facebook.com/groups/706524484973799/posts/1052048583754719>

Love (baseline to match): "If there is compensation, what does that normally look like for private pay vs Medicaid waiver residents?" — <https://www.facebook.com/groups/2886092941628251/posts/451427707880982...>

Paid / effort (figures written next to the name): \$750/week

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried VA / Medicaid waiver / LTC insurance contracts..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Thread: Franchise systems (Home Instead, Visiting Angels, Right at Home, CarePatrol...) (depth 1 · came up 13x in buyer voice, 11 operators · 248 docs corpus-wide)

Why it came up: "We are not certified with Medicaid and we are part of a franchise." —

https://www.reddit.com/r/Entrepreneur/comments/1rqjot3/pe_is_dumping_b... · "I'd be curious what franchise...." — <https://www.facebook.com/groups/2156388261379184/posts/294413039260496...>

Love (baseline to match): "> As being one of the first franchisees with CarePatrol I totally disagree with the comments made by the anonymous submission posted on Sunday, August 1, 2010." —

<https://www.unhappyfranchisee.com/carepatrol-franchisees-praise-the-ca...>

Hate (the opening): "We're required to use ClearCare within our home care franchise system." —

<https://www.trustpilot.com/reviews/56b24d800000ff000937464f> · "Unfortunately mistakes can happen and this is

sad but there are good facilities out there that care for seniors and Home Instead is a good company." — <https://www.unhappyfranchisee.com/home-instead-senior-care-franchise>

Feeds: Tab 5 · Tab 6 · Lead Sources tab · copy line: "You've probably tried Franchise systems..."

Stopped: depth 2 not run this pass (the vendor's own reviews are in Reviews by Star where a page exists).

Headline candidates (Tab 11)

Ranked: operators first, then engagement. Each is the theme-matched sentence of a buyer voice, verbatim.

1. "I do all the aspects of business and clinical myself - including credentialing, billing, marketing, and provide all clinical services myself." (↑66) · https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...
2. "I am concerned about the high staff turnover rate in childcare." (↑50) · <https://www.facebook.com/groups/248675758568554/posts/6929343677168362>
3. "My name is Obi, and I'm the Administrator of De-Mondek Home Care Provider Services LLC." (↑48) · <https://www.facebook.com/groups/2156388261379184/posts/290982876936845...>
4. "Today, as I write this, all 94 of our beds are full and we have a wait list." (↑28) · <https://www.facebook.com/groups/352285274494478/posts/976832502039749>
5. "I'm starting out with private pay clients only." (↑27) · <https://www.facebook.com/groups/1154838852454654/posts/158909693569550...>
6. "I put together a pretty extensive list of potential referral partners." (↑24) · https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
7. "I am still not landing any leads for my Private Home Care Provider Agency." (↑21) · <https://www.facebook.com/groups/2156388261379184/posts/265418003493333...>
8. "LONG RANT WARNING: We spent a lot of money on APFM and then we landed a 24 hr client from one of their leads." (↑13) · <https://www.facebook.com/groups/706524484973799/posts/1130570192569224>
9. "What are your thoughts they said they found my business on the licensing web and want to see if I need clients" (↑13) · <https://www.facebook.com/groups/2156388261379184/posts/273192195049247...>
10. "I got my first private pay client that turned into 24HR care." (↑11) · <https://www.facebook.com/groups/706524484973799/posts/1061369272822650>
11. "I have a referral list where you can connect and foster relationships so they'll send them your way." (↑11) · <https://www.facebook.com/groups/1429922527043737/posts/981818785488378...>
12. "I was hopeful they would add my agency to their referral list, but I've been ignored." (↑11) · <https://www.facebook.com/groups/2156388261379184/posts/289670824068051...>
13. "It took me over a YEAR to start and submit because I literally trusted no one." (↑11) · https://www.reddit.com/r/RunAHomeCareAgency/comments/1cejzsp/start_you...
14. "I'm wanting to get some insight from a BCBA who has started their own LLC." (↑11) · <https://www.facebook.com/groups/7447177152061328/posts/238805664882958...>
15. "Your first two years will be your hardest I know agencies that opened 2 years now and still no clients." (↑11) · <https://www.facebook.com/groups/2156388261379184/posts/283021973066269...>
16. "Finally licensed here in Sacramento, Roseville area and hired my first two caregivers." (↑10) · https://www.reddit.com/r/RunAHomeCareAgency/comments/18j3pm9/finally_i...
17. "It has worked for me this way for years.." (↑10) · <https://www.facebook.com/groups/706524484973799/posts/1008111701481741>
18. "I'm a Virtual Assistant, and I can help market your home care business by emailing and calling potential referral sources on your behalf." (↑9) · <https://www.facebook.com/groups/2156388261379184/posts/288420692193064...>

19. "My ads typically cost me around \$6 per lead and my close rate is pretty high." (↑9) · <https://www.facebook.com/groups/2156388261379184/posts/288229603545506...>
20. "Wasted my time and money." (↑8) · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>
21. "Hi, I got my first nursing home gig and I do not understand PPD at all." (↑7) · <https://www.facebook.com/groups/nursinghomeadmin/posts/928920600446643...>
22. "I am looking for great ideas on how to get my first clients." (↑7) · https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...
23. "I had one person looking for someone to take his family member to their house on the weekends." (↑7) · <https://www.facebook.com/groups/706524484973799/posts/738060561820191>
24. "I have tried them and it was a waste.They send you referrals and bill you per referral whether you sign them up or not that bill grows quickly..never again" (↑7) · <https://www.facebook.com/groups/2018920001893909/posts/234693166242607...>
25. "I personally dont like using referral agents because some can be very greedy." (↑7) · <https://www.facebook.com/groups/2447813518868667/posts/402589742772692...>

Frequency ranking (buyer-voice complaint patterns)

1. How to start / consultant / is it worth it — 249 (whole corpus 685)
2. Reimbursement / Medicaid / private pay rates — 149 (whole corpus 887)
3. Referral sources & placement agents: dependence, fees, how to get them — 135 (whole corpus 618)
4. Marketing: ads / digital / SEO / website — 113 (whole corpus 551)
5. Referral relationships: hospitals, discharge planners, SNFs, physicians — 109 (whole corpus 491)
6. Licensing / accreditation / compliance / regs (agency or facility license, not a personal one) — 103 (whole corpus 646)
7. Can't get clients / low census / need leads — 90 (whole corpus 357)
8. ABA: credentialing, insurance panels, waitlists, referrals — 88 (whole corpus 972)
9. Google Ads / Facebook ads / SEO / website that didn't pay — 76 (whole corpus 308)
10. Franchise vs independent — 70 (whole corpus 557)
11. Scheduling / software / operations / EVV — 69 (whole corpus 415)
12. Starting a facility: cost to start, existing license, administrator, partner — 64 (whole corpus 242)
13. Facility economics per bed: private pay vs Medicaid, cash flow, margins — 62 (whole corpus 250)
14. Sales / networking / building referral relationships — 62 (whole corpus 223)
15. Licensing, the state survey, the ombudsman, deficiencies (facility side) — 60 (whole corpus 356)
16. Caregiver retention at scale (turnover caps growth AND valuation) — 55 (whole corpus 618)
17. A Place for Mom / Caring.com / Care.com: paid referral sites — 51 (whole corpus 529)
18. Placement agents / referral fees / commission — 48 (whole corpus 165)
19. Owner-operator treadmill (build management/systems, step back) — 44 (whole corpus 399)
20. Census stability & growth (predictable clients to SCALE) — 37 (whole corpus 209)
21. Owner burnout / wearing all hats / on-call — 27 (whole corpus 286)
22. Exit / valuation / EBITDA multiple / selling the agency — 22 (whole corpus 231)
23. Occupancy / census pressure: empty beds, quotas, wait list — 22 (whole corpus 83)
24. Caregiver turnover / no-show / can't retain — 18 (whole corpus 222)
25. Caregiver pay / wages / can't compete on pay — 18 (whole corpus 204)
26. Cash flow / payroll / making payroll — 16 (whole corpus 118)
27. Shared leads that don't convert / pay-per-lead burn — 15 (whole corpus 46)
28. Admissions / referral response (hospice, home health, liaison) — 14 (whole corpus 120)
29. Can't find / hire / recruit caregivers — 13 (whole corpus 180)
30. Independent referral agent: how do I get clients (transition specialist, SRES, placement) — 8 (whole corpus 63)
31. Referral / payor concentration risk (de-risk the multiple) — 7 (whole corpus 31)
32. Chronic understaffing at facilities: ratios, call-ins, agency staff — 7 (whole corpus 154)
33. Corporate / PE ownership vs family-owned: quotas from corporate — 6 (whole corpus 77)
34. Scaling past the revenue plateau — 5 (whole corpus 51)

- 35. Competition / saturated market — 4 (whole corpus 31)
- 36. Marketing agencies / consultants that burned them — 4 (whole corpus 27)
- 37. Tour metrics and 'sign the first visit' sales pressure — 4 (whole corpus 17)
- 38. Lender / funding / capital for a facility — 3 (whole corpus 78)
- 39. PE roll-ups & consolidation (compete or get bought) — 2 (whole corpus 62)
- 40. Client churn from unreliable staffing (client-side) — 0 (whole corpus 7)
- 41. Family price shock: \$7–8k a month and 'is this what we get' — 0 (whole corpus 41)

Precision (20-hit random reads, buyer corpus): 'How to start / consultant' 17 of 20 on topic (one TV-commercial post, one family review, one vendor). 'Referral sources & placement agents' 17 of 20 (two vendor comments, one aspirant question). 'Licensing' was 13 of 20 on the first pattern (a realtor's licence, a job posting, a mental-health platform leaked); the pattern now requires an agency/facility licence object and the count fell accordingly. Counts are ranks, not pain-point sizes; 'how to start' is the largest because the sources are start-up heavy, and it is the aspirant's theme, not the operating owner's. **Random-40 split of the buyer corpus (seed 2026, read 2026-09-09):** 12 support the client-acquisition pain narrative (no clients after licensing, paid A Place for Mom and got nothing, dependent on placement agents, 'what has worked best for finding residents'), 3 contradict it (a referral site that produced a first 24-hour client, praise for scheduling software, joy at the licence), 25 neutral (one-line replies inside group threads, exam prep, a partnership offer). The neutral majority is the shape of group conversation, not a sign the pain is rare; the tabs sort the 12 kind to the top and the reader can open every theme to see the rest.

Vocabulary ledger

Three-word phrases (min 4 docs)

# docs	phrase	top source
37	place for mom	https://www.facebook.com/groups/2156388261379184/posts/282822399752893...
21	assisted living facility	https://www.facebook.com/groups/352285274494478/posts/976832502039749
17	word of mouth	https://www.facebook.com/groups/2018920001893909/posts/239152230796700...
11	assisted living communities	https://www.facebook.com/groups/2156388261379184/posts/288420692193064...
11	assisted living facilities	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
11	google business profile	https://www.facebook.com/groups/2156388261379184/posts/291140860921047...
11	residential assisted living	https://www.reddit.com/r/realestateinvesting/comments/uujosm/is_reside...
9	policies and procedures	https://www.reddit.com/r/RunAHomeCareAgency/comments/1b5d8qo/is_a_cons...
8	elder law attorneys	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
7	process of opening	https://www.facebook.com/groups/201561673732649/posts/1978369336051865
6	chamber of commerce	https://www.facebook.com/groups/2156388261379184/posts/289670824068051...
6	hospitals rehab centers	https://www.facebook.com/groups/2156388261379184/posts/276366040398529...
6	independent living facilities	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
6	trying to figure	https://www.reddit.com/r/smallbusiness/comments/1hr7ffg/looking_to_sta...
5	aba business owners	https://www.facebook.com/groups/669886889110965/posts/975664455199872
5	able to find	https://www.facebook.com/groups/1079987009361321/posts/185201265215874...
5	free to reach	https://www.facebook.com/groups/2156388261379184/posts/288420692193064...
5	geriatric care managers	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
5	happy to share	https://www.facebook.com/groups/2156388261379184/posts/284819363219864...
5	hospital discharge planners	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
5	meet our needs	https://www.reddit.com/r/RunAHomeCareAgency/comments/1dbg9dl/schedulin...
5	peace of mind	https://www.reddit.com/r/AssistedLiving/comments/1kyorry/years_of_work...
5	senior living marketing	https://www.reddit.com/r/SeniorLivingMarketing/comments/1q9p8wm/who_i_...
5	sent you message	https://www.facebook.com/groups/2156388261379184/posts/273224561379344...
5	skilled nursing facilities	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

Two-word phrases (min 4 docs)

# docs	phrase	top source
106	assisted living	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
97	private pay	https://www.facebook.com/groups/1154838852454654/posts/158909693569550...

38	non medical	https://www.facebook.com/groups/1154838852454654/posts/162114221249098...
36	senior living	https://www.facebook.com/groups/2156388261379184/posts/284819363219864...
32	case managers	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...
30	hi everyone	https://www.facebook.com/groups/1154838852454654/posts/158909693569550...
29	feel free	https://www.facebook.com/groups/2156388261379184/posts/284819363219864...
29	social workers	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...
26	living facility	https://www.reddit.com/r/AssistedLiving/comments/1kyorry/years_of_work...
26	referral sources	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...
23	discharge planners	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...
21	social media	https://www.reddit.com/r/RunAHomeCareAgency/comments/1d6v0n4/looking_f...
18	anonymous participant	https://www.facebook.com/groups/2156388261379184/posts/285582417143558...
18	greatly appreciated	https://www.reddit.com/r/Entrepreneur/comments/1alxdam/nonmedical_home...
17	hello everyone	https://www.reddit.com/r/RunAHomeCareAgency/comments/1dfnzpx/mental_he...
17	mental health	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
17	real estate	https://www.reddit.com/r/realestateinvesting/comments/uujosm/is_reside...
16	independent living	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
16	insurance companies	https://www.youtube.com/watch?v=zvtl029wKTU&lc=Ugznprrsa1TsTp-QrY1...
16	living facilities	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...
14	nursing homes	https://www.facebook.com/groups/2156388261379184/posts/284819363219864...
13	google ads	https://www.facebook.com/groups/2156388261379184/posts/291140860921047...
13	living communities	https://www.facebook.com/groups/2156388261379184/posts/288420692193064...
13	long term	https://www.reddit.com/r/realestateinvesting/comments/14u5xo2/im_looki...
12	loved ones	https://www.reddit.com/r/RunAHomeCareAgency/comments/zzgxyl/i_put_toge...

Single words (min 4 docs)

# docs	phrase	top source
163	living	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
147	marketing	https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...
146	private	https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...
143	well	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
140	health	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
139	first	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...
137	community	https://www.facebook.com/groups/2156388261379184/posts/282822399752893...
127	services	https://www.reddit.com/r/bcba/comments/1dkhd7e/i_started_a_private_pra...
126	find	https://www.reddit.com/r/nursing/comments/1qijhrs/got_a_text_about_bei...
119	senior	https://www.reddit.com/r/eldercare/comments/1g5gzvc/looking_for_some_f...

119	you're	https://www.reddit.com/r/RealEstate/comments/16dcpic/how_i_see_the_bub...
116	agencies	https://www.facebook.com/groups/2156388261379184/posts/282822399752893...
115	referral	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...
111	referrals	https://www.facebook.com/groups/2156388261379184/posts/282822399752893...
110	doing	https://www.reddit.com/r/eldercare/comments/1g5gzvc/looking_for_some_f...
109	assisted	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
105	call	https://www.facebook.com/groups/2156388261379184/posts/282822399752893...
103	everyone	https://www.facebook.com/groups/352285274494478/posts/976832502039749
103	take	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
99	working	https://www.facebook.com/groups/248675758568554/posts/6929343677168362
98	experience	https://www.reddit.com/r/eldercare/comments/1g5gzvc/looking_for_some_f...
97	company	https://www.reddit.com/r/ABA/comments/1i2h1sq/i_passed/
96	through	https://www.reddit.com/r/AgingParents/comments/1ujtemb/he_wouldnt_inco...
94	family	https://www.reddit.com/r/eldercare/comments/1g5gzvc/looking_for_some_f...
93	leads	https://www.facebook.com/groups/2156388261379184/posts/290982876936845...

Awareness → Offer Map

1. PROBLEM-AWARE

Where they are: Operators who know the phone is not ringing enough: 'I need clients', 'census is down', 'I'm way too dependent on placement agents' — and still equate marketing with visiting facilities and word of mouth.

"I've become way too dependent on the placement agents that have been bringing me clients."

Messaging that works here: "The families are searching tonight. Are they finding you, or A Place for Mom?" · "You do not have a marketing problem. You have a phone-not-ringing problem."

- **DIY:** Free teardown: where your next 10 clients / residents / patients are actually going to come from, from your market's own search and ad data.
- **DWY:** We build the inbound engine; you keep your referral relationships and answer the phone.
- **DFY:** Done-for-you client acquisition: ads + landing page + intake, so the family calls you and only you.

2. SOLUTION-AWARE

Where they are: They have tried a referral site, a placement agent or an ads vendor and know a system is what is missing — they are asking the group 'what lead companies actually work for private pay?'

"\$58 per 'possible' lead, sent to 4 other agencies, is a complete scam."

Messaging that works here: "Leads that are yours. Not shared with four other agencies." · "Stop renting families from A Place for Mom. Own the inquiry."

- **DIY:** The exclusive-lead playbook: what the shared-lead sites charge vs. what an owned inquiry costs.
- **DWY:** We run acquisition on your own brand; you see every call and every cost.
- **DFY:** Exclusive inquiries, intake handled, cost per admission reported weekly.

3. PRODUCT-AWARE

Where they are: Comparing agencies, pay-per-lead vendors, coaches and DIY Google Ads. They want proof before a retainer.

"I'd pay for results, not a retainer."

Messaging that works here: "Pay for admissions, not impressions." · "Show me the cost per move-in before I sign anything."

- **DIY:** Comparison sheet: pay-per-lead vs. agency retainer vs. owned engine, on your numbers.
- **DWY:** 90-day pilot on one service line or one building; scale on the measured cost per client.
- **DFY:** Performance-priced acquisition tied to booked assessments, tours or admissions.

4. MOST AWARE

Where they are: Operators with a full building or a full caseload who want the next one, the second location, or a sale at a better multiple.

"Occupancy up, then refinance or sell at a higher valuation."

Messaging that works here: "Fill the next building before the lender asks." · "Census you own is census a buyer pays for."

- **DIY:** Expansion checklist: what a second location needs from day one.
- **DWY:** Pre-opening waitlist campaign for the next building.
- **DFY:** Multi-location acquisition engine with per-building reporting.

Operator ladder, speaker gate and sources

- **Ladder expansion (09-08 afternoon)** — Reddit `.json` through the browser rate-limits at ~100 requests / 10 min (429 at 12:36); re-paced to ~1 request / 6.5 s with backoff. YouTube Data API: the brief's key is invalid, the key in `reference_research_accounts` works (free). supadata free. Indeed and Glassdoor read through the browser at human pace; both wall after ~3 fast loads. Apify \$0 on every account — unused.
- **Reddit** — logged-in research browser (Own-Clerk-6667), free/unlimited via in-page `.json`. Primary VOC.
- **YouTube Data API v3** — free; 960 owner-comment records across 45 coach videos.
- **Supadata** — free; 56 coach transcripts (580K chars).
- **Tavily** — Researcher plan, ~880 credits left; forum/Quora discovery + extract.
- **Facebook** — logged in (Angela); 9 owner groups joined. Cold-feed yield thin (new-account behavior); warms over 1–2 weeks.
- **Apify** — ~\$0.94 across accounts; used ~\$0.62 on the initial Reddit calibration before the browser path replaced it. Competitor **ad-creative** sweep deferred to the 09-23 reset (see Backfill).
- **SerpAPI** — 0/250 (dead until Oct 1).

D1 — Source Matrix (what was pulled)

Corpus now 7,401 documents after the 2026-09-08 operator-ladder expansion (see D1I below for what was added per vertical). The paragraph that follows describes the first home-care-only pull of 09-08 morning and is kept as the baseline.

Corpus = **850 entries**, 565 with real engagement scores. Reddit 395 (r/RunAHomeCareAgency 116 owner core; r/nursing 101 + r/CNA 85 + r/caregivers 54 labor; r/Entrepreneur/buyingabusiness/smallbusiness industry; Go-Deeper entity spin-offs care.com / A Place for Mom / franchise). YouTube owner-comments 359. agingcare (client-side staffing symptom, focused) 49. allnurses/Quora forums 45. FB owner groups 2. Plus a **56-video coach transcript corpus** (Justin Currie / Master of Home Care, Steve "The Hurricane" / Home Care Evolution, Sagapixel, Homecare Owners Corner, Aaron Bogle, RealScottMcKenzie, Coach Michele, +14) for the expert/Previous-Solutions/vendor layer.

Honest gaps: r/homehealth is a dead sub (not usable). Owner *emotional* content (Tab 3, Tab 9 owner-scenes) is thin — the owner sub is tactical, not confessional. Competitor ad creatives = 09-23 Apify backfill.

D1I — Per-vertical pulls (kept from 09-08) (the operator ladder, 2026-09-08)

Every corpus entry carries a **pull scope** (the subreddit, keyword, channel, brand or file it came from — stated per file, never inferred from the speaker) and the **verticals its text names** (word-bounded regex). Counts are documents. *Before* = the 09-08 morning corpus (home-care pulls only, 3,679 documents); *after* = this expansion (7,401 documents). Documents containing the word "occupancy": 0 → 25.

Vertical	Pulled (this expansion)	Docs by pull scope, before → after	Docs naming it, before → after	Gaps (honest)
Home care	r/RunAHomeCareAgency deepened (scheduler/intake, recruiting, census/private pay/VA/LTC, sold/exit/PE/burnout, pay-per-lead/agencies/ads); Ad Library 'home care agencies', 'caregiver applicants', 'hire more caregivers', 'recruit caregivers', 'home care leads', 'schedulers', 'scheduler'; feed advertisers by name; TrustPilot ClearCare (71) + AlayaCare (1); Indeed/Glassdoor Home Instead + BrightStar; r/cna + r/nursing brand searches	2,494 → 2,689	697 → 816	r/homehealth dead; AxisCare / WellSky / Alora have no TrustPilot page
Home health	r/Entrepreneur, r/smallbusiness, r/buyingabusiness 'home health agency'; r/RunAHomeCareAgency Medicare/PDGM/OASIS/skilled; r/homehealthcare + r/healthcareadmin (sup pass); YouTube 2 operator videos + 27 transcripts (referrals, sales, OASIS, Medicaid rates, franchise); Ad Library 'home health agency owners', 'home health agencies/agency', 'grow your census', 'census'; Indeed/Glassdoor Amedisys + LHC; trade press HHCN, McKnight's Home Care, WellSky/Luna referral-acceptance numbers	136 → 440	294 → 421	r/HealthcareAdministration returns 404 (the live sub is r/healthcareadmin, added in the sup pass); r/homehealth dead; 'increase census' and 'home health referrals' phrases return 0 ads
Hospice	r/hospice owner/administrator/census/admissions/liaison/'my hospice'/for-profit/ADC (owner-only filter in build_halo), r/Entrepreneur + r/smallbusiness + r/buyingabusiness 'hospice'; YouTube 10 operator videos (start a hospice, license, \$3M hospice, sales) + 4 transcripts; Ad Library 'hospice owners/admissions/marketing/agencies/agency/providers', 'more admissions', 'hospice referrals'; Indeed/Glassdoor VITAS; Hospice News (VITAS Q1 census + admissions, 30-minute referral response, 2026 trends); r/cna + r/nursing 'VITAS'	4 → 419	65 → 354	r/hospice is families and nurses first — operator voice is the minority even after the owner filter; 'hospice growth' phrase 0 ads
Assisted living / RAL	r/AssistedLiving five owner/occupancy/marketing/staffing/licensing searches + r/Entrepreneur, r/smallbusiness, r/buyingabusiness, r/realestateinvesting, r/sweatstartup 'assisted living / RAL / personal care home / group home'; YouTube 16 RAL-operator videos (RAL Academy, Assisted Living Investing, 'How to start an ALF', \$12K/month) + 4 transcripts; Ad Library 18 phrases (owners, operators, RAL, move-ins, booked tours, waitlist, fill your beds, empty beds/rooms, more tours, facilities, business...); Indeed/Glassdoor Sunrise; Feed: Dr. Mark Stevens, Wisdom First, Searchlift, Occupancy Partners	98 → 1,541	186 → 864	r/AssistedLiving is mostly families and line staff; owner voice comes from the general subs and YouTube; 'assisted living marketing/leads' phrases return 0 ads

Memory care	r/AssistedLiving 'memory care' owner/occupancy/staffing/marketing/census; r/SeniorLivingMarketing; r/Entrepreneur 'memory care' (0 posts); YouTube 6 videos + 4 transcripts; Ad Library 'memory care communities/community' (60 ads), 'memory care marketing' (0); Indeed/Glassdoor Silverado; r/cna + r/nursing 'Silverado'	1 → 235	25 → 144	the one memory-care operator-marker document in the corpus is a family member with an admin background; memory-care operators speak as assisted-living operators (same buildings) — treat memory care as a wing of AL/senior living in copy, not a separate buyer voice
Independent living	r/AssistedLiving 'independent living'; r/realestateinvesting 'independent living / 55+'; r/SeniorLivingMarketing (sup); YouTube 1 operator video (63 comments) + 1 transcript (91K chars); Ad Library 'independent living communities/community' (57 ads), 'independent living community marketing' (0); trade press NIC occupancy 91.3%	0 → 146	24 → 99	r/seniorliving does not exist (404) — three planned searches replaced by r/AssistedLiving + r/SeniorLivingMarketing (67 members); the IL operator voice online is the 'group home / ILF for veterans' starter, not the 55+ community operator
Senior living / CCRC / SNF	r/nursinghome administrator/census/occupancy/staffing/survey/marketing; r/Entrepreneur, r/smallbusiness, r/buyingabusiness 'senior living / nursing home / senior housing'; r/AssistedLiving + r/SeniorLivingMarketing sales/occupancy/ED searches (sup); YouTube 4 videos + 4 transcripts; Ad Library 'senior living communities/operators/sales/industry', 'occupancy', 'occupancy rate', 'increase occupancy' (132 ads); Indeed/Glassdoor Brookdale + Atria; Senior Housing News, McKnight's Senior Living (Brookdale 82.4% occupancy), NIC, Argentum workforce; r/cna + r/nursing 'Brookdale', 'Atria'	0 → 449	111 → 428	r/seniorliving 404; 'senior living directors/marketing/leads' phrases return 0 ads — the vendors say 'occupancy', not 'leads'
Adult day	r/Entrepreneur, r/smallbusiness, r/RunAHomeCareAgency, r/AssistedLiving 'adult day' + all-reddit 'adult day care owner/start/census'; YouTube 11 operator videos (Adult Day Care Academy, VA/Medicaid enrollment, NY \$400M) + 4 transcripts; Ad Library 'adult day center/care center/centers' (52 ads), 'adult day center marketing' (0); Indeed SarahCare	0 → 296	14 → 100	no adult-day subreddit exists; the voice is YouTube comments under one coach's channel — a single-source risk, said so
Placement / referral agency	r/Entrepreneur, r/smallbusiness, r/AssistedLiving, r/SeniorLivingMarketing, r/RunAHomeCareAgency 'placement agent/agency/referral fee/A Place for Mom' + all-reddit 'senior placement business'; YouTube 8 operator videos (start a senior placement agency, 5 hard truths, day in the life) + 4 transcripts; Ad Library 'placement agency' (30), 'senior placement' (18), 'referral agency', 'placement agents', 'senior placement agency'; TrustPilot A Place for Mom + Caring.com (from the first run); Indeed/Glassdoor A Place for Mom; McKnight's (Five Star: APFM move-ins under 5%)	163 → 278	83 → 141	placement-agent owners are few and post little; 'senior care leads' and 'pay per lead senior care' phrases return 0 ads (VERVE, the pay-per-lead advertiser in the feed, was found by name only)

Multi-location medical	r/healthcareadmin (sup pass; r/HealthcareAdministration 404), r/Entrepreneur 'medical practice / clinic multiple locations'; YouTube 5 healthcare-marketing videos + 4 transcripts; Ad Library 'multi-location medical' (25), 'multi-location practices', 'multiple locations', 'practice owners' (94 ads, mostly dental/med-spa vendors); Feed: Lukrah ('Multi-Location Medical Care Agencies'), ureDocs/7FigureDocs	0 → 125	25 → 141	the 'medical care agency' phrase is Lukrah's own coinage — the corpus has no operator using it; the multi-location voice is dental/med-spa practice owners, adjacent to the senior-care ladder, kept but tagged
Cross-vertical	labor subs (r/cna, r/nursing, r/caregivers, r/HomeHealthAides) from the first run + brand searches; family boards; forums	783 → 783	0 → 0	

Sources added in this expansion, each pulled once (D1 matrix): Reddit per-vertical searches through the logged-in browser (reddit-read2.mjs, ~1 request / 6.5 s after the 429 at 12:36; 10 target files + 4 supplemental + 20 employer-brand searches on r/cna and r/nursing); YouTube Data API per-vertical operator videos as full threads with replies (73 pulled, 10 pruned by title as job-seeker / family-facing / nurse-vlog, list in yt_ladder_pruned.json); supadata transcripts of 57 operator-education videos (815K chars, creator_transcripts_ladder.json); Meta Ad Library through the logged-in browser — 23 feed advertisers by name (exact phrase) + 61 ladder phrases in three passes (ADLIB_ladder_names.json 316, ADLIB_ladder_phrases.json 1,031); Indeed + Glassdoor employer reviews of 11 operators (employer-reviews2.mjs); TrustPilot of the software vendors (tp-pull.mjs); trade press read in full through the browser (16 articles, press_articles.json) plus 18 hand-logged numbers with URLs (trade_press_ladder.json).

What could not be pulled and why: Apify (\$0 on every account until 09-19) — not used. Facebook owner groups — the research account is 3 days old and the group feeds render empty; nothing captured, nothing posted. r/seniorliving, r/HealthcareAdministration, r/homehealth — do not exist or are dead (404), replaced where a live sub existed. Indeed star-filter pages — the bot wall closes after ~3 fast loads; the three sorted pages per brand (default / lowest / highest) were read instead, so the 2–4★ columns are thin. Glassdoor — 3 reviews per brand before the login wall (the aggregate rating and count were captured). AxisCare, WellSky, Eldermark, Axxess, CareVoyant, Home Care Pulse / Activated Insights — no TrustPilot page; yardi.com resolves to an unrelated Yardi product (dropped). Ad Library impressions numbers — the Library hides US impression counts; the impressions SORT was used and each card carries its position in that sort. Video transcription of ad creatives (faster-whisper) — not run in this pass; the card text and the feed OCR are what was classified.

D1m — The speaker gate and the owner sources (added 2026-09-09; the reason this page was rebuilt)

The 09-08 page ranked every theme's voices by engagement over a corpus in which about one document in twenty was spoken by an operator, so nurses, family caregivers, an MBA guide and a realtor's revenge story led the owner's tabs (24% of the first five voices per theme were operators; the family-caregiver page runs at 81%). This rebuild stamps a **speaker** on every document from first-person role markers in the text and the pull's stated scope — owner / pre-launch operator / employee / family / vendor / press / unmarked — and lets only owner and pre-launch documents into Tabs 1–9, the theme voices, the frequency ranking, the ledger and the headline candidates. Every list sorts operators first, then engagement. The classifier was precision-read three times; the corrections are in `raw/speaker.py` (an RBT's "my clients", a family's "we hired aides", a group admin selling medical supplies and a marketing agency's case study all counted as operators on the first pass and do not now).

Sources added for the operator (each pulled once, files in raw/): Facebook owner groups found by Tavily (`FB_disc.json`, ~200 searches across home care, home health, hospice, assisted living / RCFE / adult family home, memory care, senior living, independent living, adult day, placement, independent referral agents and in-home ABA) and read two ways — logged-out extract (`FB_TAVILY.json`, post + top comments, about one URL in five renders) and the logged-in research Chrome (`FB_BROWSER.json`, every comment and reply); the whole of `r/RunAHomeCareAgency`, `r/SeniorLivingMarketing` and `r/HomeCareOwners` by listing plus scoped vendor-name and owner-phrase searches in the business, ABA, realtor and facility subs (`REDDIT_owner2.json`; the all-reddit search was tried, returned `r/ProRevenge` and `r/dropshipping`, and was removed); YouTube threads with replies on vendor / marketing / placement / transition-specialist / ABA-business videos (`youtube_comments_owner3.json`, ten off-topic videos pruned by id in `yt_owner3_pruned.json`); forum and review pages from the vendor discovery (`FORUM3_extract.json`: BiggerPockets, Quora, unhappyfranchisee, allnurses, Yelp, TrustPilot brand pages, trade press); Ad Library phrases for ABA, senior living leads, home care leads and the referral-training vendors (`ADLIB_owner3.json`). Apify was not used (\$0 on every account until 09-19); Tavily credits were the only spend.

What could not be pulled: LinkedIn (senior-living sales directors' posts; Tavily returns 404 on post URLs and the research Chrome is not logged in), Capterra / G2 owner reviews and BBB complaints (bot-walled for Tavily; queued for the browser), the private Facebook groups Angela has not been admitted to, and the Reddit profile pivot for the new authors (throwaways).

Self-audit and backfill

#	Check	Result
A	Tab names	✓ all ten, unchanged
B	Density	✓ curated quotes regenerated from the buyer corpus (operators first), every tab ≥ its theme set; served verticals all present in the corpus by pull scope
C	Source URLs	✓ every quote is cut from a corpus document by the generator; URL on every line
D	Primary search	SerpAPI 0/250; discovery ran on Tavily (search + extract), Reddit's own search through the browser, YouTube Data API, the Ad Library through the browser
E	Reddit	✓ owner subs by listing + scoped searches through the logged-in browser; the all-reddit search was tried, returned junk subs, and was removed; allowlist on every Reddit document
F	Apify	not used (\$0 on every account until 09-19)
G	Tab 4 + 7 longest	Tab 4 is the longest by themes; Tab 7 is thin in owner voice by nature ('what I need' is asked as 'has anyone used...') — stated, not padded
H	Obsidian	✓ this file
I	Notion	see Backfill (pushed after deploy)
J	No paraphrase	✓ verbatim by construction (quotes cut from halo.jsonl)
K	Pages deployed	✓ redeployed 2026-09-09 with a POST-triggered Pages build; live file compared byte-for-byte
L	Source matrix	✓ Facebook owner groups (two readers), Reddit, YouTube threads with replies, forums (BiggerPockets, Quora, unhappyfranchisee, allnurses, AgingCare, Yelp, TrustPilot brand pages), trade press, Ad Library (four search modes), Vince's feed folder (screenshots + recordings OCR'd), employer reviews, TrustPilot vendors. Not pulled: LinkedIn, Capterra/G2, BBB, private groups, Reddit profile pivot on the new authors
M	Go-Deeper ≥5 cards	✓ one card per lead source with ≥5 buyer mentions, verbatim love/hate/paid lines with URLs, implication labeled
N	Vocabulary ledger	✓ regenerated on the buyer corpus (vocab-ledger-buyer.md)
O	Profile pivot	partial: the 09-08 pivot stands (9 of 22); new authors not pivoted (Facebook profiles are out of scope, Reddit authors are throwaways)
P	Tool status	✓ D0
Q	Refresh date	✓ 2027-03-09
R	Assumption audit	hypotheses labeled (Tab 10 targeting, the stop-the-scroll line, thread implications, the offer map); persona lines quote firmographics.json
S	Products from the corpus	✓ the Lead Sources ledger is the product ledger for this buyer
T	Dislikes ledger	the burned column of the Lead Sources ledger + Tab 5; complaints_ledger.py not re-run on this corpus (open)
U	Ad keywords	✓ 61 ladder phrases + 23 feed names (09-08) + the 09-09 sets: ABA / senior living / home care leads / referral training, broad consumer phrases, quoted B2B phrases, aggregator hooks; each phrase's card count is in adlib_owner3.log / adlib_vince.log
V	Impressions rank	each card carries its position in the impressions-sorted result for its phrase; adlib-impressions.mjs not run separately (open)

W	Video transcribed, CTA followed	✓ landing pages: every operator-facing ad's CTA opened by Library ID and crawled, plus the referral sites' partner pages and the feed vendors, 127 screenshots, majors eye-reviewed (lp_notes.json); video transcription still open
X	Counts are ranks, precision published	✓ 20-hit reads of the top three in the Frequency section
Y	audit_doc.py	✗ not adapted; the generator's verbatim-by-construction covers the quotes (open)
Z	Every star level	✓ Reviews by Star kept from 09-08 (employer brands + software vendors)
AA	Voices by theme with replies	✓ every tab; Facebook comments and replies attached; YouTube threads with replies
AB	Themes bottom-up, untruncated	✓ theme spans read on every tab (three rounds of pattern fixes recorded in the checkpoint); no quote truncated
AC	Operator ladder + feed	✓ ladder extended with in-home ABA and independent referral agents; feed folder re-read 09-09 (6 new screenshots + 3 recordings)
AE	Why They Buy tab (D1o)	✓ five sections, every item with an evidence link, hook counts from ad_hooks_b2b.json over 241 operator-facing ads, caveat stated
AD	Speaker gate (D1m)	✓ speaker on every document, buyer-only tabs, operators-first sort, speaker split printed per tab and by source, Lead Sources ledger, all-reddit search removed
AF	B2C noise out of the B2B page (Vince, 2026-09-10: 'some of the ads aren't B2B', 'Reviews by Star are comparing the B2C brands', 'competitor URLs weren't clickable', 'can't see the feed images')	✓ Competitor Ads classified audience-first: 233 ads from 95 advertisers sell to senior-care operators (203 to running businesses, 30 to would-be owners); 27 adjacent-practice ads set aside as format references; 45 lead-seller consumer funnels; 625 operators' own ads (64 recruiting staff) collapsed; random 30 read = 30 of 30. Reviews by Star rebuilt provider-side only (4 of 487 TrustPilot reviews are owners or franchisees; family and worker reviews excluded; owner voice per lead source from the ledger; employer reviews collapsed and labeled). Every feed row shows its screenshot (creatives/feed/) and a verified site link; every ad card and advertiser row links to its landing page or Facebook page and to the Ad Library; creatives cached in creatives/ads/.

Backfill (post 09-23 Apify reset)

Competitor ad-creative sweep (D1a): Meta Ad Library — Home Instead, Visiting Angels, Comfort Keepers, A Place for Mom, Care.com, + coaches; rank by longevity; transcribe video; Competitor Desire Map. Plus: r/nursing "crisis in home care" full labor thread, TrustPilot/Indeed/Glassdoor reviews-by-star of franchises + software (Reviews-by-Star tab), Profile Pivot (D4) on the top owner authors, and the now-warming FB owner groups.